

N-SUMHSS

NATIONAL SUBSTANCE USE AND MENTAL HEALTH SERVICES SURVEY

National Substance Use and Mental Health Services Survey 2024:

Data on Substance Use and Mental Health Treatment Facilities

SAMHSA

Substance Abuse and Mental Health
Services Administration

National Substance Use and Mental Health Services Survey 2024: Data on Substance Use and Mental Health Treatment Facilities

U.S. Department of Health and Human Services
Substance Use and Mental Health Services Administration
Center for Behavioral Health Statistics and Quality

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Executive Summary

The National Substance Use and Mental Health Services Survey (N-SUMHSS) is sponsored by the Center for Behavioral Health Statistics and Quality (CBHSQ) of the Substance Abuse and Mental Health Services Administration (SAMHSA). N-SUMHSS is a comprehensive national source of data on treatment services provided by substance use (SU) and mental health (MH) treatment facilities in the United States (U.S.), its territories, and the District of Columbia (D.C.).¹ Information collected from the N-SUMHSS can be used by behavioral health service providers, researchers, and federal, state, and local governments to understand the SU and MH treatment resource landscape, identify service gaps, and support evidence-based planning.

Data Collection

- The 2024 N-SUMHSS collected data from substance use and mental health treatment facilities in the U.S.¹
- The 2024 N-SUMHSS was a multimode survey and was conducted in both English and Spanish.

Data Presented in the 2024 N-SUMHSS Annual Report

- The 2024 N-SUMHSS report is based on data from 21,205 treatment facilities, including 15,953 SU facilities, 14,091 MH facilities, and 8,839 SU/MH facilities.^{2,3,4}

Response Rate

- The overall response rate for the 2024 N-SUMHSS among facilities eligible to participate was 90.4%.^{5,6}
- Thirty-three (33) states achieved response rates of 90.0% or more. The response rate for D.C. was 91.1%. Four out of seven U.S. territories achieved a response rate of 93.0% or more.¹

Substance Use and Mental Health Treatment Facilities' Characteristics, 2024 N-SUMHSS

- Private organizations operated 90.3% (n = 14,421) of SU treatment facilities, 86.0% (n = 12,114) of MH treatment facilities, and 87.5% (n = 7,733) of SU/MH treatment facilities.^{2,4,7,8}
- Outpatient care was the most frequent type of service provided among SU treatment facilities (83.8%, n = 13,369), MH treatment facilities (85.3%, n = 12,015), and SU/MH treatment facilities (91.5%, n = 8,088).^{2,4,8,9}
- Tailored programs for clients with co-occurring mental health and substance use disorders were offered at 64.9% (n = 10,349) of SU treatment facilities, 63.4% (n = 8,933) of MH treatment facilities, and 84.1% (n = 7,435) of combined SU/MH treatment facilities.^{2,4,8}
- Among SU treatment facilities utilizing pharmacotherapies, 61.7% (n = 9,842) provided medications for opioid use disorder (MOUD), 49.9% (n = 7,953) provided medication for alcohol use disorder (MAUD), and 47.8% (n = 7,628) provided tobacco cessation.^{2,4,8,10,11,12,13}
- Among MH treatment facilities utilizing pharmacotherapies, 60.2% (n = 8,489) provided antipsychotics to treat serious mental illness (SMI) and 43.1% (n = 6,071) provided medications for tobacco cessation.^{2,4,8,9,13,14}
- Overdose and Naloxone education was offered at 80.3% (n = 12,814) of SU treatment facilities.

Substance Use and Mental Health Treatment Facilities' Credentials, Operation, and Services, 2021–2024 N-SUMHSS

- In 2021, among SU treatment facilities, the top two licensing, certification, and accreditation sources were state SU agencies (75.9%, n = 10,632) and state departments of health (48.4%, n = 6,787). In 2024, the top two licensing, certification, and accreditation sources were state SU agencies (69.0%, n = 11,009) and state departments of health (46.2%, n = 7,374).^{2.4.8}
- In 2021, among MH treatment facilities, the top two licensing, certification, and accreditation sources were state MH authorities (68.8%, n = 6,226) and Centers for Medicare & Medicaid Services (CMS) (50.9%, n = 4,608). In 2024, the top two licensing, certification, and accreditation sources were state MH authorities (60.1%, n = 8,468) and state departments of health (49.1%, n = 6,917).^{2.4.8}
- In 2021, among SU treatment facilities, 90.8% (n = 12,715) were operated by private organizations. In 2024, 90.4% (n = 14,421) of SU treatment facilities were operated by private organizations.^{2.4.7.8}
- In 2021, among MH treatment facilities, 81.4% (n = 7,363) were operated by private organizations. In 2024, 86.0% (n = 12,114) of MH treatment facilities were operated by private organizations.^{2.4.8.9}
- In 2021, among SU treatment facilities offering tailored programs, the most offered programs were for adult women (54.3%, n = 7,610) and adult men (53.3%, n = 7,471). In 2024, the most offered programs were for adult women (56.5%, n = 9,013) and adult men (56.0%, n = 8,930).^{2.4.8}
- In 2021, 59.4% (n = 8,320) of SU treatment facilities and 75.9% (n = 6,869) of MH treatment facilities offered treatment for co-occurring disorders plus either SMI in adults and/or serious emotional disturbance (SED) in children. In 2024, 62.3% (n = 9,944) of SU treatment facilities and 80.4% (n = 11,334) of MH treatment facilities offered treatment for co-occurring disorders plus either SMI or SED.^{2.4.8}
- In 2021, among SU treatment facilities offering recovery support services, the two most common services offered were assistance obtaining social services (72.0%, n = 10,089) and assistance in locating housing (66.8%, n = 9,359). In 2024, the two most common services offered were assistance obtaining social services (76.2%, n = 12,151) and assistance in locating housing (71.1%, n = 11,335).^{2.4.8}
- In 2021, 45.2% (n = 6,334) of SU treatment facilities, 54.8% (n = 4,958) of MH treatment facilities, and 55.4% (n = 1,350) of combined SU/MH treatment facilities offered treatment at no charge or minimal payment to clients who could not afford to pay. In 2024, 46.3% (n = 7,383) of SU treatment facilities, 50.9% (n = 7,167) of MH treatment facilities, and 52.4% (n = 4,630) of combined SU/MH treatment facilities offered treatment at no charge or minimal payment to clients who could not afford to pay.^{2.4.8}
- In 2021, among SU treatment facilities, the two most common accepted forms of payment were cash or self-payment (90.5%, n = 12,680) and private insurance (75.2%, n = 10,539). In 2024, the two most common accepted forms of payment were cash or self-payment (89.9%, n = 14,337) and private insurance (78.3%, n = 12,494).^{2.4.8}
- In 2021, among MH treatment facilities accepting client payments or insurance, the two most common accepted forms of payment were Medicaid (87.9%, n = 7,951) and cash or self-payment (83.2%, n = 7,529). In 2024, the two most common accepted forms of payment were cash or self-payment (86.0%, n = 12,114) and Medicaid (84.5%, n = 11,904).^{2.4.8}
- In 2021, 25.7% (n = 3,604) of SU treatment facilities, 13.7% (n = 1,243) of MH treatment facilities, and 23.7% (n = 578) of combined SU/MH treatment facilities offered tailored programs for clients with HIV or AIDS. In 2024, 31.4% (n = 5,017) of SU treatment facilities, 20.1% (n = 2,830) of MH treatment facilities, and 37.9% (n = 3,351) of combined SU/MH treatment facilities offered tailored programs for clients with HIV or AIDS.^{2.4.8}
- In 2021, 28.7% (n = 4,023) of SU treatment facilities, 20.4% (n = 1,842) of MH treatment facilities, and 30.9% (n = 754) of combined SU/MH treatment facilities offered tailored programs for veterans. In 2024, 36.0% (n = 5,740) of SU treatment facilities, 28.4% (n = 4,001) of MH treatment facilities, and 44.9% (n = 3,967) of combined SU/MH treatment facilities offered tailored programs for veterans.^{2.4.8}

- In 2021, 17.9% (n = 2,507) of SU treatment facilities, 9.6% (n = 873) of MH treatment facilities, and 20.6% (n = 503) of combined SU/MH treatment facilities provided programs for active-duty military. In 2024, 23.9% (n = 3,811) of SU treatment facilities, 16.8% (n = 2,370) of MH treatment facilities, and 31.0% (n = 2,744) of combined SU/MH treatment facilities provided programs for active-duty military.^{2.4.8}
- In 2021, 46.4% (n = 6,503) of SU treatment facilities, 70.0% (n = 6,332) of MH treatment facilities, and 84.6% (n = 2,062) of combined SU/MH treatment facilities offered suicide prevention services. In 2024, 63.5% (n = 10,131) of SU treatment facilities, 68.4% (n = 9,645) of MH treatment facilities, and 82.8% (n = 7,317) of combined SU/MH treatment facilities offered suicide prevention services.^{2.4.8}
- In 2021, 69.5% (n = 9,736) of SU treatment facilities used any pharmacotherapy, with 55.3% (n = 7,753) providing MOUD, 37.9% (n = 5,312) providing MAUD, and 40.7% (n = 5,705) providing pharmacotherapies for tobacco cessation. In 2024, 76.5% (n = 12,207) of SU treatment facilities used any pharmacotherapy, with 61.7% (n = 9,842) providing MOUD, 49.9% (n = 7,953) providing MAUD, and 47.8% (n = 7,628) providing pharmacotherapies for tobacco cessation.^{2.4.8,10,11,12,13}
- In 2021, among SU treatment facilities, the two most offered assessment and pre-treatment services were screening for substance use (97.2%, n = 13,615) and comprehensive substance use assessment or diagnosis (94.9%, n = 13,302). In 2024, the two most offered assessment and pre-treatment services were screening for substance use (97.9%, n = 15,623) and comprehensive substance use assessment or diagnosis (95.5%, n = 15,239).^{2.4.8}
- In 2021, among SU treatment facilities, the two most offered testing services were drug or alcohol urine screening (88.4%, n = 12,384) and breathalyzer testing (64.0%, n = 8,960). In 2024, the two most offered testing services were drug or alcohol urine screening (88.6%, n = 14,134) and breathalyzer testing (60.9%, n = 9,719).^{2.4.8}
- In 2021, among SU treatment facilities offering medical services, 11.6% (n = 1,621) offered Hepatitis A vaccine (HAV), 11.6% (n = 1,624) offered Hepatitis B vaccine (HBV), and 69.3% (n = 9,708) offered neither. In 2024, 13.3% (n = 2,118) offered HAV, 13.8% (n = 2,207) offered HBV, and 71.4% (n = 11,398) offered neither.^{2.4.8}
- In 2021, among SU treatment facilities offering education and counseling services, the two most offered services were SU education (97.1%, n = 13,601) and individual counseling (96.0%, n = 13,456). In 2024, the two most offered services were SU education (97.7%, n = 15,587) and individual counseling (96.0%, n = 15,313).^{2.4.8}
- In 2021, among SU treatment facilities offering ancillary services, the two most offered services were case management (CM) services (83.7%, n = 11,727) and social skills development (73.5%, n = 10,293). In 2024, the two most offered services were CM services (84.7%, n = 13,515) and mental health services (75.3%, n = 12,019).^{2.4.8}
- In 2021, among MH treatment services offering services and practices, the two most offered services were CM (72.0%, n = 6,512) and family psychoeducation (70.2%, n = 6,351). In 2024, the two most offered services were CM (74.9%, n = 10,561) and screening for tobacco use (70.5%, n = 9,937).^{2.4.8}

Substance Use and Mental Health Treatment Services by Geographic Distribution, 2024 N-SUMHSS

- Among SU treatment facilities, 61.7% (n = 9,842) utilized MOUD and 49.9% (n = 7,953) utilized MAUD.^{2.4.8,11,12}
- Among SU treatment facilities, 61.7% (n = 9,842) utilized MOUD. Of the top 10 geographic distributions, more than 72.0% of SU treatment facilities utilized MOUD: New York (87.8%, n = 701), West Virginia (78.8%, n = 123), New Hampshire (77.7%, n = 73), Connecticut (77.6%, n = 149), Virginia (77.1%, n = 289), Pennsylvania (75.4%, n = 441), South Carolina (75.0%, n = 78), Ohio (74.4%, n = 548), Delaware (73.8%, n = 45), and Massachusetts (72.0%, n = 309).^{2.4.8,11}
- Among SU treatment facilities, 49.9% (n = 7,953) utilized MAUD. Of the top 10 geographic distributions, more than 60.5% of SU treatment facilities utilized MAUD: New York (80.2%, n = 640), Connecticut (75.0%, n = 144), New Hampshire (68.1%, n = 64), Delaware (67.2%, n = 41), Missouri (66.3%, n = 191), Ohio (64.3%, n = 474), Virginia (62.9%, n = 236), Massachusetts (62.2%, n = 267), West Virginia (60.9%, n = 95), and Kentucky (60.5%, n = 357).^{2.4.8,12}

- Among SU treatment facilities, 60.1% (n = 9,595) accepted clients using MOUD. Of the top 10 geographic distributions, more than 68.2% of SU treatment facilities accepted clients using MOUD: Montana (83.8%, n = 83), Alaska (80.0%, n = 72), Iowa (79.0%, n = 154), Wyoming (77.8%, n = 42), Idaho (76.9%, n = 83), Minnesota (76.6%, n = 291), North Dakota (73.9%, n = 51), Oregon (70.2%, n = 174), California (69.1%, n = 1,033), and Washington (68.2%, n = 257).^{2.4.8.11}
- Among SU treatment facilities providing medication services for OUD, 46.3% (n = 7,390) provided maintenance with methadone or buprenorphine. Of the top 10 geographic distributions, more than 60.3% of SU treatment facilities provided maintenance with methadone or buprenorphine: New York (71.1%, n = 567), Delaware (70.5%, n = 43), New Hampshire (66.0%, n = 62), West Virginia (66.0%, n = 103), Virginia (63.5%, n = 238), Connecticut (61.5%, n = 118), South Carolina (61.5%, n = 64), Vermont (60.7%, n = 34), Kentucky (60.7%, n = 358), and Pennsylvania (60.3%, n = 353).^{2.4.8.11}
- Among SU treatment facilities, 57.3% (n = 9,135) accepted clients using MAUD. Of the top 10 geographic distributions, more than 67.6% of SU treatment facilities accepted clients using MAUD: Montana (81.8%, n = 81), Alaska (81.1%, n = 73), Iowa (78.5%, n = 153), North Dakota (73.9%, n = 51), Minnesota (72.6%, n = 276), Idaho (71.3%, n = 77), Nevada (69.7%, n = 92), Wyoming (68.5%, n = 37), New Jersey (67.6%, n = 279), and Michigan (67.6%, n = 305).^{2.4.8.12}
- Among MH treatment facilities using medications to treat clients with co-occurring mental and substance use disorders, 39.8% (n = 5,603) used MOUD. Of the top 10 geographic distributions, more than 49.5% of MH treatment facilities used MOUD: Delaware (61.7%, n = 29), West Virginia (60.6%, n = 83), Oklahoma (54.8%, n = 86), Virginia (54.0%, n = 168), Missouri (53.8%, n = 148), New Hampshire (53.4%, n = 39), Kansas (51.9%, n = 69), New York (51.1%, n = 405), Other jurisdictions (50.0%, n = 3), and Colorado (49.5%, n = 108).^{2.4.8.11}
- Among MH treatment facilities using medications to treat clients with co-occurring mental and substance use disorders, 33.9% (n = 4,775) used MAUD. Of the top 10 geographic distributions, more than 42.9% of MH treatment facilities used MAUD: Delaware (61.7%, n = 29), West Virginia (52.6%, n = 72), Connecticut (49.8%, n = 121), New Hampshire (49.3%, n = 36), Missouri (48.4%, n = 133), Wyoming (46.9%, n = 23), Kansas (45.9%, n = 61), Massachusetts (44.0%, n = 143), Colorado (43.6%, n = 95), and New York (42.9%, n = 340).^{2.4.8.12}
- Among SU treatment facilities, 63.5% (n = 10,131) provided suicide prevention services. Of the top 10 geographic distributions, more than 76.5% of SU treatment facilities provided suicide prevention services: Wyoming (88.9%, n = 48), Other jurisdictions (87.5%, n = 7), Arkansas (82.6%, n = 128), Puerto Rico (81.7%, n = 58), Alaska (80.0%, n = 72), Missouri (79.2%, n = 228), Montana (78.8%, n = 78), Kentucky (78.0%, n = 460), Oklahoma (76.8%, n = 136), and Nevada (76.5%, n = 101).^{2.4.8}
- Among MH treatment facilities, 68.4% (n = 9,645) provided suicide prevention services. Of the top 10 geographic distributions, more than 79.3% of MH treatment facilities provided suicide prevention services: Oklahoma (89.8%, n = 141), South Dakota (89.1%, n = 41), Wyoming (87.8%, n = 43), South Carolina (85.3%, n = 81), Delaware (83.0%, n = 39), New Mexico (82.0%, n = 100), Puerto Rico (82.0%, n = 50), Indiana (80.7%, n = 331), Missouri (80.4%, n = 221), and Louisiana (79.3%, n = 138).^{2.4.8}
- Among combined SU and MH treatment facilities, 82.8% (n = 7,317) provided suicide prevention services. Of the top 10 geographic distributions, more than 91.3% of combined SU and MH treatment facilities provided suicide prevention services: Other jurisdictions (100.0%, n = 6), Oklahoma (97.6%, n = 123), Montana (96.9%, n = 62), South Dakota (96.2%, n = 25), Kansas (93.3%, n = 83), New Hampshire (92.9%, n = 39), Wyoming (92.9%, n = 39), Alaska (92.8%, n = 64), New Mexico (92.2%, n = 95), and Kentucky (91.3%, n = 283).^{2.4.8}

Section 1 | Introduction

Key Takeaways

- The 2024 N-SUMHSS collected data from substance use and mental health treatment facilities in the U.S.¹
- The 2024 N-SUMHSS was a multimode survey and was conducted in both English and Spanish.
- The 2024 N-SUMHSS report is based on data from 21,205 treatment facilities, including 15,953 SU facilities, 14,091 MH facilities, and 8,839 SU/MH facilities.^{2,3,4}
- The overall response rate for the 2024 N-SUMHSS among facilities eligible to participate was 90.4%.⁵
- Thirty-three (33) states achieved response rates of 90.0% or more. The response rate for the D.C. was 91.1%. Four out of seven U.S. territories achieved a response rate of 93.0% or more.¹

The N-SUMHSS is a comprehensive national source of data on SU and MH treatment facilities. It collects data on the location, characteristics, and utilization of SU and MH treatment services in the U.S., its territories, and D.C.¹ The N-SUMHSS provides behavioral health service providers; researchers; and federal, state, and local governments with information to understand the SU and MH treatment resource landscape, identify service gaps, and support evidence-based planning.

Prior to 2021, SAMHSA had collected data on the SU and MH treatment services offered by facilities for more than two decades. Data were collected by using two surveys, the National Survey of Substance Abuse Treatment Services (N-SSATS) and the National Mental Health Services Survey (N-MHSS). The N-SSATS and N-MHSS were later combined into one survey, to reduce the burden on facilities offering both SU and MH treatment services, optimize government resources to collect data, and enhance the quality of data collected on the treatment facilities.

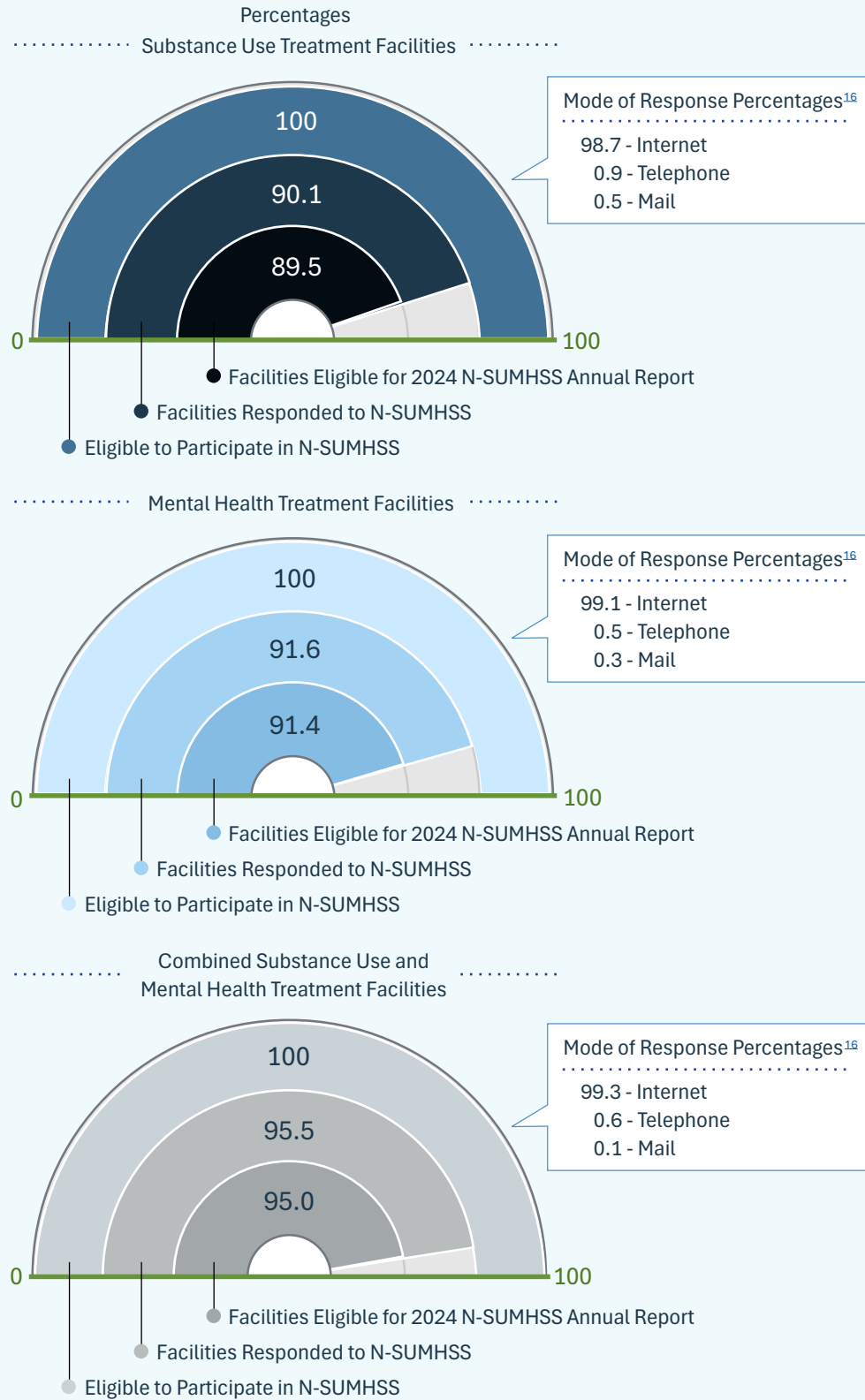
Data Collection

The 2024 N-SUMHSS was a multimode survey, utilizing three survey modes: 1) a secured web-based survey, 2) a postal paper survey, and 3) a computer-assisted telephone interview (CATI). There were 27,957 SU and MH treatment facilities known to SAMHSA in the U.S. in 2024, of which 4,009 were found to be either closed or ineligible for the N-SUMHSS.^{1,6,15} Out of 23,948 facilities eligible for the 2024 N-SUMHSS, 85.7% (n = 17,829) facilities provided SU treatment, 86.9% (n = 15,421) facilities provided MH treatment, and 87.7% (n = 9,302) facilities provided both SU and MH treatment.^{2,4,6} The 2024 N-SUMHSS was conducted in both English and Spanish. For more information about data collection, please see the Survey Products and Dissemination Section in Appendix B.

Data Presented in the 2024 N-SUMHSS Annual Report

Of the 23,948 facilities eligible to participate in the N-SUMHSS, this report includes data and findings from 21,205 facilities who responded to the survey.³ These facilities are composed of 15,953 SU facilities (89.5% of 17,829 eligible SU facilities), 14,091 MH facilities (91.4% of 15,421 eligible MH facilities), and 8,839 SU/MH facilities (95% of 9,302 eligible SU/MH facilities).^{2,3} Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Figure 1.1 below provides percentages of facilities included in this report by facility type. For more information about data collection, please refer to Appendix B.

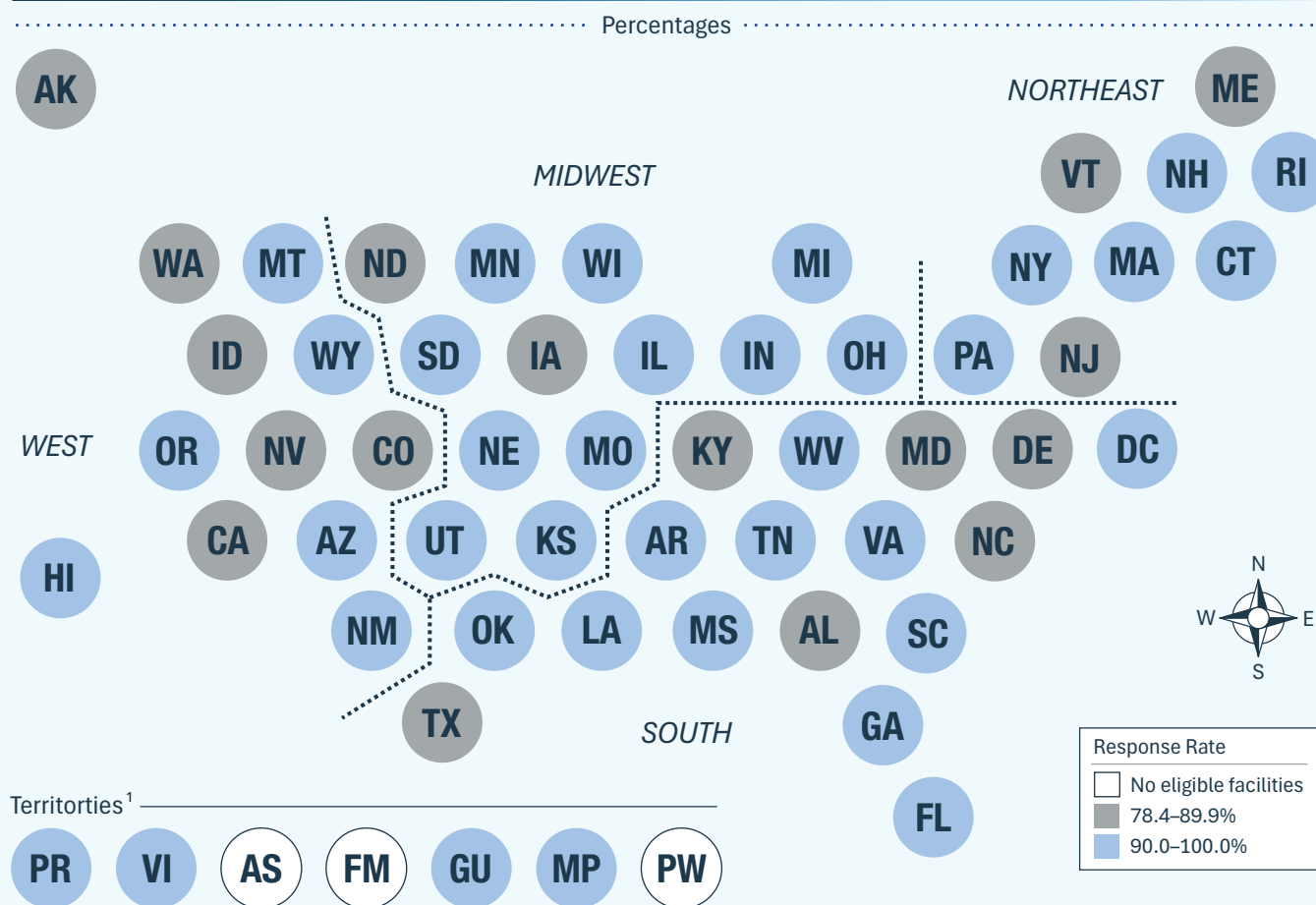
Figure 1.1: Substance Use and Mental Health Treatment Facilities by Eligibility Status and Mode of Survey Response, 2024 N-SUMHSS



2024 N-SUMHSS Response Rates

The overall response rate among facilities that were eligible to participant in the 2024 N-SUMHSS was 90.4%. The response rate was calculated using the American Association for Public Opinion Research (AAPOR) Response Rate 4 (RR4) standard definition.⁵ Figure 1.2 provides a map of the 2024 N-SUMHSS response rates across the U.S., its territories, and D.C. overlaid by Census regions.¹ The response rates for participating territories are displayed in separate circles at the bottom of the map. To see the response rate for each individual state, territory, or D.C., please refer to Appendix A.

Figure 1.2: Response Rates, 2024 N-SUMHSS



The 2024 N-SUMHSS response rates for the 50 states ranged from 78.4% (North Dakota) to 96.1% (Hawaii and Missouri). Thirty-three (33) states achieved response rates of 90% or more. The response rate for D.C. was 91.1%. Four out of seven U.S. territories/jurisdictions achieved a response rate of 93% or more.¹

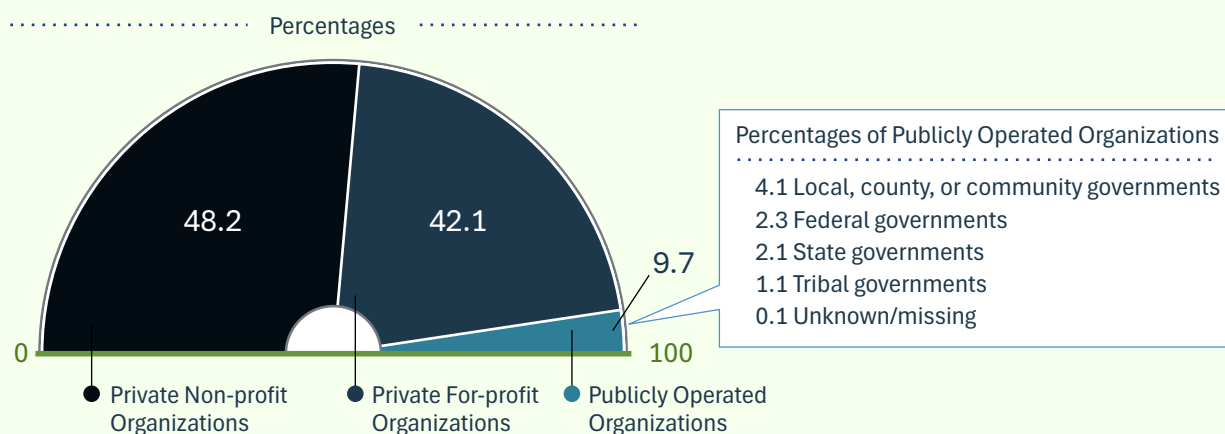
Section 2 | Substance Use and Mental Health Treatment Facilities' Characteristics, 2024 N-SUMHSS

This section provides data on operational characteristics and the use of pharmacotherapies (medications) by SU, MH, and/or SU/MH facilities from the 2024 N-SUMHSS. Percentages are calculated based on the total facility count of each SU, MH, and SU/MH facility type.^{2,4} Since medications used for SU treatment vary from those used for MH treatment, data are presented separately. The data are visualized as percentages and the narratives describing the findings from the graphs include counts for reference. For detailed data on counts and percentages for this section, please refer to Appendix A.

Key Takeaways

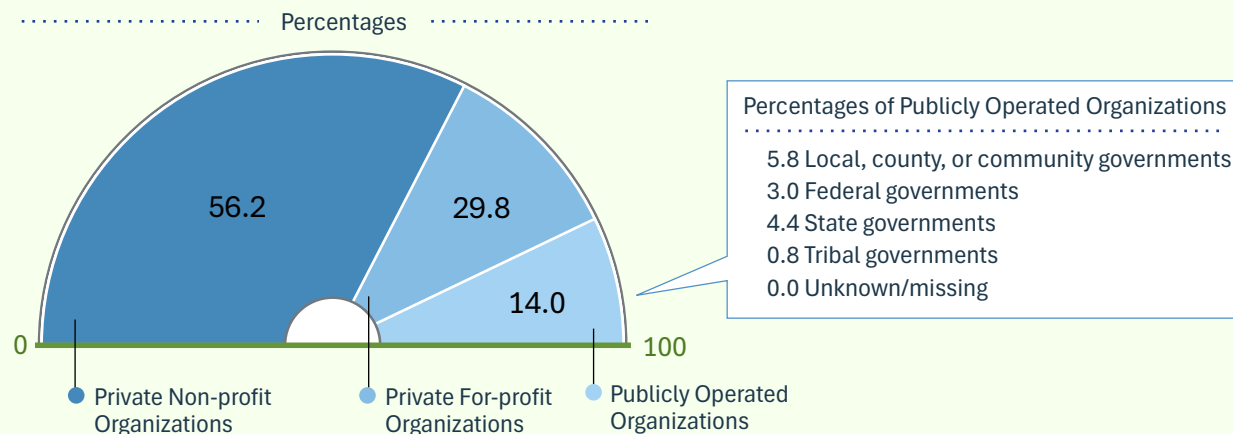
- Private organizations operated 90.3% (n = 14,421) of SU treatment facilities, 86.0% (n = 12,114) of MH treatment facilities, and 87.5% (n = 7,733) of SU/MH treatment facilities.^{2,4,7,8}
- Outpatient care was the most frequent type of service provided among SU treatment facilities (83.8%, n = 13,369), MH treatment facilities (85.3%, n = 12,015), and SU/MH treatment facilities (91.5%, n = 8,088).^{2,4,8,9}
- Tailored programs for clients with co-occurring mental health and substance use disorders were offered at 64.9% (n = 10,349) of SU treatment facilities, 63.4% (n = 8,933) of MH treatment facilities, and 84.1% (n = 7,435) of combined SU/MH treatment facilities.^{2,4,8}
- Among SU treatment facilities utilizing pharmacotherapies, 61.7% (n = 9,842) provided MOUD, 49.9% (n = 7,953) provided MAUD, and 47.8% (n = 7,628) provided tobacco cessation.^{2,4,8,10,11,12,13}
- Among MH treatment facilities utilizing pharmacotherapies, 60.2% (n = 8,489) provided antipsychotics to treat SMI, and 43.1% (n = 6,071) provided medications for tobacco cessation.^{2,4,8,13,14}
- Overdose and Naloxone education was offered at 80.3% (n = 12,814) of SU treatment facilities.^{2,4,7,8}

Figure 2.1.1: Substance Use Treatment Facilities by Operation Type, 2024 N-SUMHSS



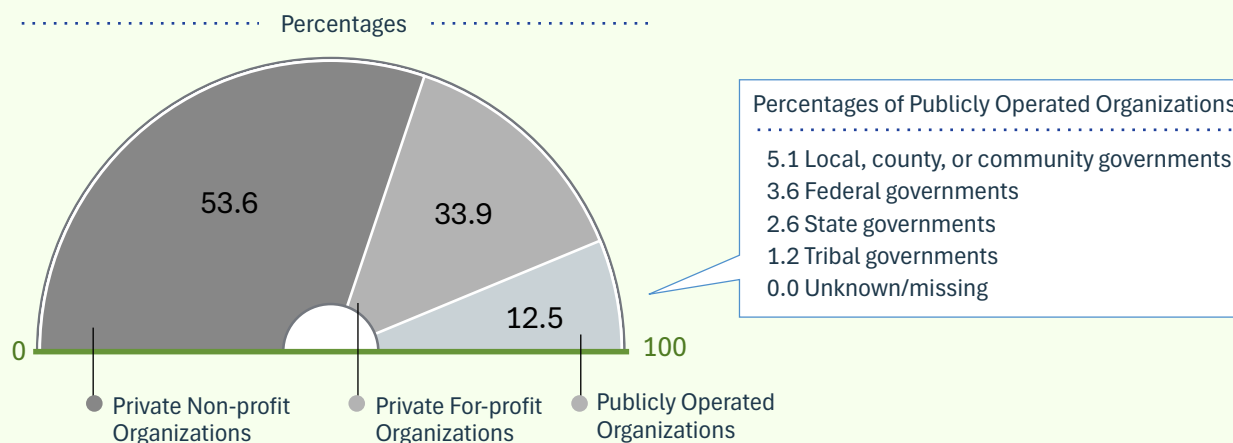
SU treatment facilities were operated by 48.2% (n = 7,697) of private non-profit organizations; 42.1% (n = 6,724) of private for-profit organizations; 4.1% (n = 655) of local, county, or community governments; 2.3% (n = 359) of federal governments; 2.1% (n = 331) of state governments; and 1.1% (n = 176) of tribal governments.^{2,4,7,8}

Figure 2.1.2: Mental Health Treatment Facilities by Operation Type, 2024 N-SUMHSS



MH treatment facilities were operated by 56.2% (n = 7,915) of private non-profit organizations; 29.8% (n = 4,199) of private for-profit organizations; 5.8% (n = 817) of local, county, or community governments; 3.0% (n = 424) of federal governments; 4.4% (n = 621) of state governments; and 0.8% (n = 110) of tribal governments.^{2,4,7,8}

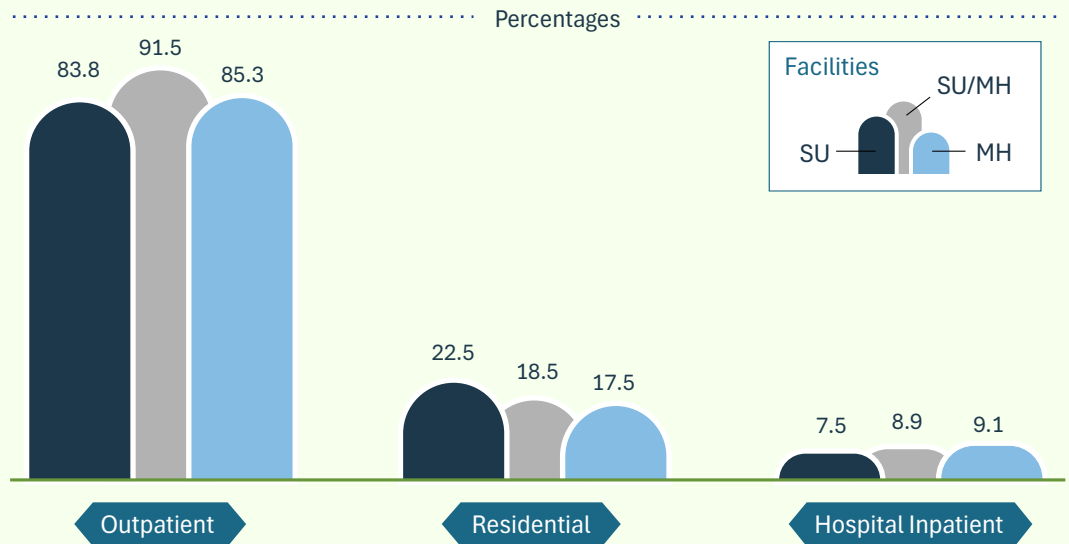
Figure 2.1.3: Combined Substance Use and Mental Health Treatment Facilities by Operation Type, 2024 N-SUMHSS



Combined SU/MH treatment facilities were operated by 53.6% (n = 4,740) of private non-profit organizations; 33.9% (n = 2,993) of private for-profit organizations; 5.1% (n = 450) of local, county, or community governments; 3.6% (n = 318) of federal governments; 2.6% (n = 231) of state governments; and 1.2% (n = 107) of tribal governments.^{2,4,8}

SU (83.8%, n = 13,369), MH (85.3%, n = 12,015), and combined SU/MH (91.5%, n = 8,088) treatment facilities predominantly offered outpatient care. Residential care was offered at 22.5% (n = 3,596) of SU treatment facilities, 17.5% (n = 2,460) of MH treatment facilities, and 18.5% (n = 1,639) of combined SU/MH treatment facilities. Hospital inpatient care was offered at 7.5% (n = 1,201) of SU treatment facilities, 9.1% (n = 1,288) of MH treatment facilities, and 8.9% (n = 786) of combined treatment SU/MH facilities.^{2,4,8,9}

Figure 2.2: Substance Use and Mental Health Treatment Facilities by Type of Care, 2024 N-SUMHSS



Among SU, MH, and combined SU/MH treatment facilities, tailored programs for specific groups were offered most often for clients with co-occurring mental health and substance use disorders, with 64.9% (n = 10,349) of SU treatment facilities, 63.4% (n = 8,933) of MH treatment facilities, and 84.1% (n = 7,435) of combined SU/MH treatment facilities offering tailored services. Among SU treatment facilities, tailored programs were offered least often for active-duty military (23.9%, n = 3,811). Among MH treatment facilities, tailored programs were offered least often for clients who have experienced intimate partner violence or domestic violence (14.1%, n = 1,989). Among combined SU/MH treatment facilities, tailored programs were offered least often for active-duty military (31.0%, n = 2,744).^{2,4,8}

Figure 2.3: Substance Use and Mental Health Facilities Offering Tailored Programs for Specific Groups, N-SUMHSS 2024

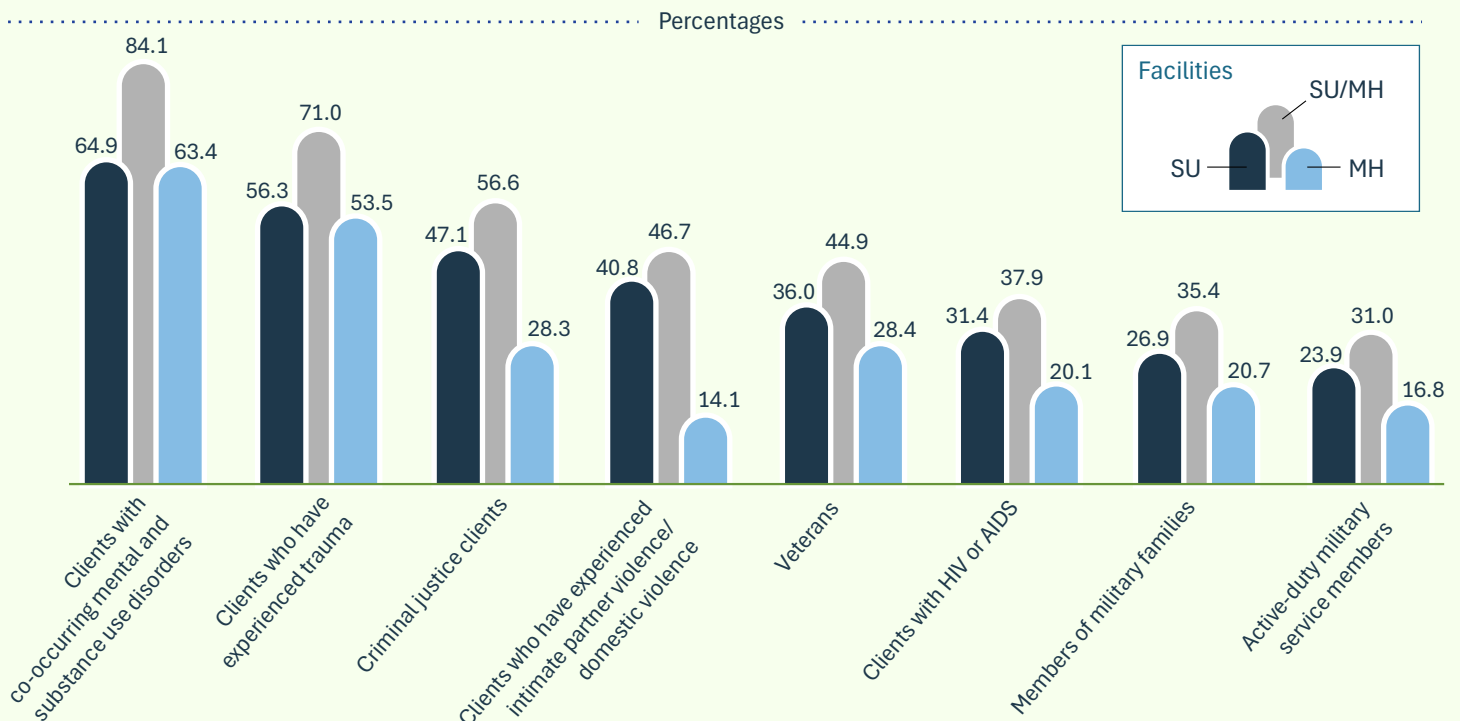
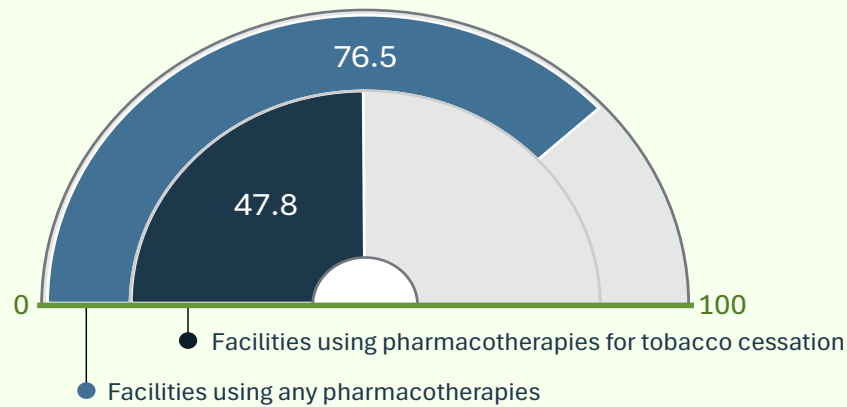
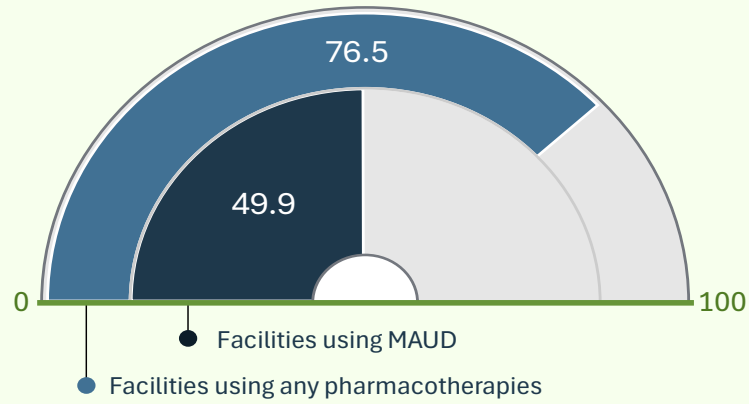
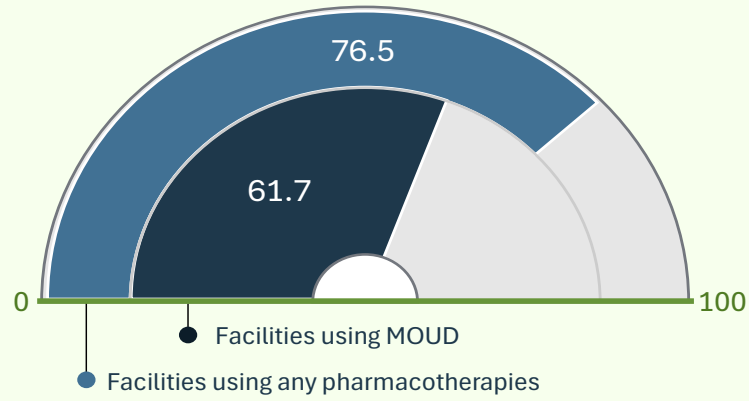


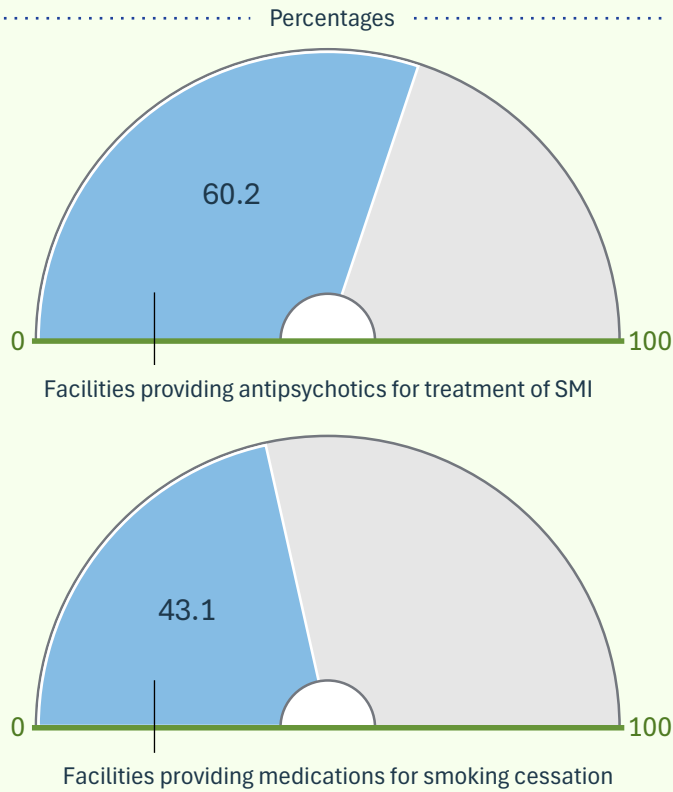
Figure 2.4: Substance Use Facilities Utilizing Pharmacotherapies, 2024 N-SUMHSS

Percentages



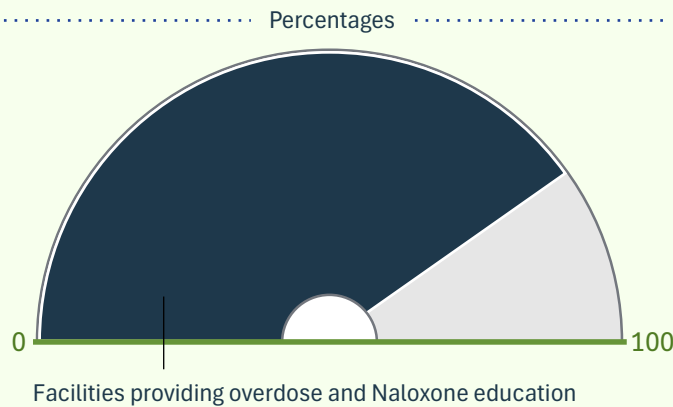
Among SU treatment facilities utilizing pharmacotherapies, 61.7% (n = 9,842) provided MOUD, 49.9% (n = 7,953) provided MAUD, and 47.8% (n = 7,628) provided tobacco cessation.[2,4,8,10,11,12,13](#)

Figure 2.5: Mental Health Treatment Facilities Utilizing Pharmacotherapies, 2024 N-SUMHSS



Among MH treatment facilities utilizing pharmacotherapies, 60.2% (n = 8,489) provided antipsychotics for the treatment of SMI and 43.1% (n = 6,071) provided medications for smoking cessation. [2,4,8,13,14](#)

Figure 2.6: Substance Use Treatment Facilities Offering Overdose and Naloxone Education, 2024 N-SUMHSS



Among SU treatment facilities, 80.3% (n = 12,814) offered overdose and Naloxone education. [2,4,8](#)

Section 3 | Substance Use and Mental Health Treatment Facilities' Credentials, Operations, and Services, 2021–2024 N-SUMHSS

This section provides data on the facilities' credentials, operations, and services by SU, MH, and/or combined SU/MH facilities^{2.4} in 2021–2024 N-SUMHSS. The data are visualized as percentages and the narratives describing the findings from the graphs include counts for reference. Please note, many of the figures in this section only visualize the top five selected variables. For detailed data on counts and percentages for all variables, please refer to Appendix A.

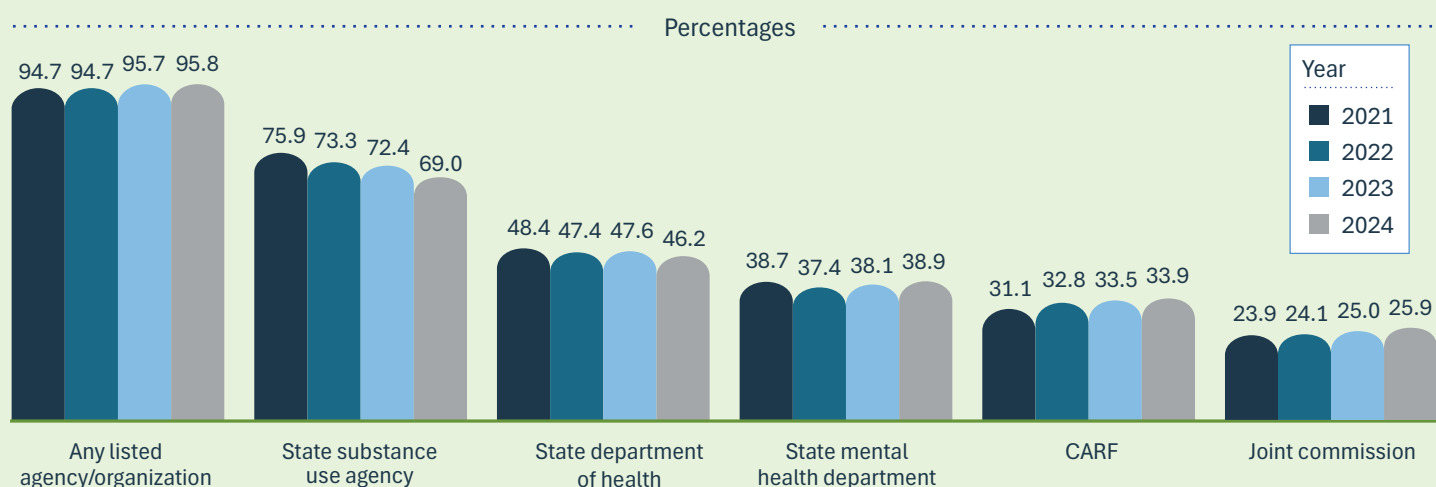
Key Takeaways

- In 2021, among SU treatment facilities, the top two licensing, certification, and accreditation sources were state SU agencies (75.9%, n = 10,632) and state departments of health (48.4%, n = 6,787). In 2024, the top two licensing, certification, and accreditation sources were state SU agencies (69.0%, n = 11,009) and state departments of health (46.2%, n = 7,374).^{4.11.12}
- In 2021, among MH treatment facilities, the top two licensing, certification, and accreditation sources were state MH authorities (68.8%, n = 6,226) and CMS (50.9%, n = 4,608). In 2024, the top two licensing, certification, and accreditation sources were state MH authorities (60.1%, n = 8,468) and state departments of health (49.1%, n = 6,917).^{2.4.8}
- In 2021, among SU treatment facilities, 90.8% (n = 12,715) were operated by private organizations. In 2024, 90.4% (n = 14,421) SU treatment facilities were operated by private organizations.^{2.4.7.8}
- In 2021, among SU treatment facilities offering tailored programs, the most offered programs were for adult women (54.3%, n = 7,610) and adult men (53.3%, n = 7,471). In 2024, the most offered programs were for adult women (56.5%, n = 9,013) and adult men (56.0%, n = 8,930).^{2.4.8}
- In 2021, 59.4% (n = 8,320) of SU treatment facilities and 75.9% (n = 6,869) of MH treatment facilities offered treatment for co-occurring disorders plus either SMI in adults and/or SED in children. In 2024, 62.3% (n = 9,944) of SU treatment facilities and 80.4% (n = 11,334) of MH treatment facilities offered treatment for co-occurring disorders plus either SMI or SED.^{2.4.8.16}
- In 2021, among SU treatment facilities offering recovery support services, the two most common services offered were assistance obtaining social services (72.0%, n = 10,089) and assistance in locating housing (66.8%, n = 9,359). In 2024, the two most common services offered were assistance obtaining social services (76.2%, n = 12,151) and assistance in locating housing (71.1%, n = 11,335).^{2.4.8}
- In 2021, 45.2% (n = 6,334) of SU treatment facilities, 54.8% (n = 4,958) of MH treatment facilities, and 55.4% (n = 1,350) of combined SU/MH treatment facilities offered treatment at no charge or minimal payment to clients who could not afford to pay. In 2024, 46.3% (n = 7,383) of SU treatment facilities, 50.9% (n = 7,167) of MH treatment facilities, and 52.4% (n = 4,630) of combined SU/MH treatment facilities offered treatment at no charge or minimal payment to clients who could not afford to pay.^{2.4.8}
- In 2021, among SU treatment facilities, the two most commonly accepted forms of payment were cash or self-payment (90.5%, n = 12,680) and private insurance (75.2%, n = 10,539). In 2024, the two most commonly accepted forms of payment were cash or self-payment (89.9%, n = 14,337) and private insurance (78.3%, n = 12,494).^{2.4.8}
- In 2021, among MH treatment facilities accepting client payments or insurance, the two most commonly accepted forms of payment were Medicaid (87.9%, n = 7,951) and cash or self-payment (83.2%, n = 7,529). In 2024, the two most commonly accepted forms of payment were cash or self-payment (86.0%, n = 12,114) and Medicaid (84.5%, n = 11,904).^{2.4.8}
- In 2021, 25.7% (n = 3,604) of SU treatment facilities, 13.7% (n = 1,243) of MH treatment facilities, and 23.7% (n = 578) of combined SU/MH treatment facilities offered tailored programs for clients with HIV or AIDS. In 2024, 31.4% (n = 5,017) of SU treatment facilities, 20.1% (n = 2,830) of MH treatment facilities, and 37.9% (n = 3,351) of combined SU/MH treatment facilities offered tailored programs for clients with HIV or AIDS.^{2.4.8}

- In 2021, 28.7% (n = 4,023) of SU treatment facilities, 20.4% (n = 1,842) of MH treatment facilities, and 30.9% (n = 754) of combined SU/MH treatment facilities offered tailored programs for veterans. In 2024, 36.0% (n = 5,740) of SU treatment facilities, 28.4% (n = 4,001) of MH treatment facilities, and 44.9% (n = 3,967) of combined SU/MH treatment facilities offered tailored programs for veterans.^{2,4,8}
- In 2021, 17.9% (n = 2,507) of SU treatment facilities, 9.6% (n = 873) of MH treatment facilities, and 20.6% (n = 503) of combined SU/MH treatment facilities provided programs for active-duty military. In 2024, 23.9% (n = 3,811) of SU treatment facilities, 16.8% (n = 2,370) of MH treatment facilities, and 31.0% (n = 2,744) of combined SU/MH treatment facilities provided programs for active-duty military.^{2,4,8}
- In 2021, 46.4% (n = 6,503) of SU treatment facilities, 70.0% (n = 6,332) of MH treatment facilities, and 84.6% (n = 2,062) of combined SU/MH treatment facilities offered suicide prevention services. In 2024, 63.5% (n = 10,131) of SU treatment facilities, 68.4% (n = 9,645) of MH treatment facilities, and 82.8% (n = 7,317) of combined SU/MH treatment facilities offered suicide prevention services.^{2,4,8}
- In 2021, 69.5% (n = 9,736) of SU treatment facilities used any pharmacotherapy, with 55.3% (n = 7,753) providing MOUD, 37.9% (n = 5,312) providing MAUD, and 40.7% (n = 5,705) providing pharmacotherapies for tobacco cessation. In 2024, 76.5% (n = 12,207) of SU treatment facilities used any pharmacotherapy, with 61.7% (n = 9,842) providing MOUD, 49.9% (n = 7,953) providing MAUD, and 47.8% (n = 7,628) providing pharmacotherapies for tobacco cessation.^{2,4,8,10,11,12,13}
- In 2021, among SU treatment facilities, the two most offered assessment and pre-treatment services were screening for substance use (97.2%, n = 13,615) and comprehensive substance use assessment or diagnosis (94.9%, n = 13,302). In 2024, the two most offered assessment and pre-treatment services were screening for substance use (97.9%, n = 15,623) and comprehensive substance use assessment or diagnosis (95.5%, n = 15,239).^{2,4,8}
- In 2021, among SU treatment facilities, the two most offered testing services were drug or alcohol urine screening (88.4%, n = 12,384) and breathalyzer testing (64.0%, n = 8,960). In 2024, the two most offered testing services were drug or alcohol urine screening (88.6%, n = 14,134) and breathalyzer testing (60.9%, n = 9,719).^{2,4,8}
- In 2021, among SU treatment facilities offering medical services, 11.6% (n = 1,621) offered Hepatitis A vaccine (HAV), 11.6% (n = 1,624) offered Hepatitis B vaccine (HBV), and 69.3% (n = 9,708) offered neither. In 2024, 13.3% (n = 2,118) offered HAV, 13.8% (n = 2,207) offered HBV, and 71.4% (n = 11,398) offered neither.^{2,4,8}
- In 2021, among SU treatment facilities offering education and counseling services, the two most offered services were substance use education (97.1%, n = 13,601) and individual counseling (96.0%, n = 13,456). In 2024, the two most offered services were substance use education (97.7%, n = 15,587) and individual counseling (96.0%, n = 15,313).^{2,4,8}
- In 2021, among SU treatment facilities offering ancillary services, the two most offered services were case management (CM) services (83.7%, n = 11,727) and social skills development (73.5%, n = 10,293). In 2024, the two most offered services were CM services (84.7%, n = 13,515) and mental health services (75.3%, n = 12,019).^{2,4,8}

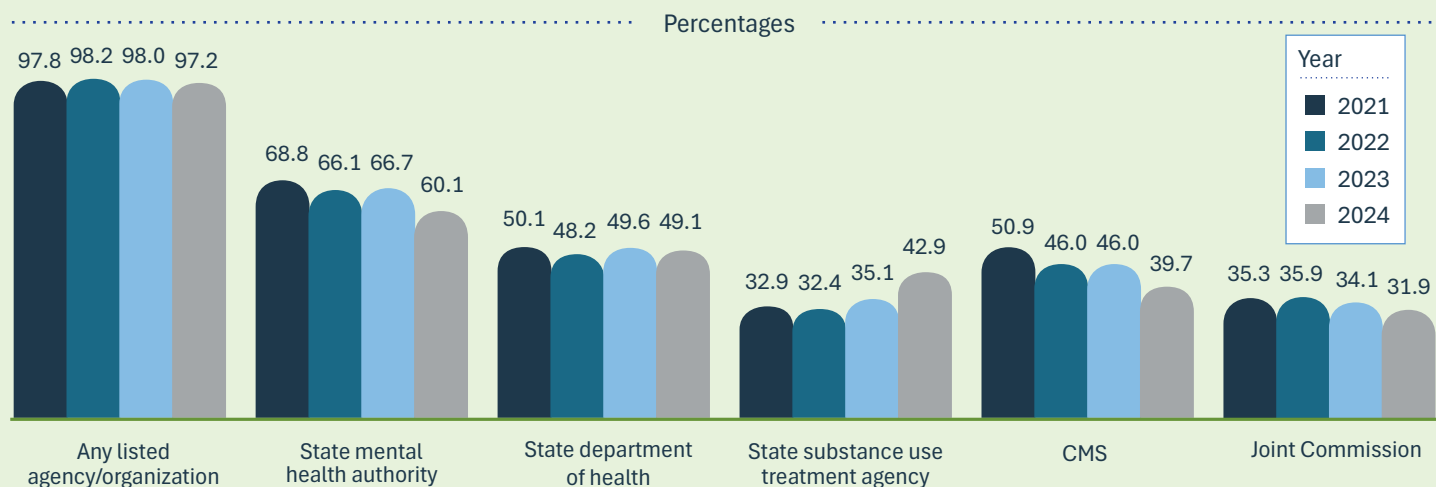
3.1: Substance Use and Mental Health Treatment Facilities' Licensing, Certification, or Accreditation, 2021–2024 N-SUMHSS

Figure 3.1.1: Substance Use Treatment Facilities' Licensing, Certification, or Accreditation (Top Five), 2021–2024 N-SUMHSS



In 2021, 94.7% (n = 13,263) of SU treatment facilities obtained licensing, certification, or accreditation. The top five licensing, certification, and accreditation sources were state SU treatment agencies (75.9%, n = 10,632), state departments of health (48.4%, n = 6,787), state MH departments (38.7%, n = 5,427), Commission of Accreditation of Rehabilitation Facilities (CARF) (31.1%, n = 4,354), and the Joint Commission (23.9%, n = 3,346). In 2024, 95.8% (n = 15,288) of SU treatment facilities obtained licensing, certification, or accreditation. The top five sources of licensing, certification, and accreditation were state substance use treatment agencies (69.0%, n = 11,009), state departments of health (46.2%, n = 7,374), state mental health departments (38.9%, n = 6,201), CARF (33.9%, n = 5,414), and the Joint Commission (25.9%, n = 4,133).^{2,4,8}

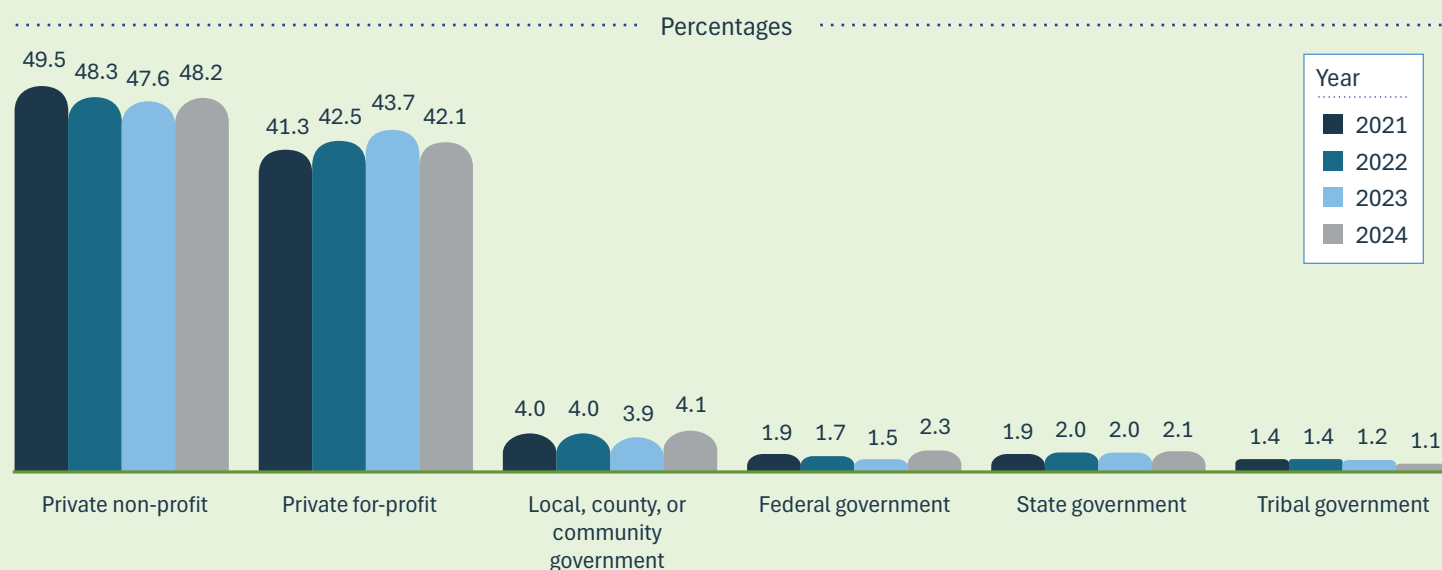
Figure 3.1.2: Mental Health Treatment Facilities' Licensing, Certification, or Accreditation (Top Five), 2021–2024 N-SUMHSS



In 2021, 97.8% (n = 8,854) of MH treatment facilities obtained licensing, certification, or accreditation. The top five licensing, certification, and accreditation sources were state MH authorities (68.8%, n = 6,226), CMS (50.9%, n = 4,608), state departments of health (50.1%, n = 4,533), the Joint Commission (35.3%, n = 3,198), and state SU treatment agencies (32.9%, n = 2,976). In 2024, 97.2% (n = 13,701) of MH treatment facilities obtained licensing, certification, or accreditation. The top five licensing, certification, and accreditation sources were state mental health authorities (60.1%, n = 8,468), state departments of health (49.1%, n = 6,917), state SU treatment agencies (42.9%, n = 6,052), CMS (39.7%, n = 5,588), and the Joint Commission (31.9%, n = 4,499).^{2,4,8}

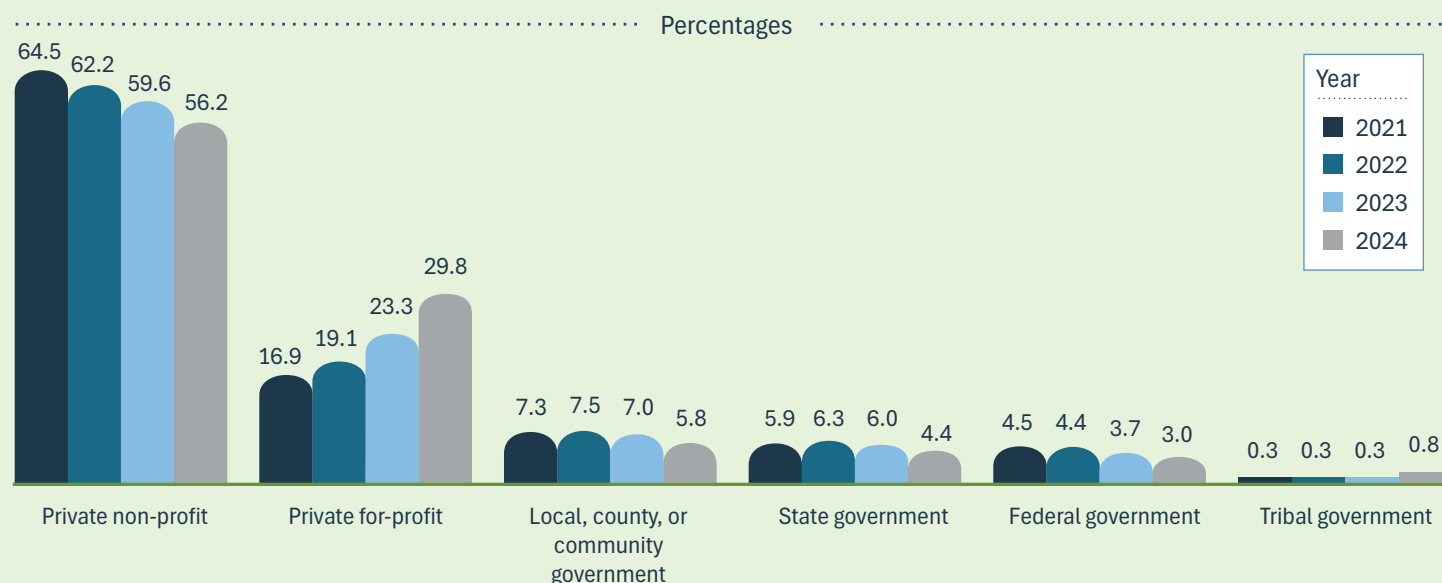
3.2: Substance Use and Mental Health Treatment Facilities by Operation Type, 2021–2024 N-SUMHSS

Figure 3.2.1: Substance Use Treatment Facilities by Operation Type, 2021–2024 N-SUMHSS



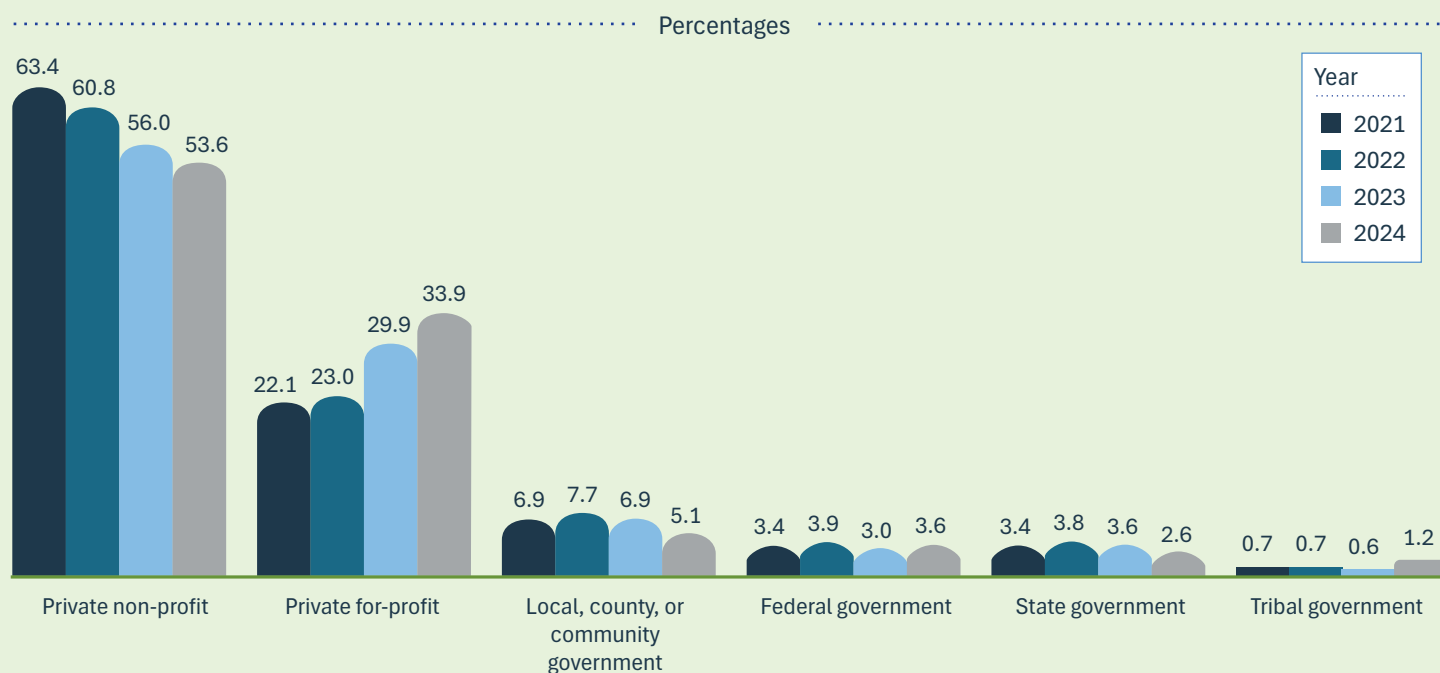
SU treatment facilities were operated predominantly by private entities. In 2021, private non-profit organizations operated 49.5% (n = 6,932) of SU treatment facilities and private for-profit organizations operated 41.3% (n = 5,783) of SU facilities. In 2024, private non-profit organizations operated 48.2% (n = 7,697) of SU treatment facilities and private for-profit organizations operated 42.1% (n = 6,724) of SU facilities. [2,4,7,8](#)

Figure 3.2.2: Mental Health Treatment Facilities by Operation Type, 2021–2024 N-SUMHSS



MH treatment facilities were operated predominantly by private entities. In 2021, private non-profit organizations operated 64.5% (n = 5,835) of MH facilities and private for-profit organizations operated 16.9% (n = 1,528) of MH treatment facilities. In 2024, private non-profit organizations operated 56.2% (n = 7,915) of MH treatment facilities and private for-profit organizations operated 29.8% (n = 4,199) of MH facilities. [2,4,7,8](#)

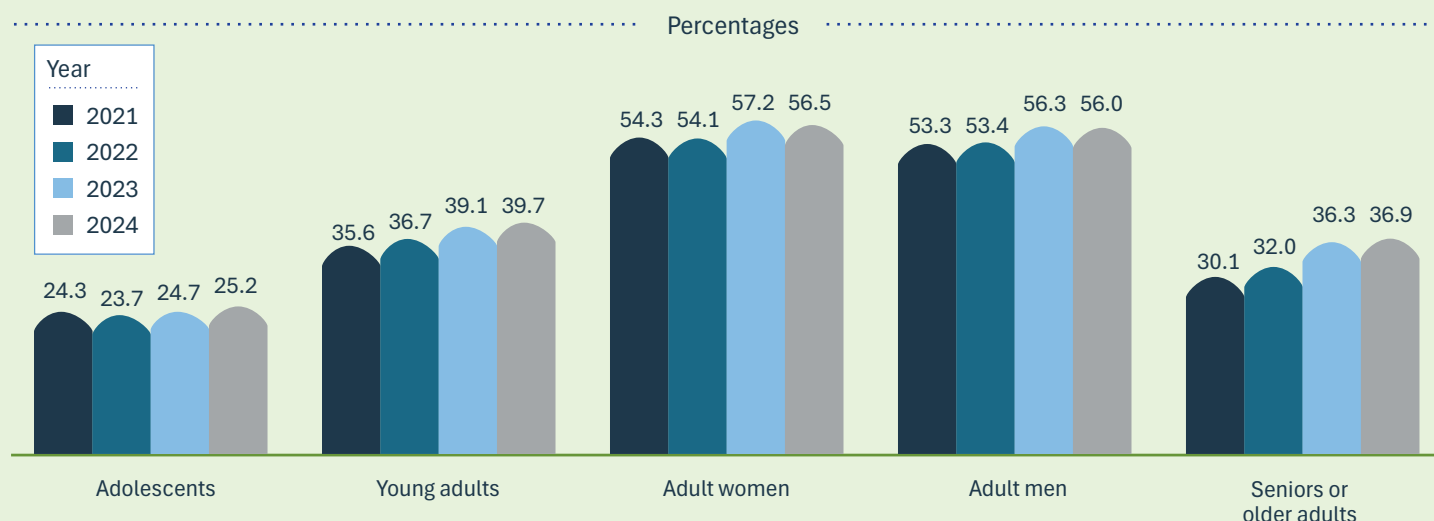
Figure 3.2.3: Combined Substance Use and Mental Health Treatment Facilities by Operation Type, 2021–2024 N-SUMHSS



Combined SU/MH treatment facilities were operated predominantly by private entities. In 2021, private non-profit organizations operated 63.4% (n = 1,546) of combined SU/MH facilities and private for-profit organizations operated 22.1% (n = 538) of combined SU/MH treatment facilities. In 2024, private non-profit organizations operated 53.6.% (n = 4,740) of combined SU/MH treatment facilities and private for-profit organizations operated 33.9% (n = 2,993) of combined SU/MH treatment facilities. [2.4.7.8](#)

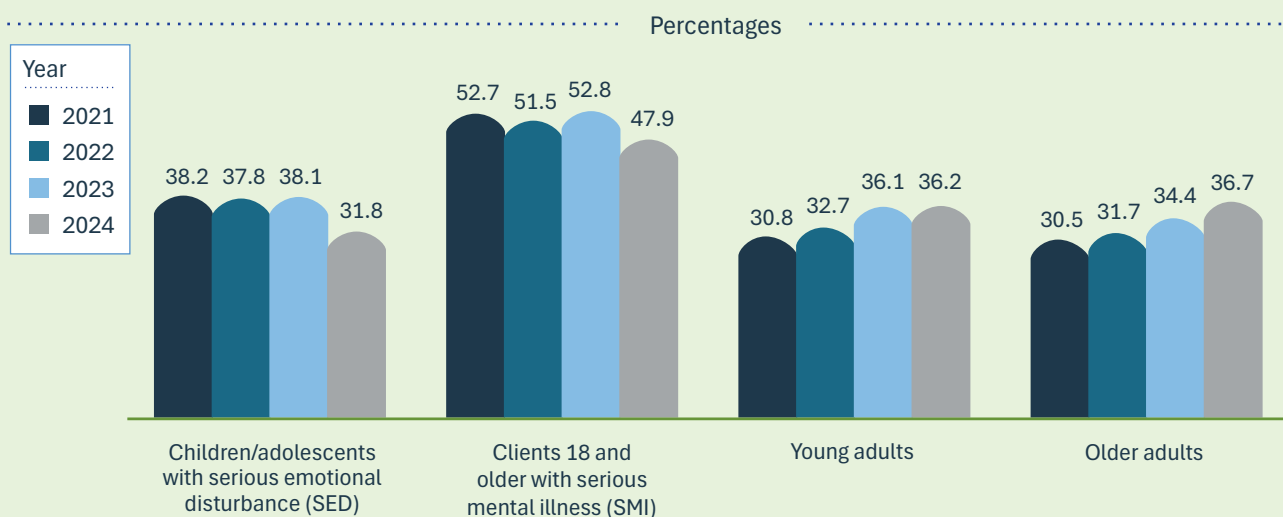
3.3: Substance Use and Mental Health Treatment Facilities Offering Tailored Programs for Specific Age Groups, 2021–2024 N-SUMHSS

Figure 3.3.1: Substance Use Treatment Facilities Offering Tailored Programs for Specific Age Groups, 2021–2024 N-SUMHSS



In 2021, among SU treatment facilities offering tailored programs for specific age groups, 24.3% (n = 3,400) offered programs for adolescents, 35.6% (n = 4,983) for young adults, 54.3% (n = 7,610) for adult women, 53.3% (n = 7,471) for adult men, and 30.1% (n = 4,222) for seniors or older adults. In 2024, 25.2% (n = 4,026) offered programs for adolescents, 39.7% (n = 6,329) for young adults, 56.5% (n = 9,013) for adult women, 56.0% (n = 8,930) for adult men, and 36.9% (n = 5,892) for seniors or older adults. [2,4,8,17](#)

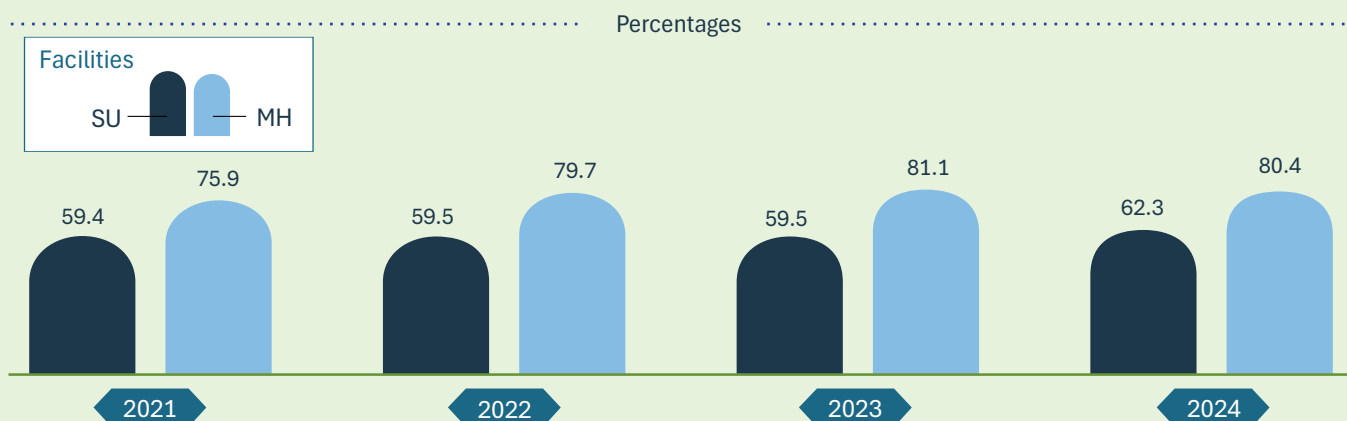
Figure 3.3.2: Mental Health Treatment Facilities Offering Tailored Programs for Specific Age Groups, 2021–2024 N-SUMHSS



In 2021, among MH treatment facilities offering tailored programs for specific age groups, 38.2% (n = 3,461) offered programs for children/adolescents with SED, 52.7% (n = 4,773) for clients 18 and older with SMI, 30.8% (n = 2,791) for young adults, and 30.5% (n = 2,758) for older adults. In 2024, among MH treatment facilities offering tailored programs for specific age groups, 31.8% (n = 4,484) offered programs for children/adolescents with SED, 47.9% (n = 6,748) for clients 18 and older with SMI, 36.2% (n = 5,099) for young adults, and 36.7% (n = 5,165) for older adults. [2,4,8,17](#)

3.4: Substance Use and Mental Health Treatment Facilities Offering Treatment for Co-occurring Disorders Plus Either Serious Mental Illness (SMI) in Adults and/or Serious Emotional Disturbance (SED) in Children, 2021–2024 N-SUMHSS

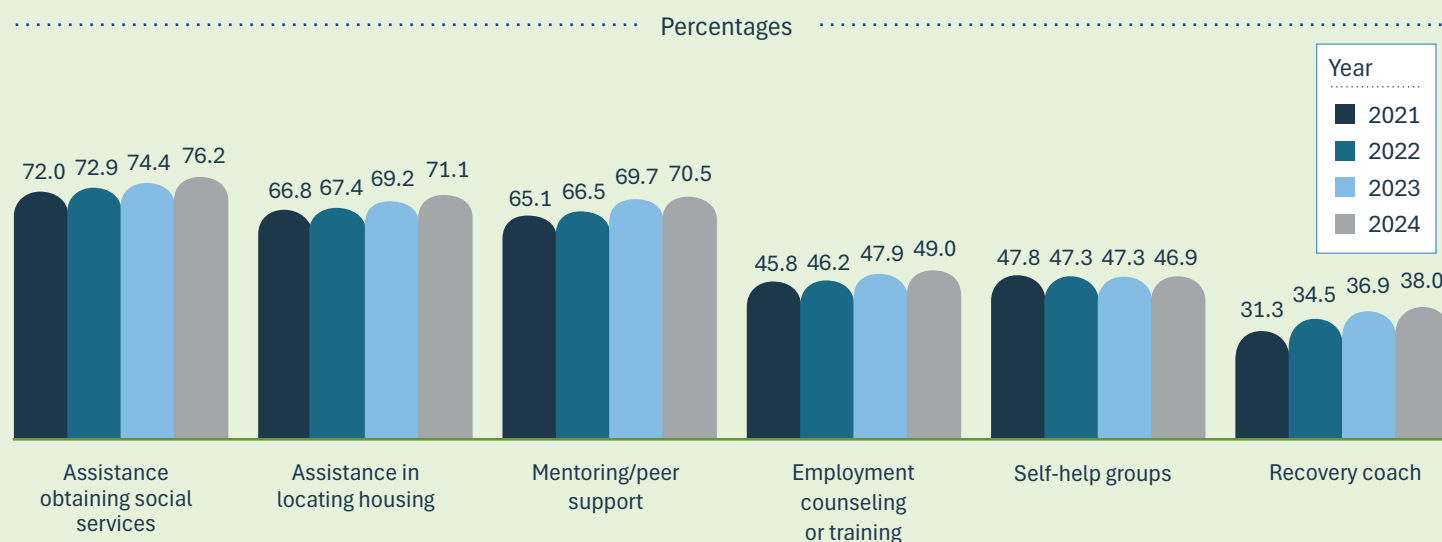
Figure 3.4: Substance Use and Mental Health Treatment Facilities Offering Treatment for Co-occurring Disorders Plus Either SMI in Adults and/or SED in Children, 2021–2024 N-SUMHSS



In 2021, 59.4% (n = 8,320) of SU treatment facilities and 75.9% (n = 6,869) of MH treatment facilities offered treatment for co-occurring disorders plus either SMI in adults and/or SED in children. In 2024, 62.3% (n = 9,944) of SU treatment facilities and 80.4% (n = 11,334) of MH treatment facilities offered treatment for co-occurring disorders plus either SMI in adults and/or SED in children.^{2,4,8,16}

3.5: Substance Use Treatment Facilities Offering Recovery Support Services, 2021–2024 N-SUMHSS

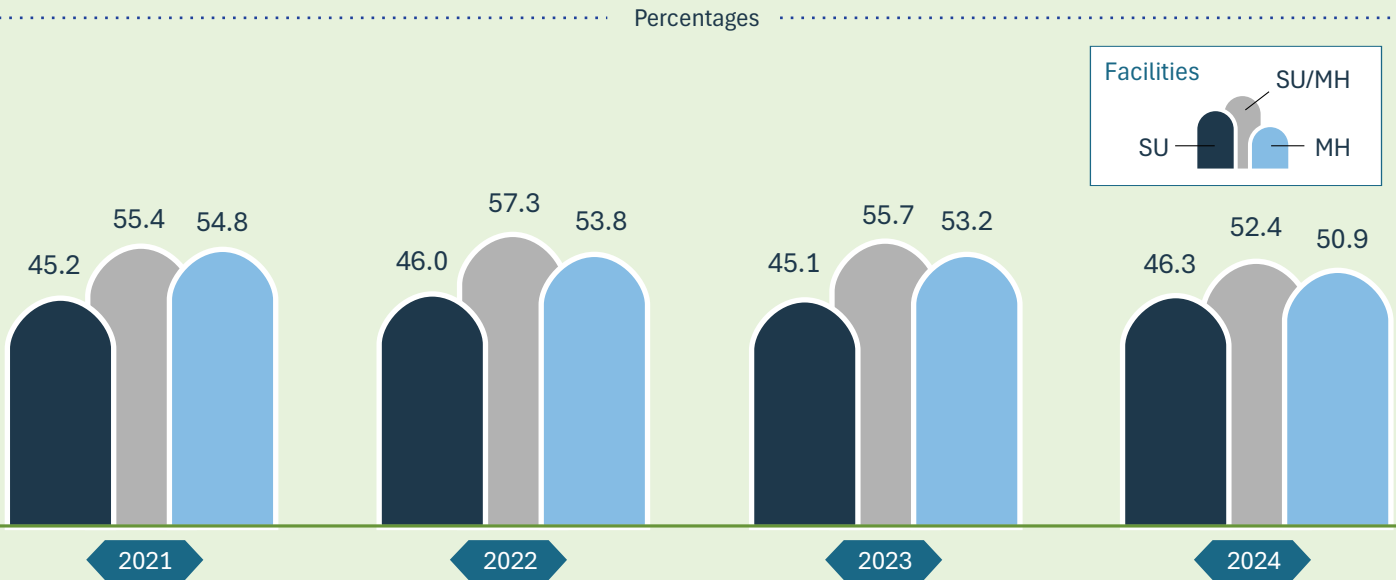
Figure 3.5: Substance Use Treatment Facilities Offering Recovery Support Services, 2021–2024 N-SUMHSS



In 2021, among SU treatment facilities offering recovery support services, 72.0% (n = 10,089) offered assistance obtaining social services, 66.8% (n = 9,359) assistance in locating housing, 65.1% (n = 9,127) offered mentoring/peer support, 47.8% (n = 6,697) offered self-help groups, 45.8% (n = 6,423) offered employment counseling or training, 31.3% (n = 4,392) offered recovery coaching, and 8.1% (n = 1,138) offered none of the recovery support services listed at their facility. In 2024, among SU treatment facilities offering recovery support services, 76.2% (n = 12,151) offered assistance obtaining social services, 71.1% (n = 11,335) offered assistance in locating housing, 70.5% (n = 11,254) offered mentoring/peer support, 49.0% (n = 7,813) offered employment counseling or training, 46.9% (n = 7,481) offered self-help groups, 38.0% (n = 6,069) offered recovery coaching, and 6.6% (n = 1,058) offered none of the recovery support services listed at their facility.^{2,4,8}

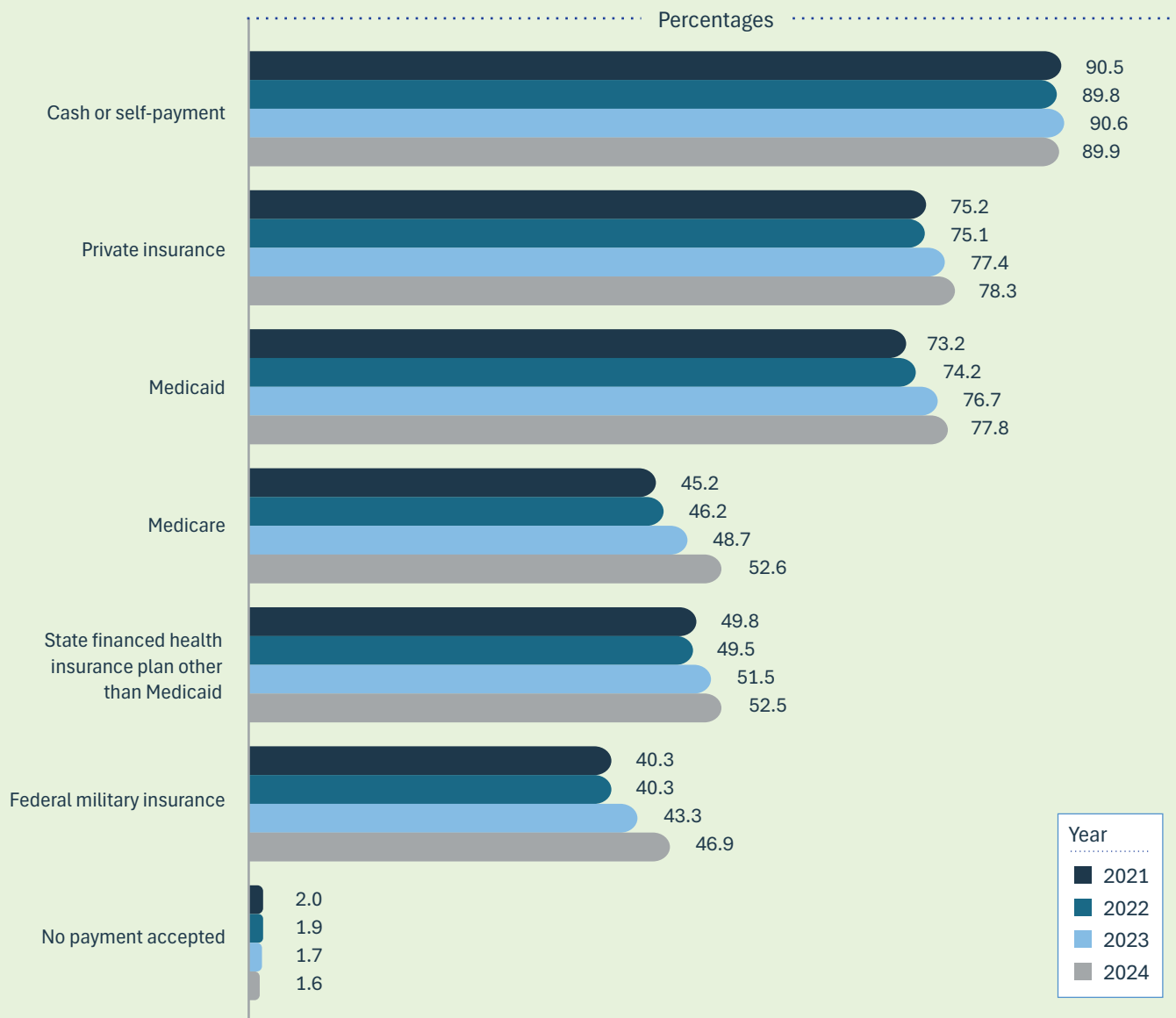
3.6: Substance Use and Mental Health Treatment Facilities by Types of Client Payments or Insurance Accepted, 2021–2024 N-SUMHSS

Figure 3.6.1: Substance Use and Mental Health Treatment Facilities Offering Treatment at No Charge or Minimal Payment to Clients who Cannot Afford to Pay, 2021–2024 N-SUMHSS



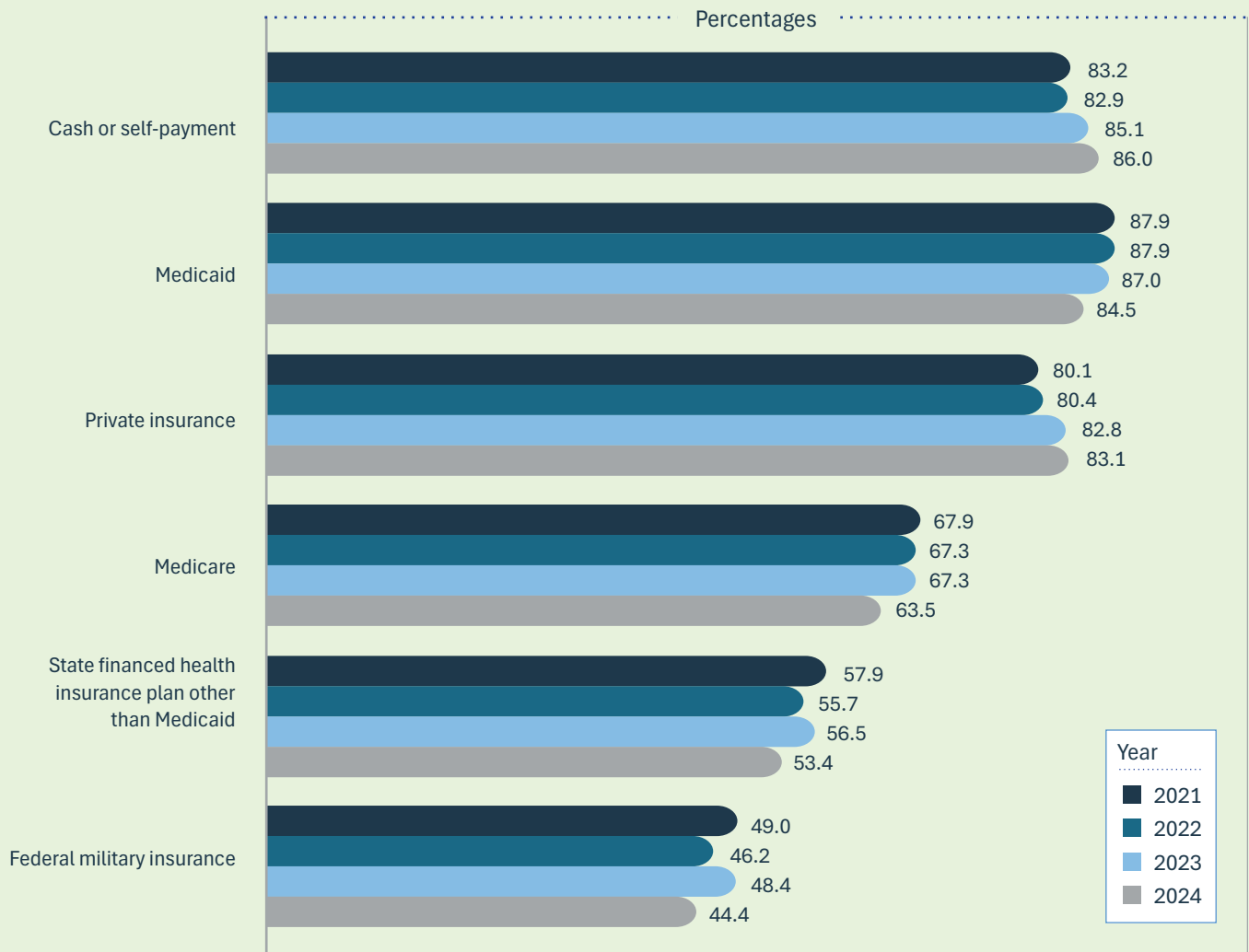
In 2021, 45.2% (n = 6,334) of SU treatment facilities, 54.8% (n = 4,958) of MH treatment facilities, and 55.4% (n = 1,350) of combined SU and MH treatment facilities offered treatment at no charge or at minimal payment to clients who could not afford to pay. In 2024, 46.3% (n = 7,383) of SU treatment facilities, 50.9% (n = 7,167) of MH treatment facilities, and 52.4% (n = 4,630) of combined SU/MH treatment facilities offered treatment at no charge or at minimal payment to clients who could not afford to pay.^{2,4,8}

Figure 3.6.2: Substance Use Treatment Facilities by Types of Client Payments or Insurance Accepted, 2021–2024 N-SUMHSS



In 2021, 2.0% (n = 286) of SU facilities did not accept any form of payment. Among SU treatment facilities accepting client payments or insurance, 90.5% (n = 12,680) accepted cash or self-payment, 75.2% (n = 10,539) accepted private insurance, 73.2% (n = 10,259) accepted Medicaid, 49.8% (n = 6,971) accepted state-financed health insurance plans other than Medicaid, 45.2% (n = 6,334) accepted Medicare, and 40.3% (n = 5,641) accepted federal military insurance. In 2024, 1.6% (n = 253) of SU facilities did not accept any form of payment. Among SU treatment facilities accepting client payments or insurance, 89.9% (n = 14,337) accepted cash or self-payment, 78.3% (n = 12,494) accepted private insurance, 77.8% (n = 12,418) accepted Medicaid, 52.6% (n = 8,397) accepted Medicare, 52.5% (n = 8,380) accepted state-financed health insurance plans other than Medicaid, and 46.9% (n = 7,484) accepted federal military insurance.^{2.4.8}

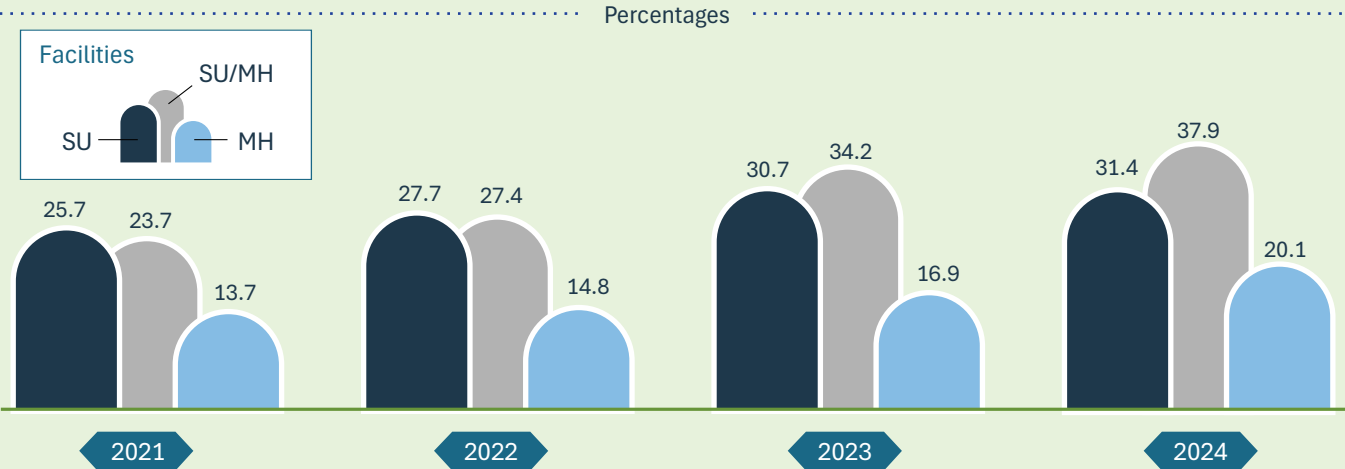
Figure 3.6.3: Mental Health Treatment Facilities by Types of Client Payments or Insurance Accepted, 2021–2024 N-SUMHSS



In 2021, among MH treatment facilities accepting client payments or insurance, 87.9% (n = 7,951) accepted Medicaid, 83.2% (n = 7,529) cash or self-payment, 80.1% (n = 7,247) private insurance, 67.9% (n = 6,143) Medicare, 57.9% (n = 5,242) state-financed health insurance plans other than Medicaid, and 49.0% (n = 4,435) federal military insurance. In 2024, among MH treatment facilities accepting client payments or insurance, 86.0% (n = 12,114) accepted cash or self-payment, 84.5% (n = 11,904) accepted Medicaid, 83.1% (n = 11,710) accepted private insurance, 63.5% (n = 8,953) accepted Medicare, 53.4% (n = 7,523) accepted state-financed health insurance plans other than Medicaid, and 44.4% (n = 6,255) accepted federal military insurance.^{2,4,8}

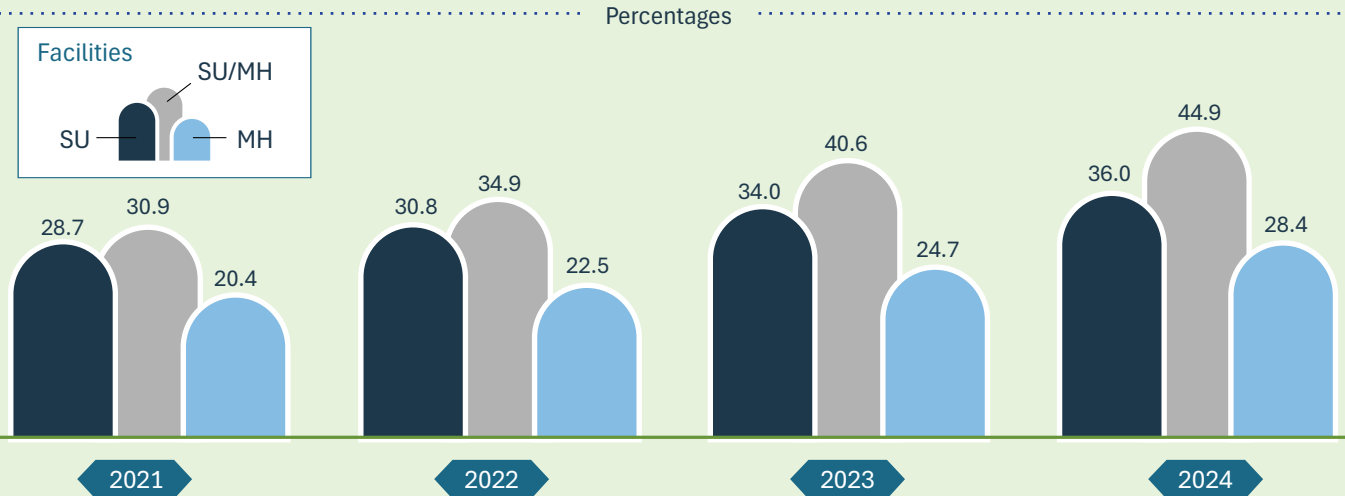
3.7: Substance Use and Mental Health Treatment Facilities Offering Tailored Programs for Specific Groups, 2021–2024
N-SUMHSS

Figure 3.7.1: Substance Use and Mental Health Treatment Facilities Offering Tailored Programs for Clients with HIV or AIDS, 2021–2024 N-SUMHSS



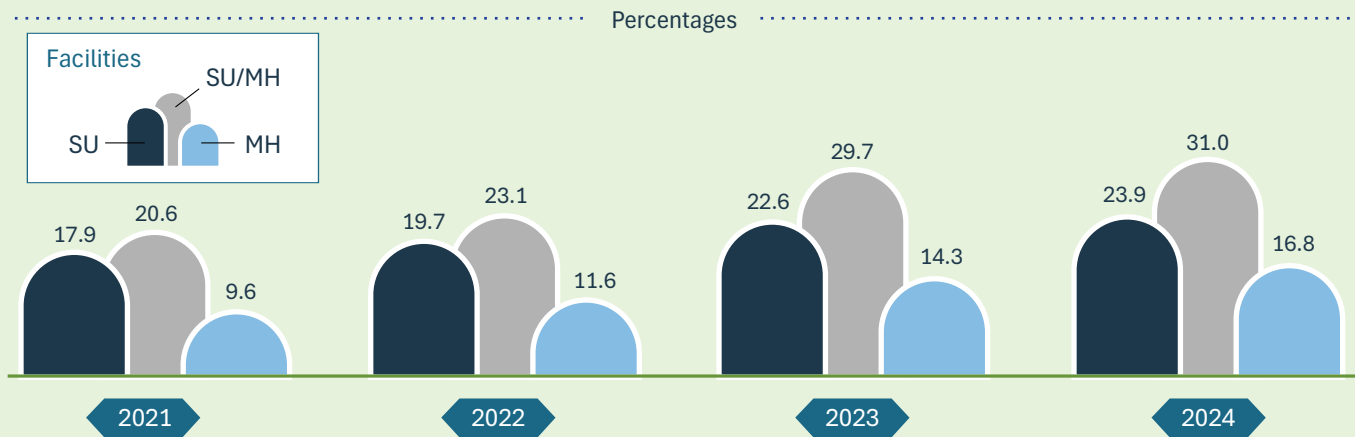
In 2021, 25.7% (n = 3,604) of SU treatment facilities, 13.7% (n = 1,243) of MH treatment facilities, and 23.7% (n = 578) of combined SU/MH treatment facilities offered tailored programs for clients with HIV or AIDS. In 2024, 31.4% (n = 5,017) of SU treatment facilities, 20.1% (n = 2,830) of MH treatment facilities, and 37.9% (n = 3,351) of combined SU/MH treatment facilities offered tailored programs for clients with HIV or AIDS.^{2,4,8}

Figure 3.7.2: Substance Use and Mental Health Treatment Facilities Offering Tailored Programs for Veterans, 2021–2024 N-SUMHSS



In 2021, 28.7% (n = 4,023) of SU treatment facilities, 20.4% (n = 1,842) of MH treatment facilities, and 30.9% (n = 754) of combined SU/MH treatment facilities offered tailored programs for veterans. In 2024, 36.0% (n = 5,740) of SU treatment facilities, 28.4% (n = 4,001) of MH treatment facilities, and 44.9% (n = 3,967) of combined SU/MH treatment facilities offered tailored programs for veterans.^{2,4,8}

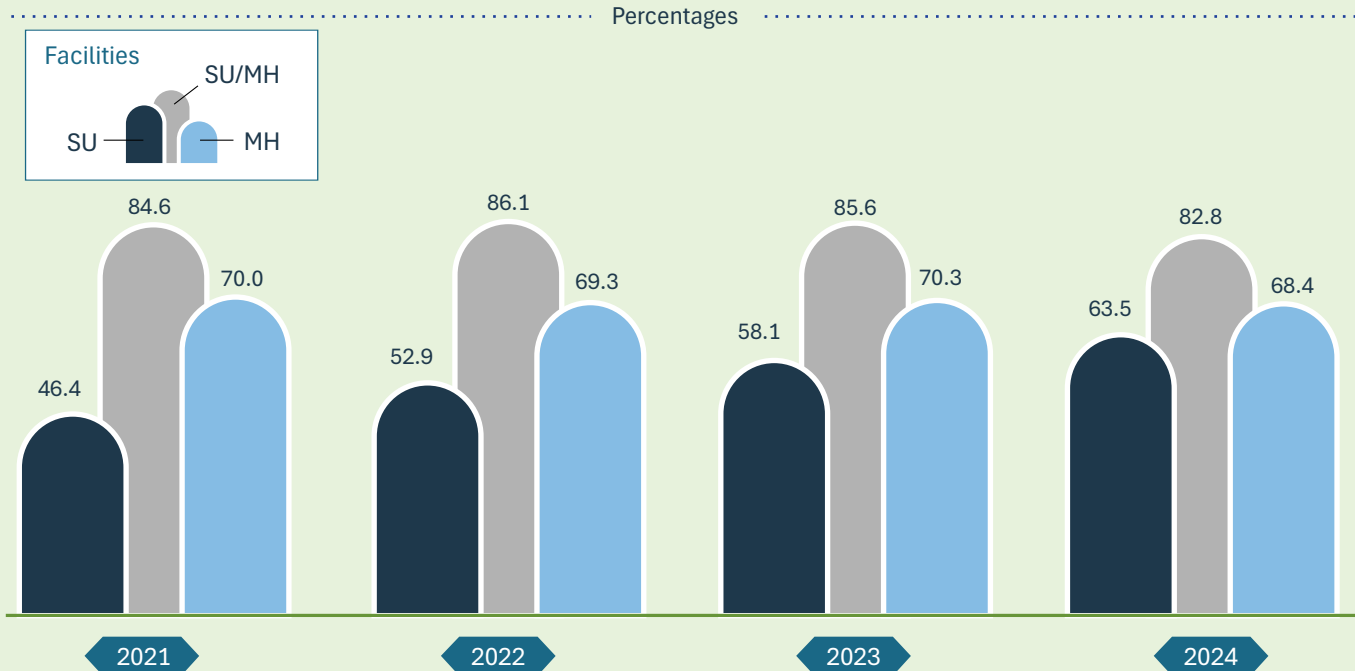
Figure 3.7.3: Substance Use and Mental Health Treatment Facilities Offering Tailored Programs for Active-duty Military, 2021–2024 N-SUMHSS



In 2021, 17.9% (n = 2,507) of SU treatment facilities, 9.6% (n = 873) of MH treatment facilities, and 20.6% (n = 503) of combined SU/MH treatment facilities offered tailored programs for active-duty military. In 2024, 23.9% (n = 3,811) of SU treatment facilities, 16.8% (n = 2,370) of MH treatment facilities, and 31.0% (n = 2,744) of combined SU/MH treatment facilities offered tailored programs for active-duty military.^{2,4,8}

3.8: Substance Use and Mental Health Treatment Facilities Offering Suicide Prevention Services, 2021–2024 N-SUMHSS

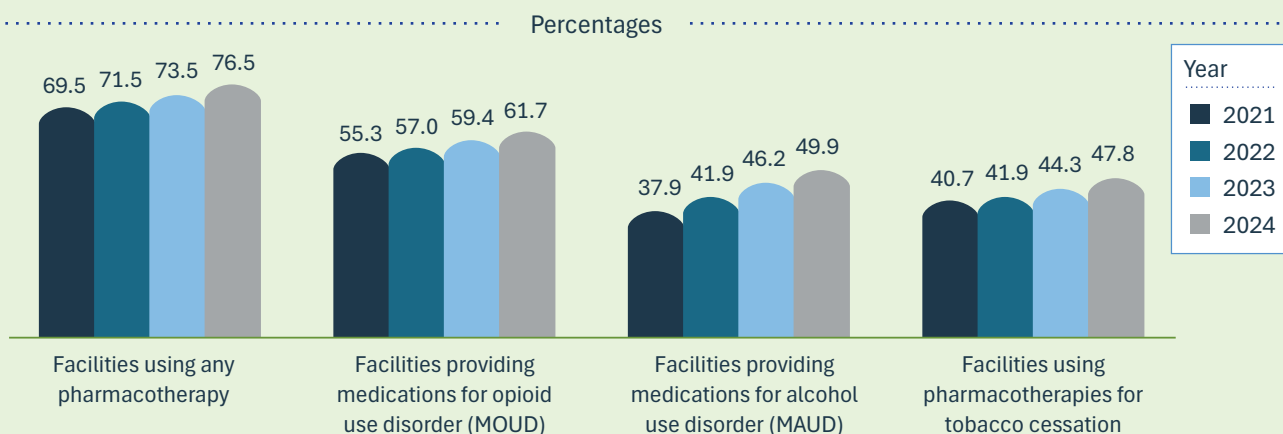
Figure 3.8: Substance Use and Mental Health Treatment Facilities Offering Suicide Prevention Services, 2021–2024 N-SUMHSS



In 2021, 46.4% (n = 6,503) of SU treatment facilities, 70.0% (n = 6,332) of MH treatment facilities, and 84.6% (n = 2,062) of combined SU/MH treatment facilities offered suicide prevention services. In 2024, 63.5% (n = 10,131) of SU treatment facilities, 68.4% (n = 9,645) of MH treatment facilities, and 82.8% (n = 7,317) of combined SU/MH treatment facilities offered suicide prevention services.^{2,4,8}

3.9: Substance Use Treatment Facilities Utilizing Pharmacotherapies, 2021–2024 N-SUMHSS

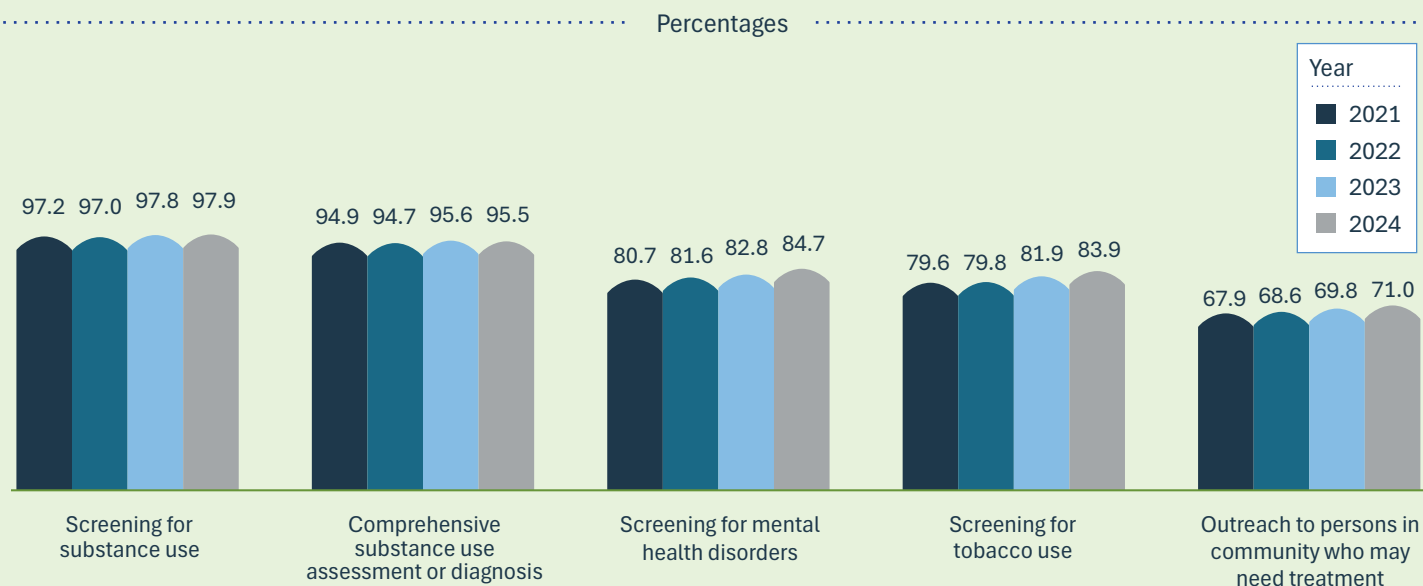
Figure 3.9: Substance Use Treatment Facilities Utilizing Pharmacotherapies, 2021–2024 N-SUMHSS



In 2021, among SU treatment facilities utilizing pharmacotherapies, 69.5% (n = 9,736) reported using any pharmacotherapy, 55.3% (n = 7,753) provided MOUD, 37.9% (n = 5,312) provided MAUD, and 40.7% (n = 5,705) used pharmacotherapies for tobacco cessation. In 2024, among SU treatment facilities utilizing pharmacotherapies, 76.5% (n = 12,207) reported using any pharmacotherapy, 61.7% (n = 9,842) provided MOUD, 49.9% (n = 7,953) provided MAUD, and 47.8% (n = 7,628) used pharmacotherapies for tobacco cessation.^{2,4,8,10,11,12,13}

3.10: Substance Use Treatment Facilities Offering Assessment and Pre-treatment Services, 2021–2024 N-SUMHSS

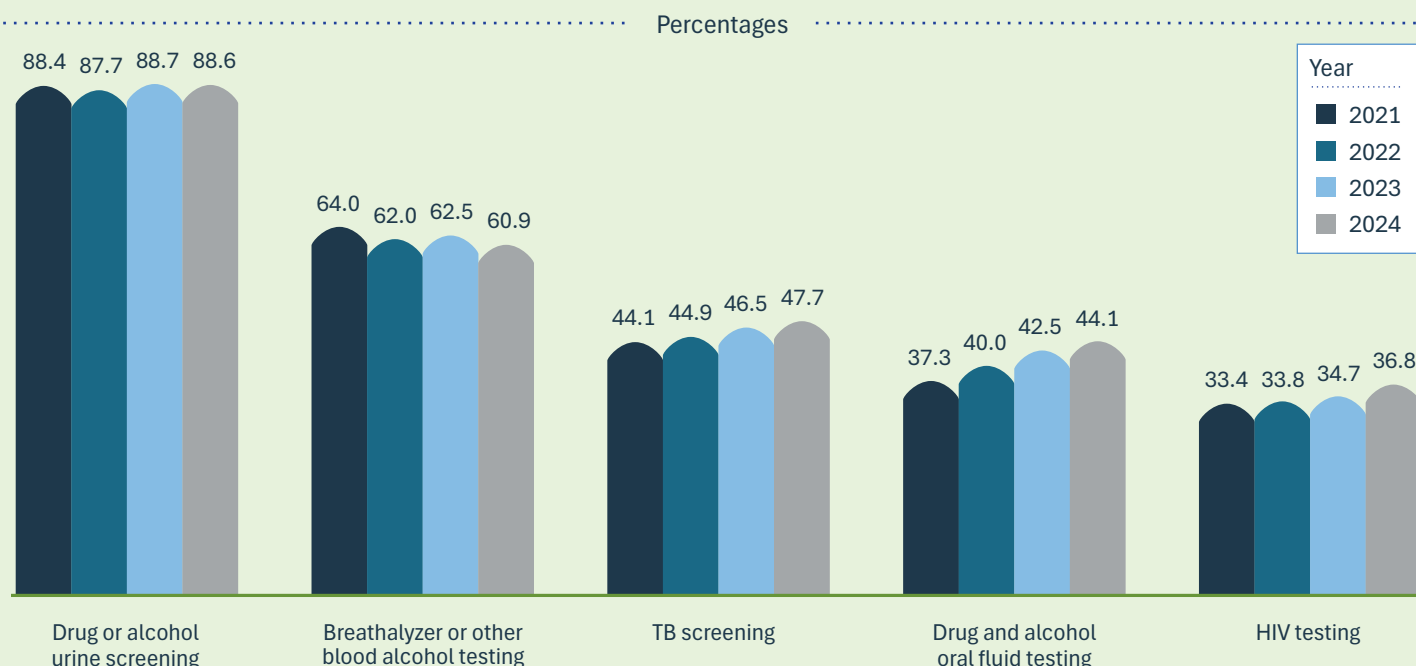
Figure 3.10: Substance Use Treatment Facilities Offering Assessment and Pre-treatment Services (Top Five), 2021–2024 N-SUMHSS



In 2021, among SU treatment facilities offering assessment and pre-treatment services, the top five most offered assessment and pre-treatment services were screening for substance use (97.2%, n = 13,615), comprehensive substance use assessment or diagnosis (94.9%, n = 13,302), screening for mental health disorders (80.7%, n = 11,300), screening for tobacco use (79.6%, n = 11,149), and outreach to persons in the community who may need treatment (67.9%, n = 9,513). In 2024, the top five most offered testing services were screening for substance use (97.9%, n = 15,623), comprehensive substance use assessment or diagnosis (95.5%, n = 15,239), screening for mental health disorders (84.7%, n = 13,519), screening for tobacco use (83.9%, n = 13,384), and outreach to persons in the community who may need treatment (71.0%, n = 11,330).^{2,4,8}

3.11: Substance Use Treatment Facilities Offering Testing Services, 2021–2024 N-SUMHSS

Figure 3.11: Substance Use Treatment Facilities Offering Testing Services (Top Five), 2021–2024 N-SUMHSS

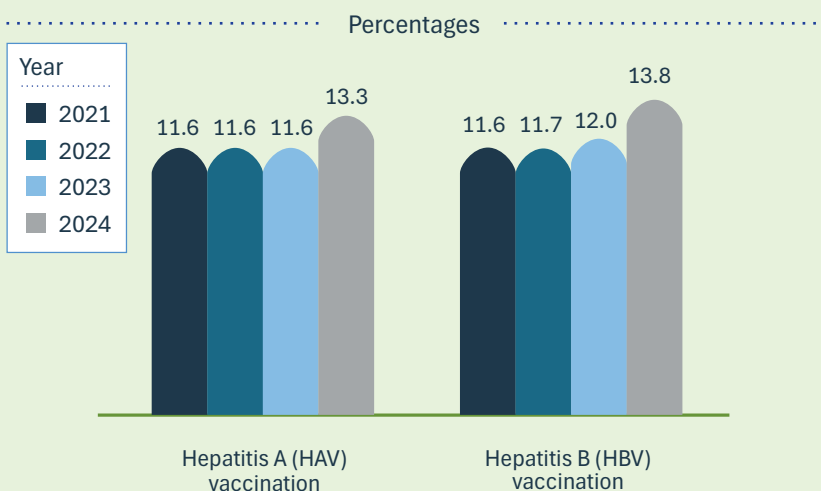


In 2021, among SU treatment facilities offering testing services, the top five most offered testing services were drug or alcohol urine screening (88.4%, n = 12,384), breathalyzer or other blood alcohol testing (64.0%, n = 8,960), tuberculosis (TB) screening (44.1%, n = 6,175), drug and alcohol oral fluid testing (37.3%, n = 5,226), and HIV testing (33.4%, n = 4,685). In 2024, the top five most offered testing services were drug or alcohol urine screening (88.6%, n = 14,134), breathalyzer or other blood alcohol testing (60.9%, n = 9,719), TB screening (47.7%, n = 7,607), drug and alcohol oral fluid testing (44.1%, n = 7,029), and HIV testing (36.8%, n = 5,874).^{2.4.8}

3.12: Substance Use Treatment Facilities Offering Medical Services, 2021–2024 N-SUMHSS

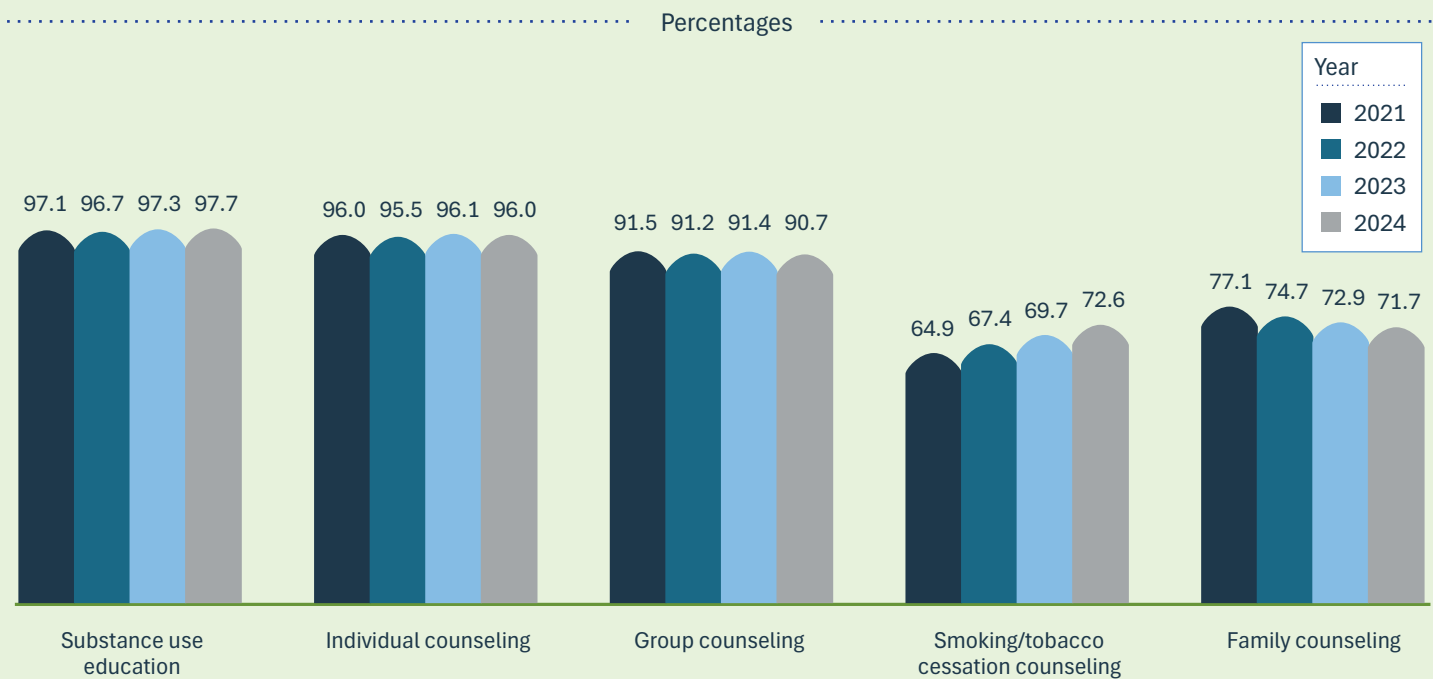
In 2021, among SU treatment facilities offering medical services 11.6% (n = 1,621) offered HAV, 11.6% (n = 1,624) offered HBV, and 69.3% (n = 9,708) offered none of the services above. In 2024, among SU treatment facilities offering medical services 13.3% (n = 2,118) offered HAV, 13.8% (n = 2,207) offered HBV, and 71.4% (n = 11,398) offered none of the services above.^{2.4.8}

Figure 3.12: Substance Use Treatment Facilities Offering Medical Services, 2021–2024 N-SUMHSS



3.13: Substance Use Treatment Facilities Offering Education and Counseling Services, 2021–2024 N-SUMHSS

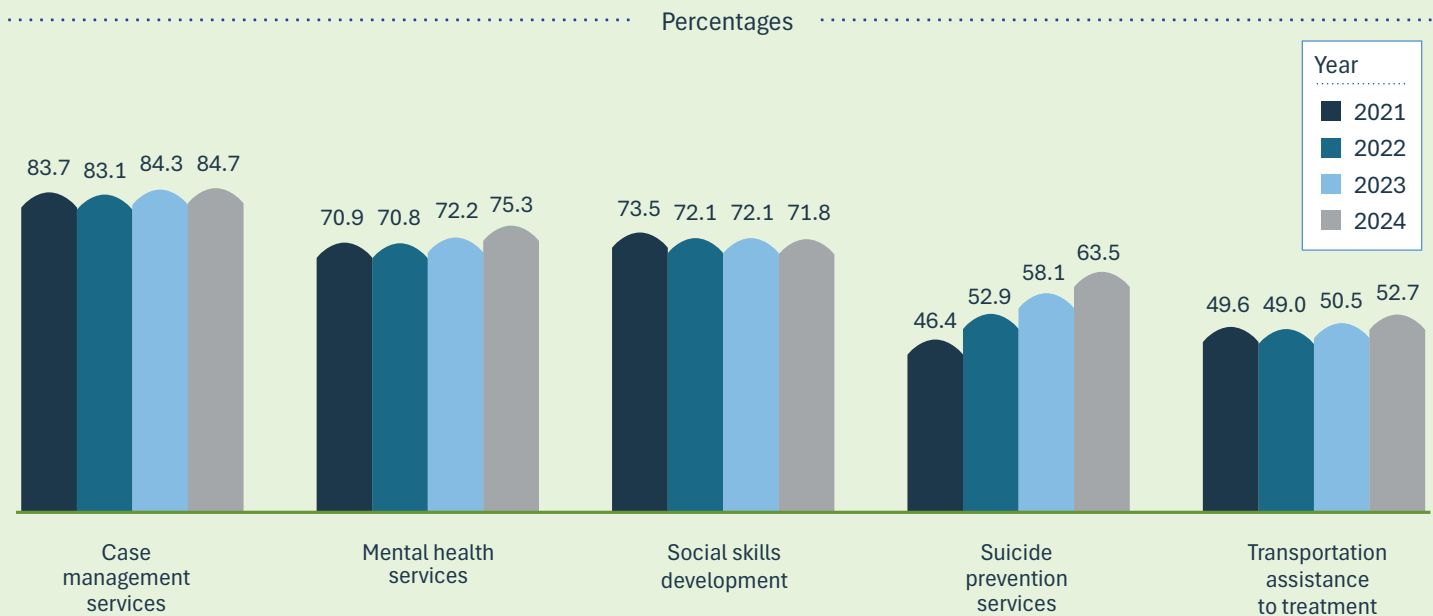
Figure 3.13: Substance Use Treatment Facilities Offering Education and Counseling Services, (Top Five) 2021–2024 N-SUMHSS



In 2021, among SU treatment facilities offering education and counseling services, the top five most offered services were substance use education (97.1%, n = 13,601), individual counseling (96.0%, n = 13,456), group counseling (91.5%, n = 12,816), family counseling (77.1%, n = 10,803), and smoking/tobacco cessation counseling (64.9%, n = 9,093). In 2024, the top five most offered services were substance use education (97.7%, n = 15,587), individual counseling (96.0%, n = 15,313), group counseling (90.7%, n = 14,469), smoking/tobacco cessation counseling (72.6%, n = 11,582), and family counseling (71.7%, n = 11,442).^{2,4,8}

3.14: Substance Use Treatment Facilities Offering Ancillary Services, 2021–2024 N-SUMHSS

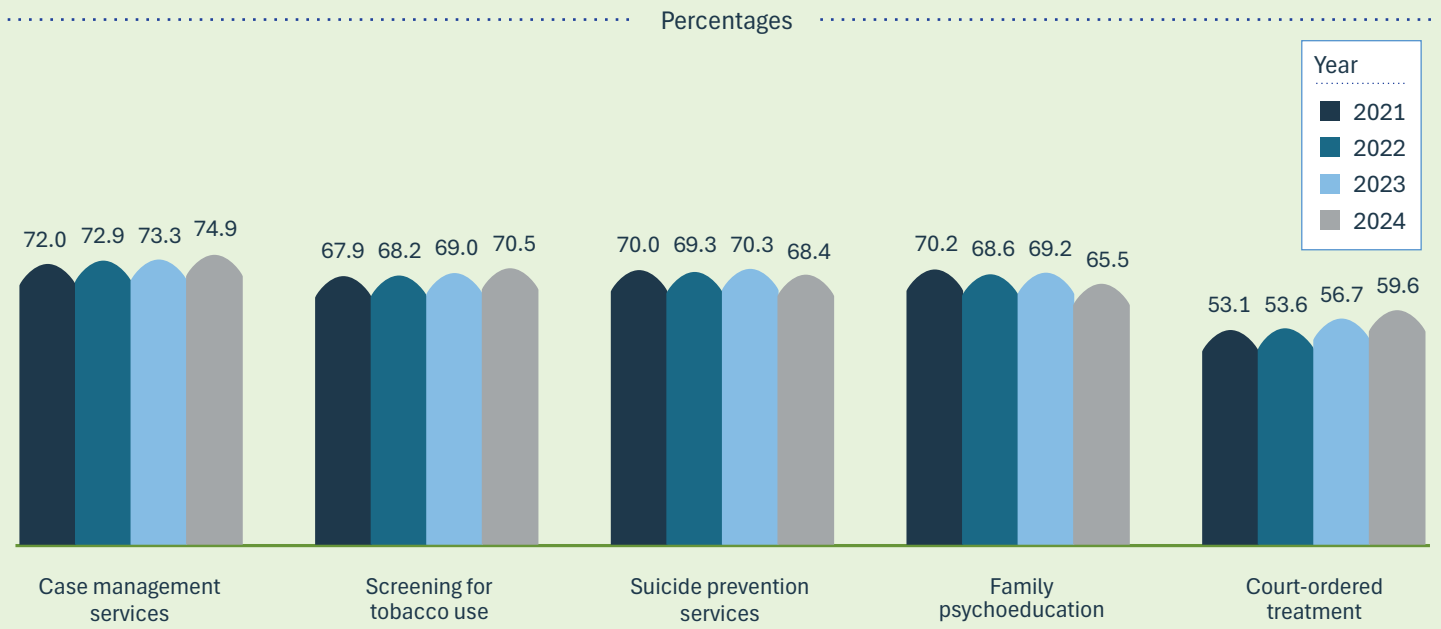
Figure 3.14: Substance Use Treatment Facilities Offering Ancillary Services (Top Five), 2021–2024 N-SUMHSS



In 2021, among SU treatment facilities offering ancillary services, the top five most offered services were CM services (83.7%, n = 11,727), social skills development (73.5%, n = 10,293), mental health services (70.9%, n = 9,931), transportation assistance to treatment (49.6%, n = 6,946), and suicide prevention services (46.4%, n = 6,503). In 2024, the top five most offered ancillary services were CM services (84.7%, n = 13,515), mental health services (75.3%, n = 12,019), social skills development (71.8%, n = 11,460), suicide prevention services (63.5%, n = 10,131), and transportation assistance to treatment (52.7%, n = 8,406).^{2,4,8}

3.15: Mental Health Treatment Facilities Offering Services and Practices, 2021–2024 N-SUMHSS

Figure 3.15: Mental Health Treatment Facilities Offering Services and Practices (Top Five), 2021–2024 N-SUMHSS



In 2021, among MH treatment facilities offering services and practices, the top five most offered services were CM (72.0%, n = 6,512), family psychoeducation (70.2%, n = 6,351), suicide prevention services (70.0%, n = 6,332), screening for tobacco use (67.9%, n = 6,144), and court-ordered treatment (53.1%, n = 4,806). In 2024, the top five most offered services and practices were CM (74.9%, n = 10,561), screening for tobacco use (70.5%, n = 9,937), suicide prevention services (68.4%, n = 9,645), family psychoeducation (65.5%, n = 9,235), and court-ordered treatment (59.6%, n = 8,401).^{2,4,8}

Section 4 | Substance Use and Mental Health Treatment Services by Geographic Distribution, 2024 N-SUMHSS

This section presents geographical distributions of data on substance use and mental health treatment services by facilities using MOUD and MAUD, treating opioid use disorder (OUD), providing medication services for OUD, treating alcohol use disorder (AUD), using MOUD and MAUD to treat clients with co-occurring mental and substance use disorders, and providing suicide prevention services. The maps presented in this section present data by manually sorting states, D.C., and Other jurisdictions/territories from high to low on percentages and selecting a cut-off value that corresponds to the 10th-highest percentage. In this section, Other jurisdictions/territories combines the following U.S. territories: Guam, Northern Mariana Islands, and the U.S. Virgin Islands. For data tables corresponding to the maps presented in this section, please refer to Appendix A.

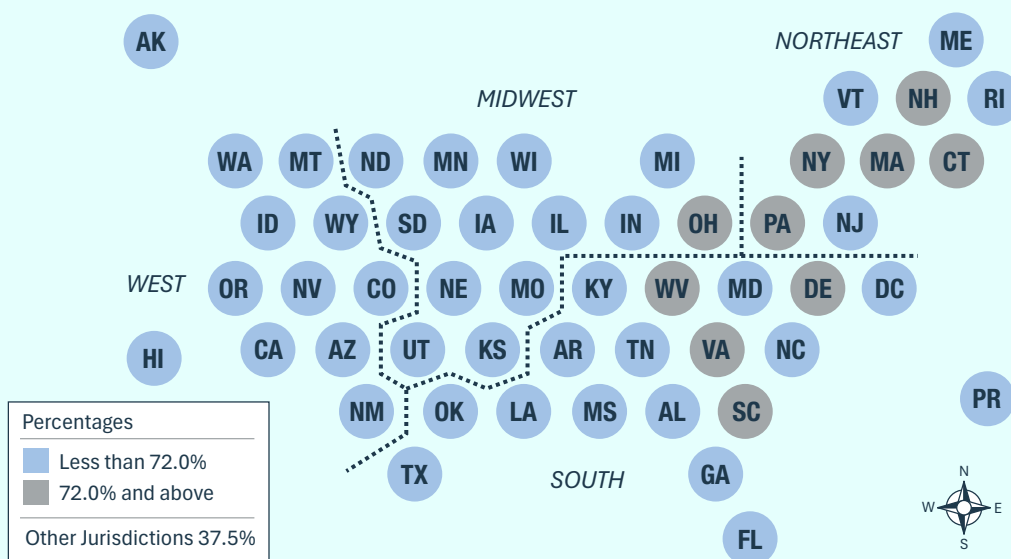
Key Takeaways

- Among SU treatment facilities, 61.7% (n = 9,842) utilized MOUD and 49.9% (n = 7,953) utilized MAUD. [2.4.8.11.12](#)
- Among SU treatment facilities, 61.7% (n = 9,842) utilized MOUD. Of the top 10 geographic distributions, more than 72.0% of SU treatment facilities utilized MOUD: New York (87.8%, n = 701), West Virginia (78.8%, n = 123), New Hampshire (77.7%, n = 73), Connecticut (77.6%, n = 149), Virginia (77.1%, n = 289), Pennsylvania (75.4%, n = 441), South Carolina (75.0%, n = 78), Ohio (74.4%, n = 548), Delaware (73.8%, n = 45), and Massachusetts (72.0%, n = 309). [2.4.8.11](#)
- Among SU treatment facilities, 49.9% (n = 7,953) utilized MAUD. Of the top 10 geographic distributions, more than 60.5% of SU treatment facilities utilized MAUD: New York (80.2%, n = 640), Connecticut (75.0%, n = 144), New Hampshire (68.1%, n = 64), Delaware (67.2%, n = 41), Missouri (66.3%, n = 191), Ohio (64.3%, n = 474), Virginia (62.9%, n = 236), Massachusetts (62.2%, n = 267), West Virginia (60.9%, n = 95), and Kentucky (60.5%, n = 357). [2.4.8.12](#)
- Among SU treatment facilities, 60.1% (n = 9,595) accepted clients using MOUD. Of the top 10 geographic distributions, more than 68.2% of SU treatment facilities accepted clients using MOUD: Montana (83.8%, n = 83), Alaska (80.0%, n = 72), Iowa (79.0%, n = 154), Wyoming (77.8%, n = 42), Idaho (76.9%, n = 83), Minnesota (76.6%, n = 291), North Dakota (73.9%, n = 51), Oregon (70.2%, n = 174), California (69.1%, n = 1,033), and Washington (68.2%, n = 257). [2.4.8.11](#)
- Among SU treatment facilities providing medication services for OUD, 46.3% (n = 7,390) provided maintenance with methadone or buprenorphine. Of the top 10 geographic distributions, more than 60.3% of SU treatment facilities provided maintenance with methadone or buprenorphine: New York (71.1%, n = 567), Delaware (70.5%, n = 43), New Hampshire (66.0%, n = 62), West Virginia (66.0%, n = 103), Virginia (63.5%, n = 238), Connecticut (61.5%, n = 118), South Carolina (61.5%, n = 64), Vermont (60.7%, n = 34), Kentucky (60.7%, n = 358), and Pennsylvania (60.3%, n = 353). [2.4.8.11](#)
- Among SU treatment facilities, 57.3% (n = 9,135) accepted clients using MAUD. Of the top 10 geographic distributions, more than 67.6% of SU treatment facilities accepted clients using MAUD: Montana (81.8%, n = 81), Alaska (81.1%, n = 73), Iowa (78.5%, n = 153), North Dakota (73.9%, n = 51), Minnesota (72.6%, n = 276), Idaho (71.3%, n = 77), Nevada (69.7%, n = 92), Wyoming (68.5%, n = 37), New Jersey (67.6%, n = 279), and Michigan (67.6%, n = 305). [2.4.8.12](#)
- Among MH treatment facilities using medications to treat clients with co-occurring mental and substance use disorders, 39.8% (n = 5,603) used MOUD. Of the top 10 geographic distributions, more than 49.5% of MH treatment facilities used MOUD: Delaware (61.7%, n = 29), West Virginia (60.6%, n = 83), Oklahoma (54.8%, n = 86), Virginia (54.0%, n = 168), Missouri (53.8%, n = 148), New Hampshire (53.4%, n = 39), Kansas (51.9%, n = 69), New York (51.1%, n = 405), Other jurisdictions (50.0%, n = 3), and Colorado (49.5%, n = 108). [2.4.8.11](#)

- Among MH treatment facilities using medications to treat clients with co-occurring mental and substance use disorders, 33.9% (n = 4,775) used MAUD. Of the top 10 geographic distributions, more than 42.9% of MH treatment facilities used MAUD: Delaware (61.7%, n = 29), West Virginia (52.6%, n = 72), Connecticut (49.8%, n = 121), New Hampshire (49.3%, n = 36), Missouri (48.4%, n = 133), Wyoming (46.9%, n = 23), Kansas (45.9%, n = 61), Massachusetts (44.0%, n = 143), Colorado (43.6%, n = 95), and New York (42.9%, n = 340). [2.4.8.12](#)
- Among SU treatment facilities, 63.5% (n = 10,131) provided suicide prevention services. Of the top 10 geographic distributions, more than 76.5% of SU treatment facilities provided suicide prevention services: Wyoming (88.9%, n = 48), Other jurisdictions (87.5% n = 7), Arkansas (82.6%, n = 128), Puerto Rico (81.7%, n = 58), Alaska (80.0%, n = 72), Missouri (79.2%, n = 228), Montana (78.8%, n = 78), Kentucky (78.0%, n = 460), Oklahoma (76.8%, n = 136), and Nevada (76.5%, n = 101). [2.4.8](#)
- Among MH treatment facilities, 68.4% (n = 9,645) provided suicide prevention services. Of the top 10 geographic distributions, more than 79.3% of MH treatment facilities provided suicide prevention services: Oklahoma (89.8%, n = 141), South Dakota (89.1%, n = 41), Wyoming (87.8%, n = 43), South Carolina (85.3%, n = 81), Delaware (83.0%, n = 39), New Mexico (82.0%, n=100), Puerto Rico (82.0% n = 50), Indiana (80.7%, n = 331), Missouri (80.4%, n = 221), and Louisiana (79.3%, n=138). [2.4.8](#)
- Among combined SU and MH treatment facilities, 82.8% (n = 7,317) provided suicide prevention services. Of the top 10 geographic distributions, more than 91.3% of combined SU and MH treatment facilities provided suicide prevention services: Other jurisdictions (100.0%, n = 6), Oklahoma (97.6%, n = 123), Montana (96.9%, n = 62), South Dakota (96.2%, n = 25), Kansas (93.3%, n = 83), New Hampshire (92.9%, n = 39), Wyoming (92.9%, n = 39), Alaska (92.8%, n = 64), New Mexico (92.2%, n = 95), and Kentucky (91.3%, n = 283). [2.4.8](#)

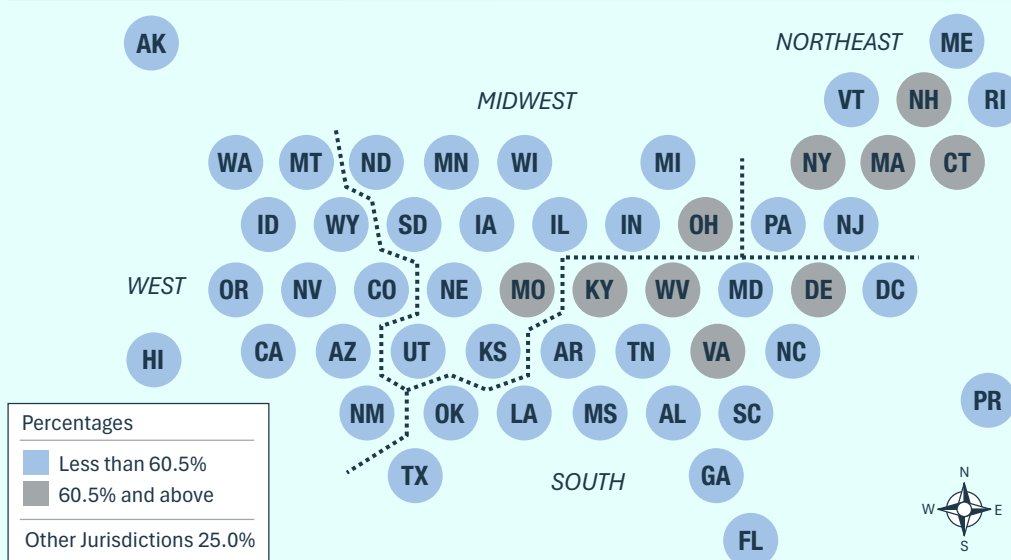
4.1: Substance Use Treatment Facilities Using Medications for Opioid Use Disorder (MOUD) and Medications for Alcohol Use Disorder (MAUD) by State, 2024 N-SUMHSS

Figure 4.1.1: Percentages of SU Treatment Facilities Using MOUD



Among SU treatment facilities, 61.7% (n = 9,842) utilized MOUD. Of the top 10 geographic distributions, more than 72.0% of SU treatment facilities utilized MOUD: New York (87.8%, n = 701), West Virginia (78.8%, n = 123), New Hampshire (77.7%, n = 73), Connecticut (77.6%, n = 149), Virginia (77.1%, n = 289), Pennsylvania (75.4%, n = 441), South Carolina (75.0%, n = 78), Ohio (74.4%, n = 548), Delaware (73.8%, n = 45), and Massachusetts (72.0%, n = 309). [2.4.8.11](#)

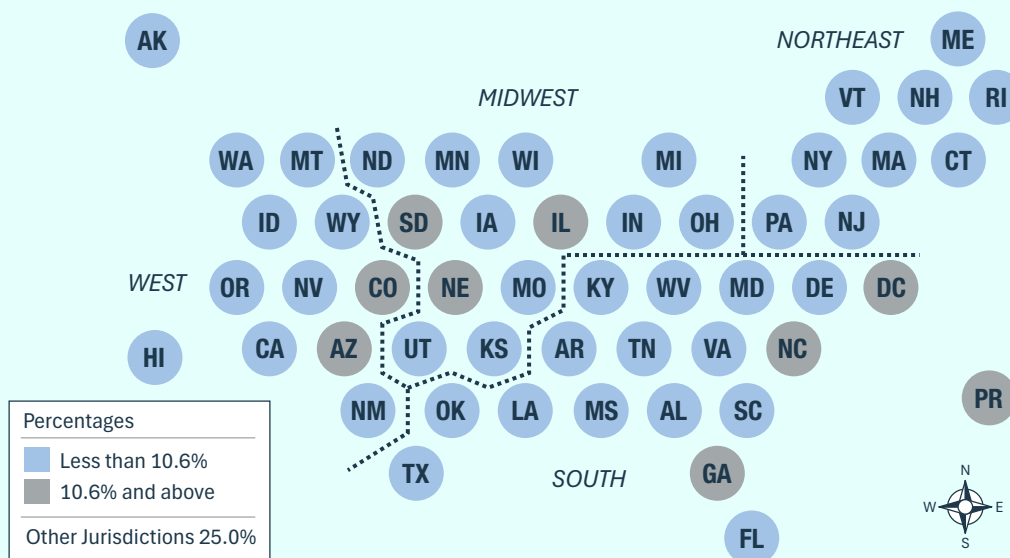
Figure 4.1.2: Percentages of SU Treatment Facilities Using MAUD



Among SU treatment facilities, 49.9% (n = 7,953) utilized MAUD. Of the top 10 geographic distributions, more than 60.5% of SU treatment facilities utilized MAUD: New York (80.2%, n = 640), Connecticut (75.0%, n = 144), New Hampshire (68.1%, n = 64), Delaware (67.2%, n = 41), Missouri (66.3%, n = 191), Ohio (64.3%, n = 474), Virginia (62.9%, n = 236), Massachusetts (62.2%, n = 267), West Virginia (60.9%, n = 95), and Kentucky (60.5%, n = 357). [2.4.8.12](#)

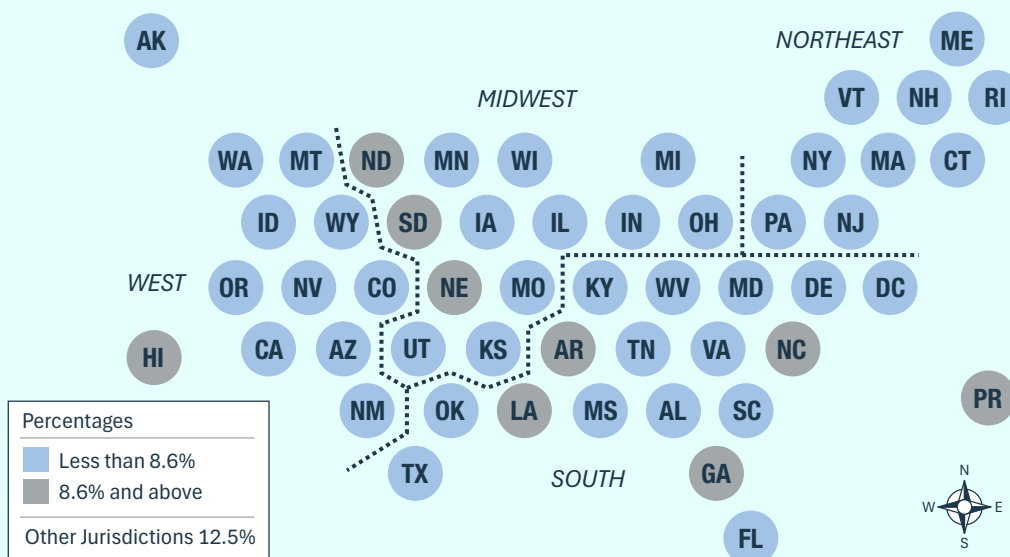
4.2: Substance Use Treatment Facilities Treating Opioid Use Disorder (OUD) by State, 2024 N-SUMHSS

Figure 4.2.1: Percentages of SU Treatment Facilities Not Treating OUD



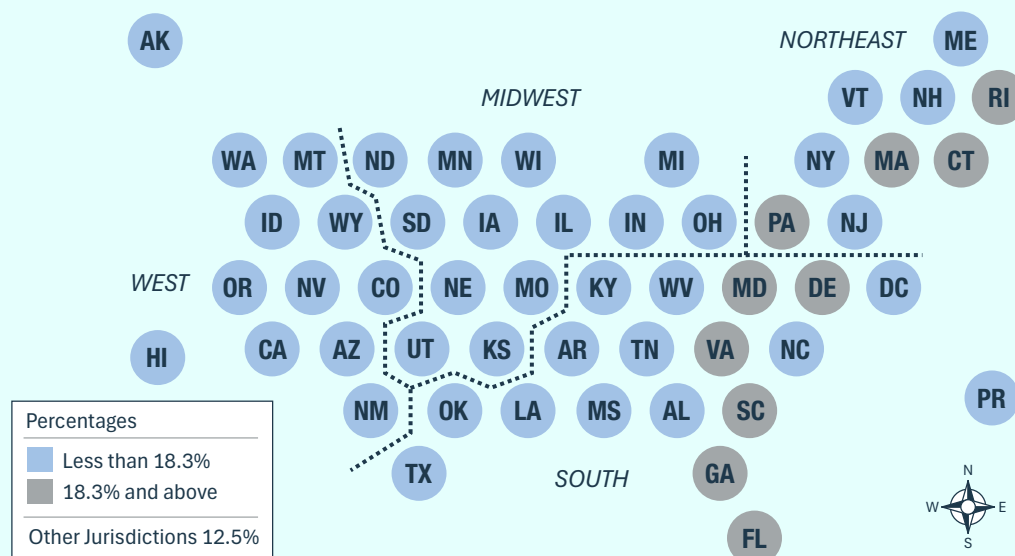
Among SU treatment facilities, 7.5% (n = 1,192) did not treat OUD. Of the top 10 geographic distributions, more than 10.6% SU treatment facilities did not treat OUD: Other jurisdictions (25%, n = 2), D.C. (20.7%, n = 6), Georgia (15.4%, n = 54), Illinois, (15.1%, n = 98), Arizona (13%, n = 70), Nebraska (12.4%, n = 17), North Carolina (11.9%, n = 63), South Dakota (11.5%, n = 6), Puerto Rico (11.3%, n = 8), and Colorado (10.6%, n = 34).^{2,4,8,11}

Figure 4.2.2: Percentages of SU Treatment Facilities Not Using Medications



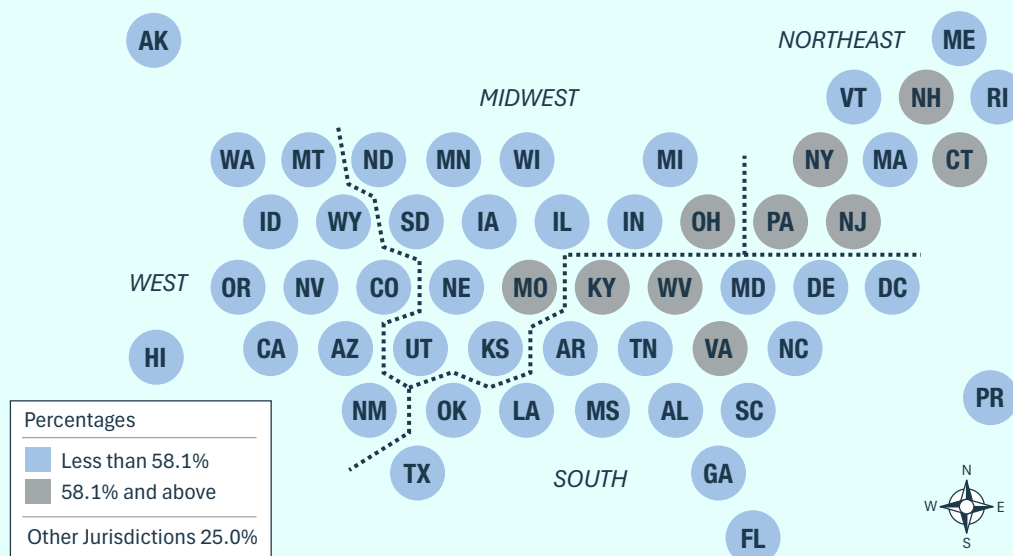
Among SU treatment facilities, 4.0% (n = 640) did not use MOUD treatment. Of the top 10 geographic distributions, more than 8.6% of SU treatment facilities did not use MOUD treatment: Hawaii (51.4%, n = 55), Puerto Rico (16.9%, n = 12), Nebraska (16.1%, n = 22), South Dakota (13.5%, n = 7), Other jurisdictions (12.5%, n = 1), Arkansas (12.3%, n = 19), North Carolina (10.4%, n = 55), Louisiana (9.5%, n = 19), North Dakota (8.7%, n = 6), and Georgia (8.6%, n = 30).^{2,4,8,11}

Figure 4.2.3: Percentages of SU Treatment Facilities Treating OUD, Federally Certified Opioid Treatment Program (OTP)



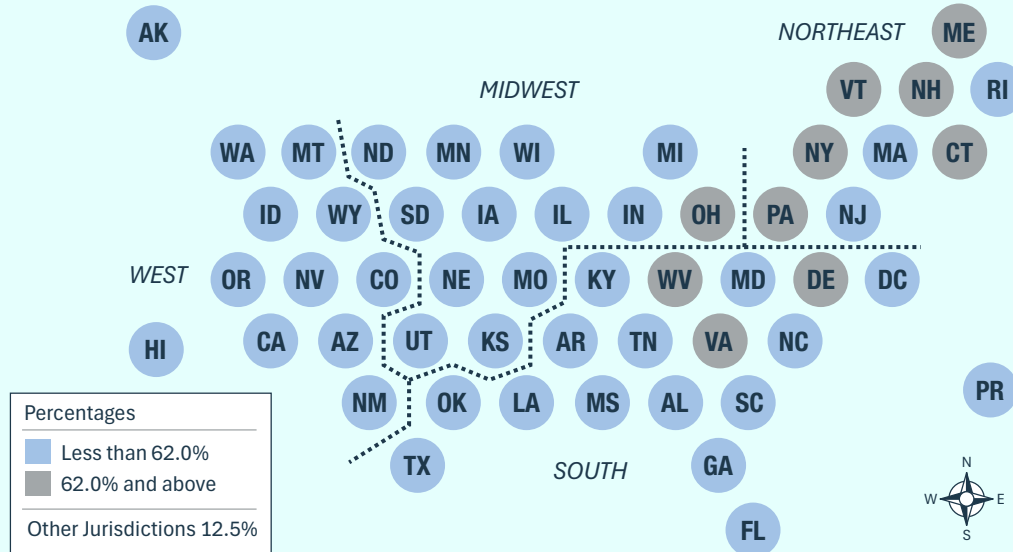
Among SU treatment facilities, 13.9% (n = 2,224) were federally certified OTPs. Of the top 10 geographic distributions, more than 18.3% of SU treatment facilities were federally certified OTPs: Rhode Island (32.8%, n = 20), South Carolina (27.9%, n = 29), Delaware (27.9%, n = 17), Connecticut (25.0%, n = 48), Georgia (21.7%, n = 76), Massachusetts (21.4%, n = 92), Maryland (21.3%, n = 119), Pennsylvania (19.7%, n = 115), Virginia (18.9%, n = 71), and Florida (18.3%, n = 127).^{2.4.8.11}

Figure 4.2.4: Percentages of SU Treatment Facilities Treating OUD, Prescribed Naltrexone



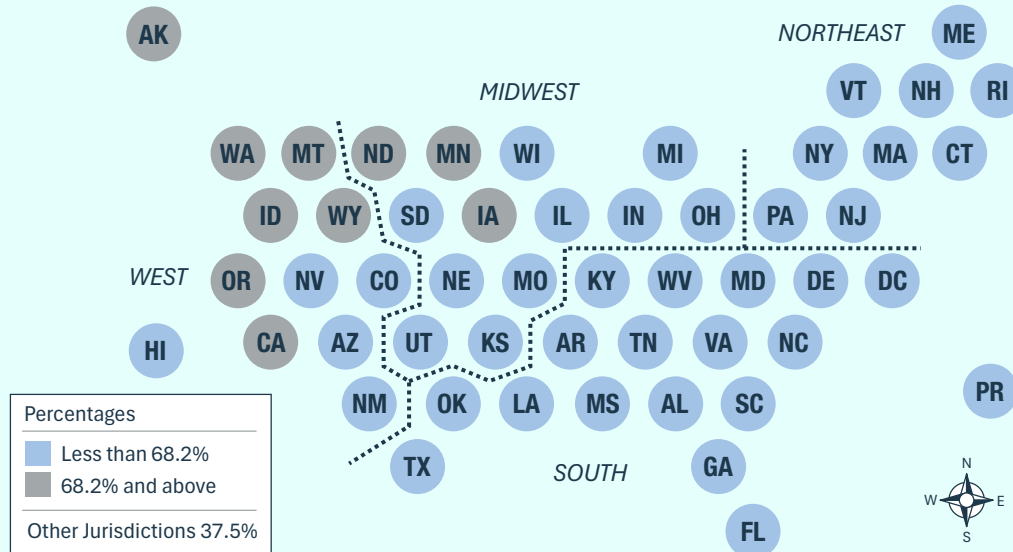
Among SU treatment facilities, 48.6% (n = 7,760) prescribed naltrexone. Of the top 10 geographic distributions, more than 58.1% of SU treatment facilities prescribed naltrexone: New York (76.2%, n = 608), Connecticut (68.8%, n = 132), Ohio (68.0%, n = 501), West Virginia (64.1%, n = 100), Kentucky (62.2%, n = 367), Missouri (61.5%, n = 177), New Hampshire (60.6%, n = 57), Virginia (59.5%, n = 223), New Jersey (58.8%, n = 243), and Pennsylvania (58.1%, n = 340).^{2.4.8.11}

Figure 4.2.5: Percentages of SU Treatment Facilities Treating OUD, Utilized Prescribers of Buprenorphine



Among SU treatment facilities, 52.8% (n = 8,417) utilized prescribers of buprenorphine. Of the top 10 geographic distributions, more than 62.0% of SU treatment facilities utilized prescribers of buprenorphine: New York (79.8%, n = 637), West Virginia (75.6%, n = 118), Connecticut (72.9%, n = 140), Delaware (70.5%, n = 43), Virginia (68.8%, n = 258), New Hampshire (68.1%, n = 64), Pennsylvania (64.8%, n = 379), Vermont (64.3%, n = 36), Ohio (63.6%, n = 469), and Maine (62.0%, n = 93).^{2,4,8,11}

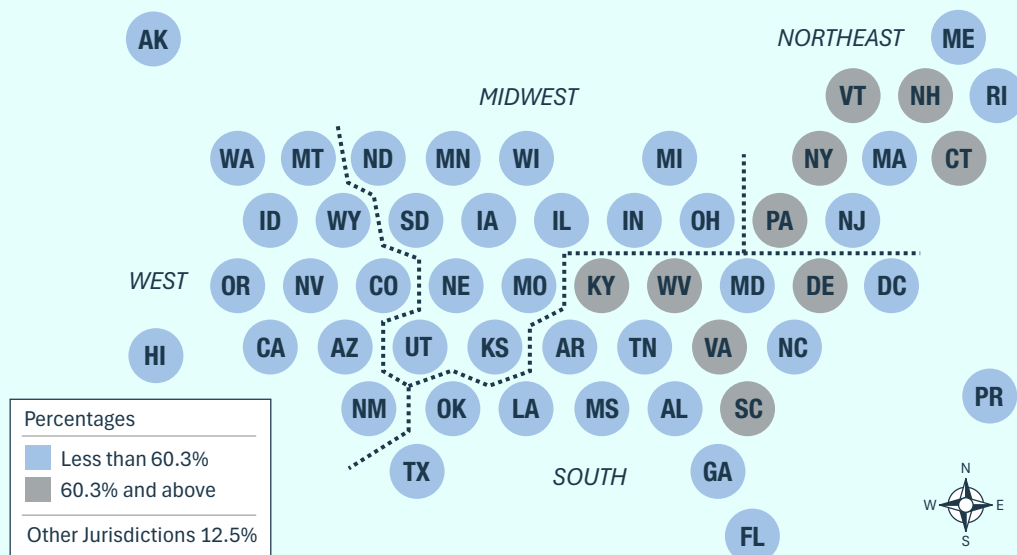
Figure 4.2.6: Percentages of SU Treatment Facilities Treating OUD, Accepted Clients Using Medications for OUD



Among SU treatment facilities, 60.1% (n = 9,595) accepted clients using MOUD. Of the top 10 geographic distributions, more than 68.2% of SU treatment facilities accepted clients using MOUD: Montana (83.8%, n = 83), Alaska (80.0%, n = 72), Iowa (79.0%, n = 154), Wyoming (77.8%, n = 42), Idaho (76.9%, n = 83), Minnesota (76.6%, n = 291), North Dakota (73.9%, n = 51), Oregon (70.2%, n = 174), California (69.1%, n = 1,033), and Washington (68.2%, n = 257).^{2,4,8,11}

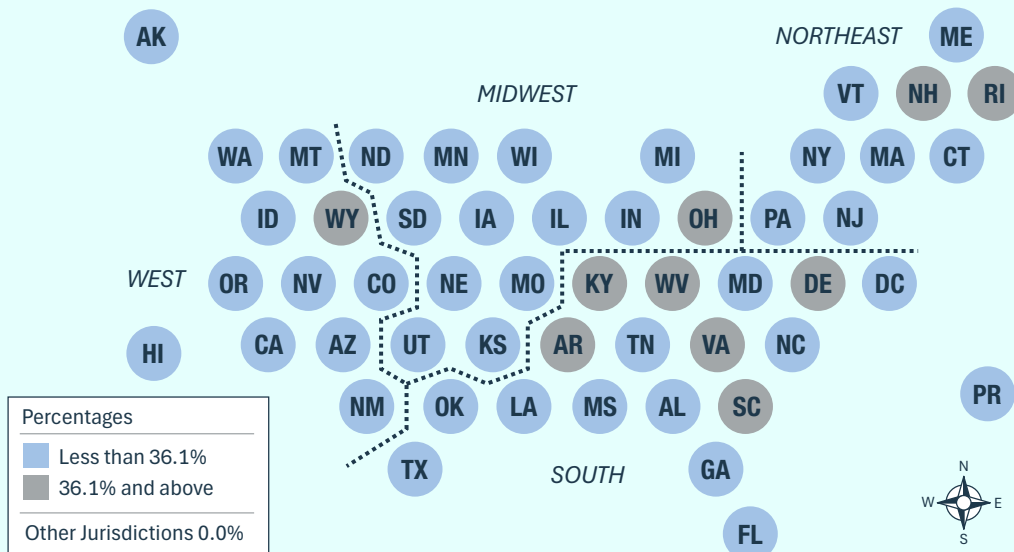
4.3: Substance Use Treatment Facilities Providing Medication Services for Opioid Use Disorder (OUD) by State, 2024 N-SUMHSS

Figure 4.3.1: Percentages of SU Treatment Facilities Providing Services for OUD, Maintenance with Methadone or Buprenorphine



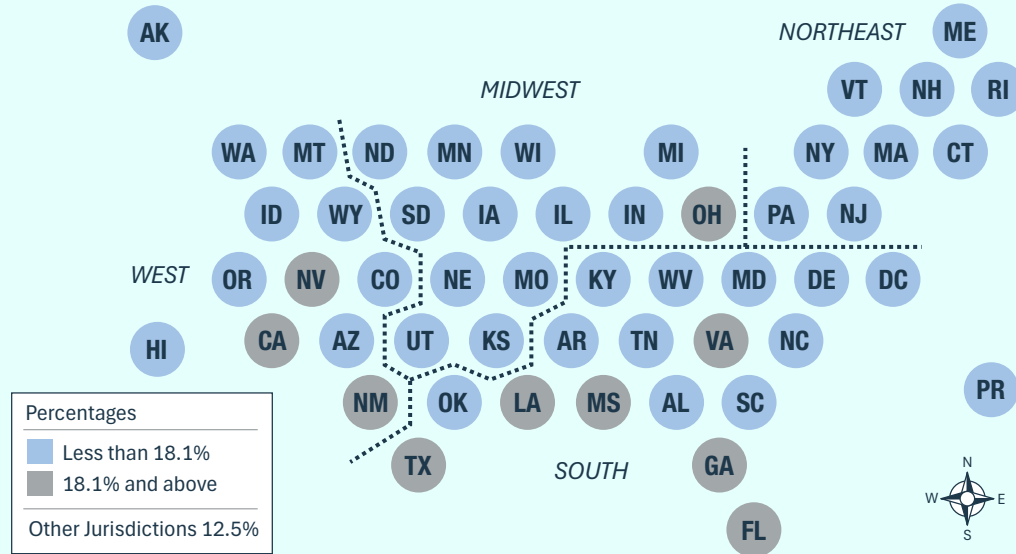
Among SU treatment facilities providing medication services for OUD, 46.3% (n = 7,390) provided maintenance with methadone or buprenorphine. Of the top 10 geographic distributions, more than 60.3% of SU treatment facilities provided maintenance with methadone or buprenorphine: New York (71.1%, n = 567), Delaware (70.5%, n = 43), New Hampshire (66.0%, n = 62), West Virginia (66.0% n = 103), Virginia (63.5% n = 238), Connecticut (61.5%, n = 118), South Carolina (61.5%, n = 64), Vermont (60.7%, n = 34), Kentucky (60.7%, n = 358), and Pennsylvania (60.3%, n = 353).^{2,4,8,11}

Figure 4.3.2: Percentages of SU Treatment Facilities Providing Services for OUD, Medically Supervised Withdrawal



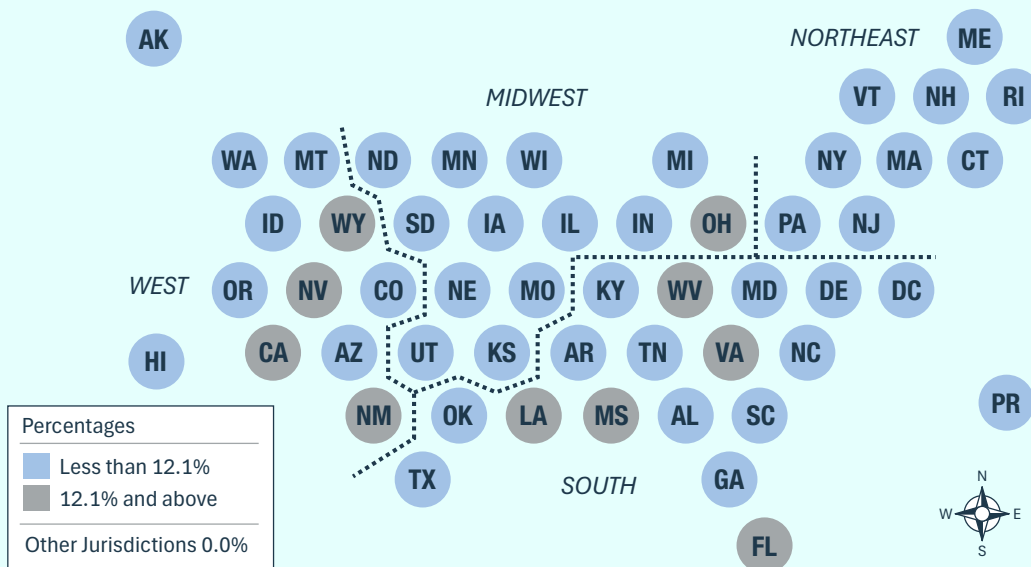
Among SU treatment facilities providing medication services for OUD, 28.1% (n = 4,485) provided medically supervised withdrawal. Of the top 10 geographic distributions, more than 36.1% of SU treatment facilities provided medically supervised withdrawal: Delaware (49.2%, n = 30), Rhode Island (44.3%, n = 27), West Virginia (42.9%, n = 67), Wyoming (42.6%, n = 23), Virginia (41.1%, n = 154), South Carolina (39.4%, n = 41), Arkansas (37.4%, n = 58), Kentucky (36.8%, n = 217), New Hampshire (36.2%, n = 34), and Ohio (36.1%, n = 266).^{2,4,8,11}

Figure 4.3.3: Percentages of SU Treatment Facilities Providing Services for OUD, Detoxification with Methadone or Buprenorphine



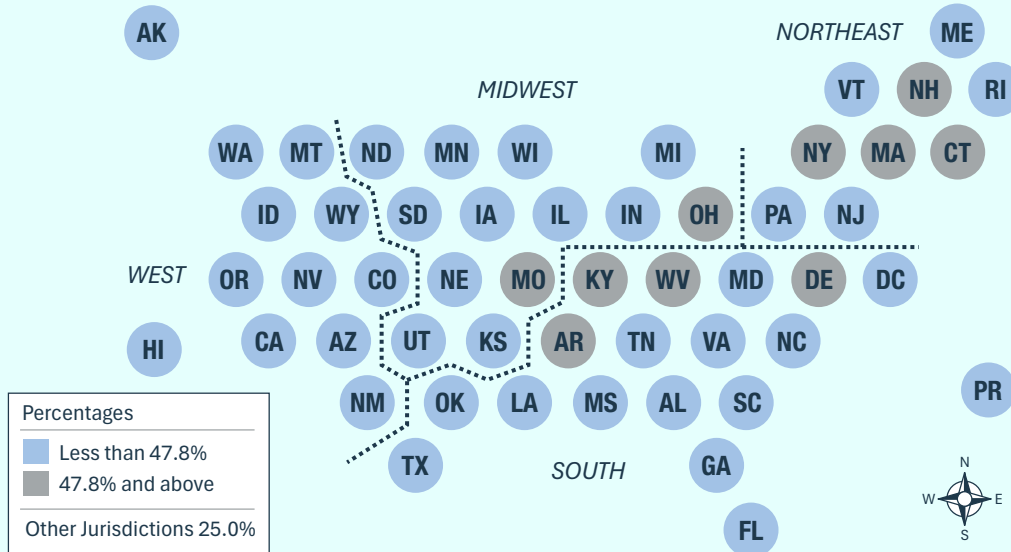
Among SU treatment facilities providing medication services for OUD, 14.2% (n = 2,268) provided detoxification with methadone or buprenorphine. Of the top 10 geographic distributions, more than 18.1% of SU treatment facilities provided detoxification with methadone or buprenorphine: California (26.2%, n = 392), Louisiana (25.1%, n = 50), Mississippi (20.7%, n = 24), Virginia (19.7%, n = 74), Ohio (18.9%, n = 139), Texas (18.9%, n = 106), Nevada (18.9%, n = 25), Florida (18.6%, n = 129), Georgia (18.3%, n = 64), and New Mexico (18.1%, n = 27).^{2,4,8,11}

Figure 4.3.4: Percentages of SU Treatment Facilities Providing Services for OUD, Detoxification with Lofexidine or Clonidine



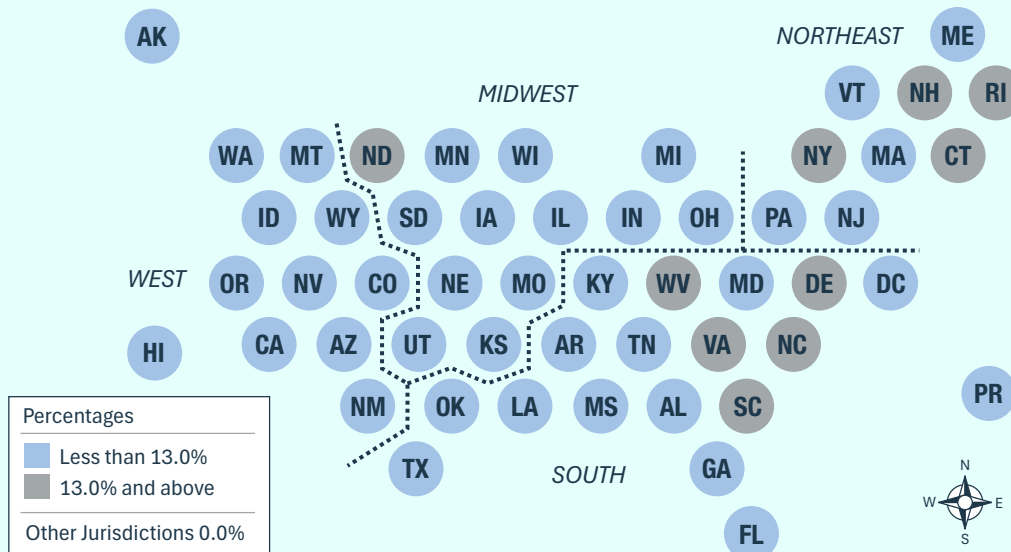
Among SU treatment facilities providing medication services for OUD, 9.3% (n = 1,482) provided detoxification with lofexidine or clonidine. Of the top 10 geographic distributions, more than 12.1% of SU treatment facilities provided detoxification with lofexidine or clonidine: Louisiana (20.6%, n = 41), California (17.0%, n = 254), Virginia (15.2%, n = 57), Ohio (14.9%, n = 110), Wyoming (14.8%, n = 8), West Virginia (14.7%, n = 23), Nevada (13.6%, n = 18), Florida (12.7%, n = 88), Mississippi (12.1%, n = 14), and New Mexico (12.1%, n = 18).^{2,4,8,11}

Figure 4.3.5: Percentages of SU Treatment Facilities Providing Services for OUD, Relapse Prevention



Among SU treatment facilities providing medication services for OUD, 37.5% (n = 5,983) provided relapse prevention services. Of the top 10 geographic distributions, more than 47.8% of SU treatment facilities provided relapse prevention services: New York (59.4%, n = 474), Missouri (56.3%, n = 162), New Hampshire (53.2%, n = 50), West Virginia (53.2%, n = 83), Delaware (52.5%, n = 32), Connecticut (52.1%, n = 100), Arkansas (50.3%, n = 78), Ohio (49.0%, n = 361), Massachusetts (48.5%, n = 208), and Kentucky (47.8%, n = 282).^{2.4.8.11}

Figure 4.3.6: Percentages of SU Treatment Facilities Providing Services for OUD, Other



Among SU treatment facilities providing medication services for OUD, 10.0% (n = 1,597) provided other types of medication services. Of the top 10 geographic distributions, more than 13.0% of SU treatment facilities provided other types of medication services: Connecticut (22.4%, n = 43), South Carolina (22.1%, n = 23), New York (18.4%, n = 147), Rhode Island (18.0%, n = 11), Virginia (17.9%, n = 67), New Hampshire (17.0%, n = 16), Delaware (16.4%, n = 10), West Virginia (15.4%, n = 24), North Carolina (14.7%, n = 78), and North Dakota (13.0%, n = 9).^{2.4.8.11}

Percentages

- Less than 6.9%
- 6.9% and above

Other Jurisdictions 0.0%

4.4: Substance Use Treatment Facilities Treating AUD by State, 2024 N-SUMHSS

Percentages

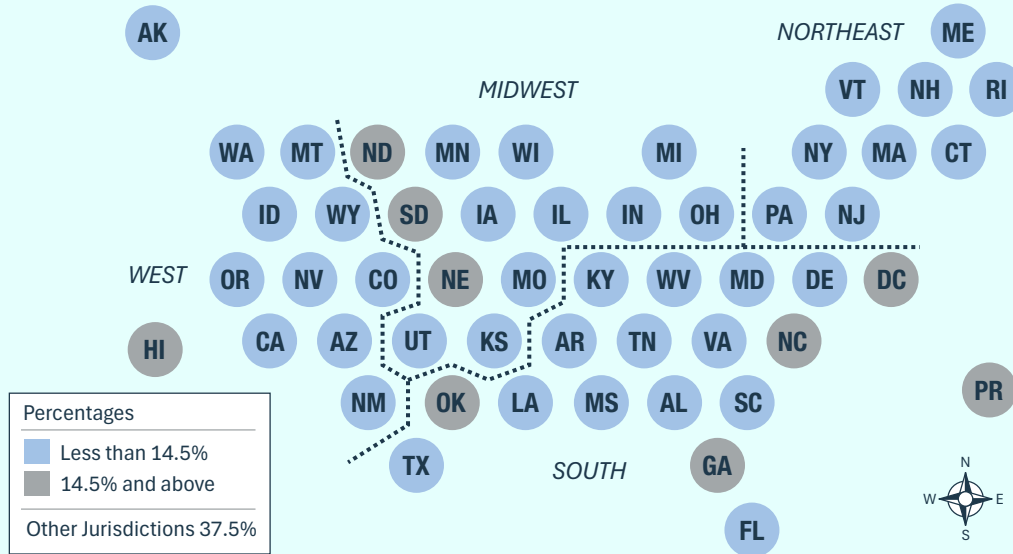
- Less than 14.4%
- 14.4% and above

Other Jurisdictions 0.0%

Map showing the percentage of the population aged 18 and over who are non-Hispanic white by state. The map is divided into four regions: NORTHEAST, MIDWEST, WEST, and SOUTH. States are colored blue for 'Less than 14.4%' and brown for '14.4% and above'. The legend indicates that states with 14.4% and above are shaded brown, while states with less than 14.4% are shaded blue. Other jurisdictions are marked as 0.0%.

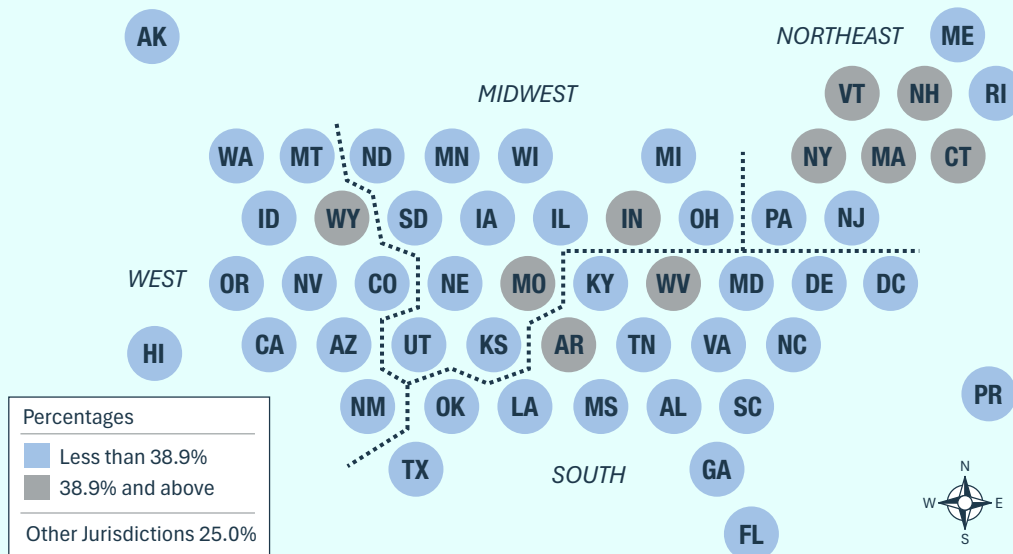
39

Figure 4.4.2: Percentages of SU Treatment Facilities Treating AUD, Not Using Medications



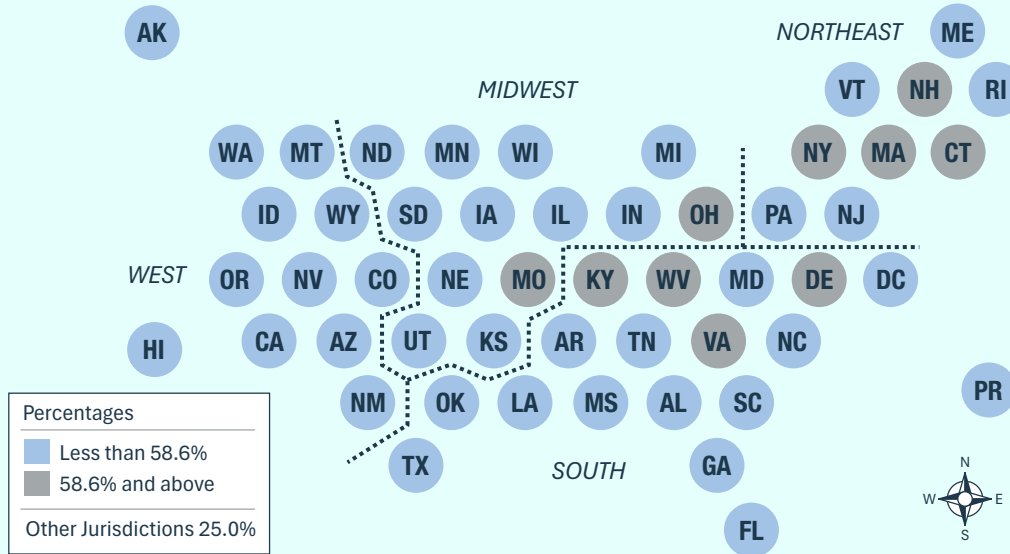
Among SU treatment facilities, 8.9% (n = 1,422) did not use medications to treat AUD. Of the top 10 geographic distributions, more than 14.5% of SU treatment facilities did not use MAUD treatment: Hawaii (49.5%, n = 53), Other jurisdictions (37.5%, n = 3), South Dakota (25.0%, n = 13), Oklahoma (23.2%, n = 41), North Carolina (19.4%, n = 103), Puerto Rico (18.3%, n = 13), Georgia (17.7%, n = 62), Nebraska (17.5%, n = 24), D.C. (17.2%, n = 5), and North Dakota (14.5%, n = 10).^{2,4,8,12}

Figure 4.4.3: Percentages of SU Treatment Facilities Treating AUD, Disulfiram



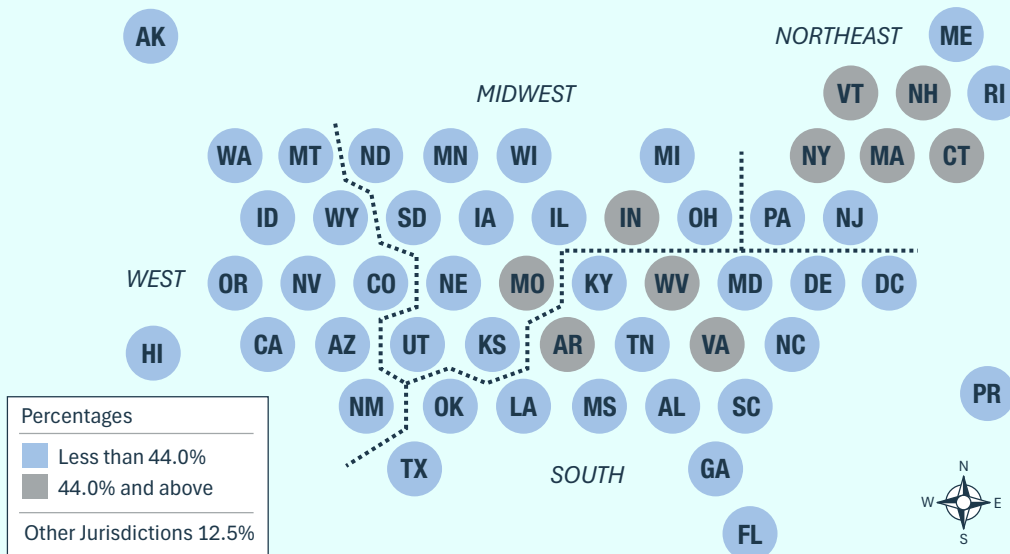
Among SU treatment facilities providing medication services for AUD, 30.6% (n = 4,878) provided disulfiram. Of the top 10 geographic distributions, more than 38.9% of SU treatment facilities provided disulfiram: Connecticut (63.5%, n = 122), New York (54.9%, n = 438), New Hampshire (54.3%, n = 51), Missouri (47.2%, n = 136), Vermont (46.4%, n = 26), West Virginia (44.9%, n = 70), Massachusetts (44.5%, n = 191), Arkansas (41.9%, n = 65), Indiana (40.9%, n = 203), and Wyoming (38.9%, n = 21).^{2,4,8,12}

Figure 4.4.4: Percentages of SU Treatment Facilities Treating AUD, Naltrexone



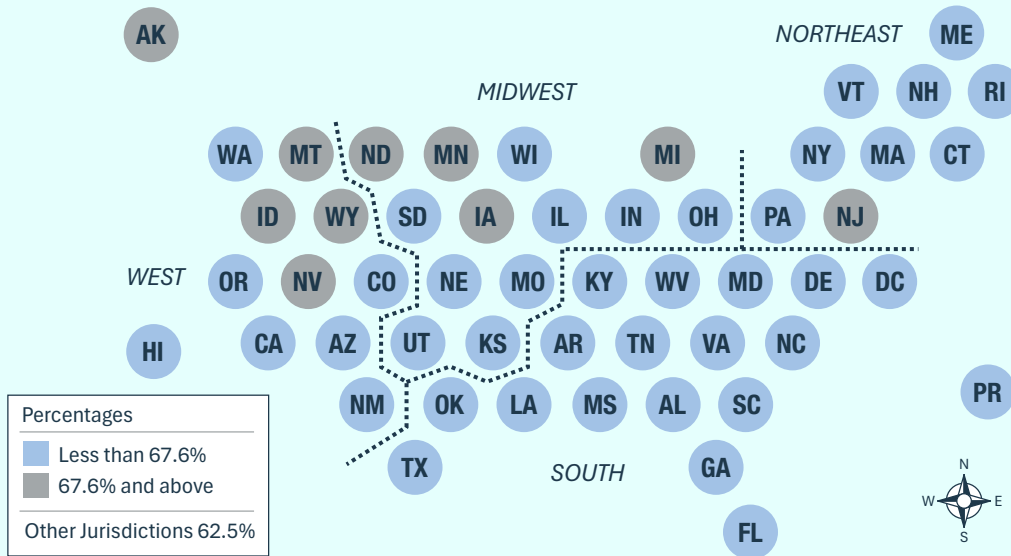
Among SU treatment facilities providing medication services for AUD, 48.3% (n = 7,709) provided naltrexone. Of the top 10 geographic distributions, more than 58.6% of SU treatment facilities provided naltrexone: New York (76.4%, n = 610), Connecticut (72.9%, n = 140), New Hampshire (67.0%, n = 63), Missouri (64.2%, n = 185), Delaware (63.9%, n = 39), Ohio (63.4%, n = 467), Virginia (60.8%, n = 228), Massachusetts (60.6%, n = 260), West Virginia (60.3%, n = 94), and Kentucky (58.6%, n = 346). [2.4.8.12](#)

Figure 4.4.5: Percentages of SU Treatment Facilities Treating AUD, Acamprosate



Among SU treatment facilities providing medication services for AUD, 34.2% (n = 5,448) provided acamprosate. Of the top 10 geographic distributions, more than 44.0% of SU treatment facilities provided acamprosate: Connecticut (63.5%, n = 122), New York (63.3%, n = 505), New Hampshire (56.4%, n = 53), Missouri (50.0%, n = 144), Massachusetts (49.0%, n = 210), West Virginia (48.1%, n = 75), Vermont (46.4%, n = 26), Arkansas (45.8%, n = 71), Indiana (45.2%, n = 224), and Virginia (44.0%, n = 165). [2.4.8.12](#)

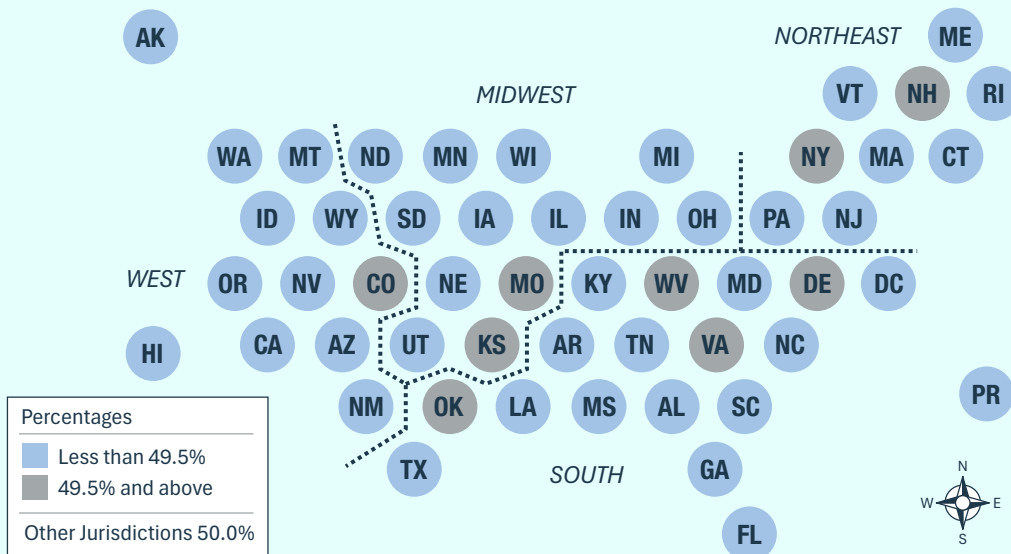
Figure 4.4.6: Percentages of SU Treatment Facilities Treating AUD, Accepts Clients Using Medications for AUD



Among SU treatment facilities, 57.3% (n = 9,135) accepted clients using MAUD. Of the top 10 geographic distributions, more than 67.6% of SU treatment facilities accepted clients using medications for alcohol use disorder: Montana (81.8%, n = 81), Alaska (81.1%, n = 73), Iowa (78.5%, n = 153), North Dakota (73.9%, n = 51), Minnesota (72.6%, n = 276), Idaho (71.3%, n = 77), Nevada (69.7%, n = 92), Wyoming (68.5%, n = 37), New Jersey (67.6%, n = 279), and Michigan (67.6%, n = 305). [2.4.8.12](#)

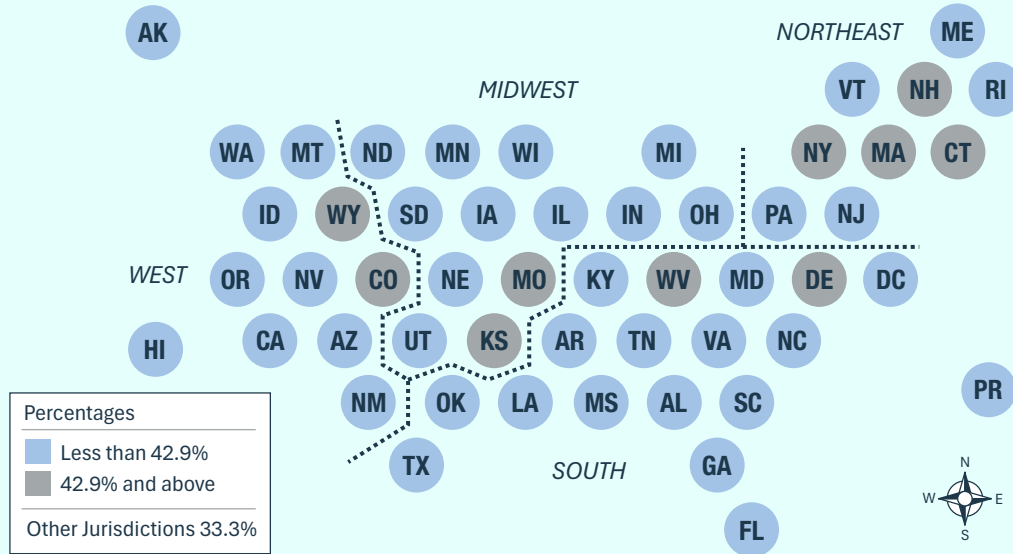
4.5: Mental Health Treatment Facilities Using Medications for Opioid Use Disorder (MOUD) and Medications for Alcohol Use Disorder (MAUD) to Treat Clients with Co-occurring Mental and Substance Use Disorders by State, 2024 N-SUMHSS

Figure 4.5.1: Percentages of MH Treatment Facilities Using MOUD to Treat Clients with Co-occurring Mental Health and Substance Use Disorders



Among MH treatment facilities using medications to treat clients with co-occurring mental and substance use disorders, 39.8% (n = 5,603) used MOUD. Of the top 10 geographic distributions, more than 49.5% of MH treatment facilities used MOUD: Delaware (61.7%, n = 29), West Virginia (60.6%, n = 83), Oklahoma (54.8%, n = 86), Virginia (54.0%, n = 168), Missouri (53.8%, n = 148), New Hampshire (53.4%, n = 39), Kansas (51.9%, n = 69), New York (51.1%, n = 405), Other jurisdictions (50.0%, n = 3), and Colorado (49.5%, n = 108). [2.4.8.12](#)

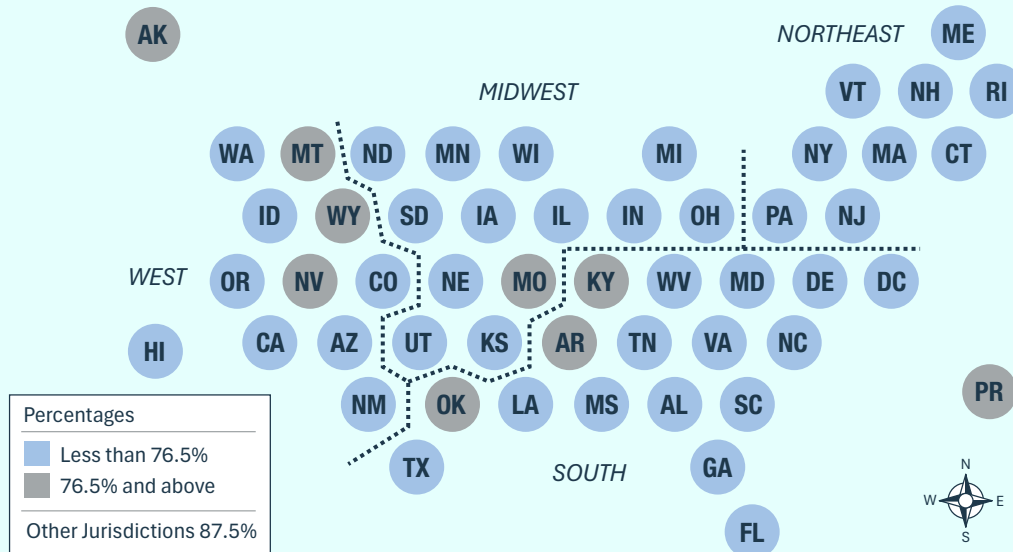
Figure 4.5.2: Percentages of MH Treatment Facilities Using MAUD to Treat Clients with Co-occurring Mental Health and Substance Use Disorders



Among MH treatment facilities using medications to treat clients with co-occurring mental and substance use disorders, 33.9% (n = 4,775) used MAUD. Of the top 10 geographic distributions, more than 42.9% of SU treatment facilities used MAUD: Delaware (61.7%, n = 29), West Virginia (52.6%, n = 72), Connecticut (49.8%, n = 121), New Hampshire (49.3%, n = 36), Missouri (48.4%, n = 133), Wyoming (46.9%, n = 23), Kansas (45.9%, n = 61), Massachusetts (44.0%, n = 143), Colorado (43.6%, n = 95), and New York (42.9%, n = 340).^{2,4,8,12}

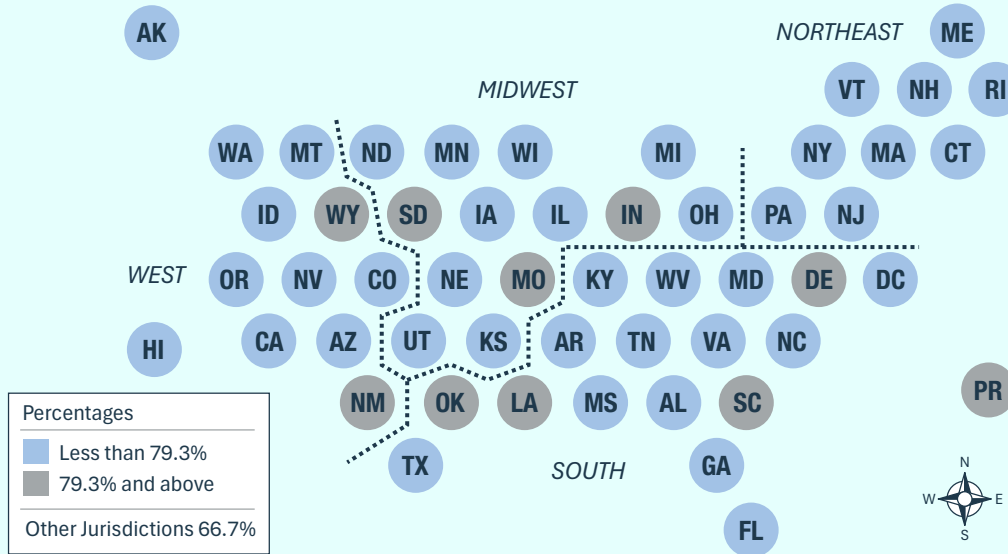
4.6: Substance Use and Mental Health Treatment Facilities Providing Suicide Prevention Services by State, 2024 N-SUMHSS

Figure 4.6.1: Percentages of SU Treatment Facilities Providing Suicide Prevention Services



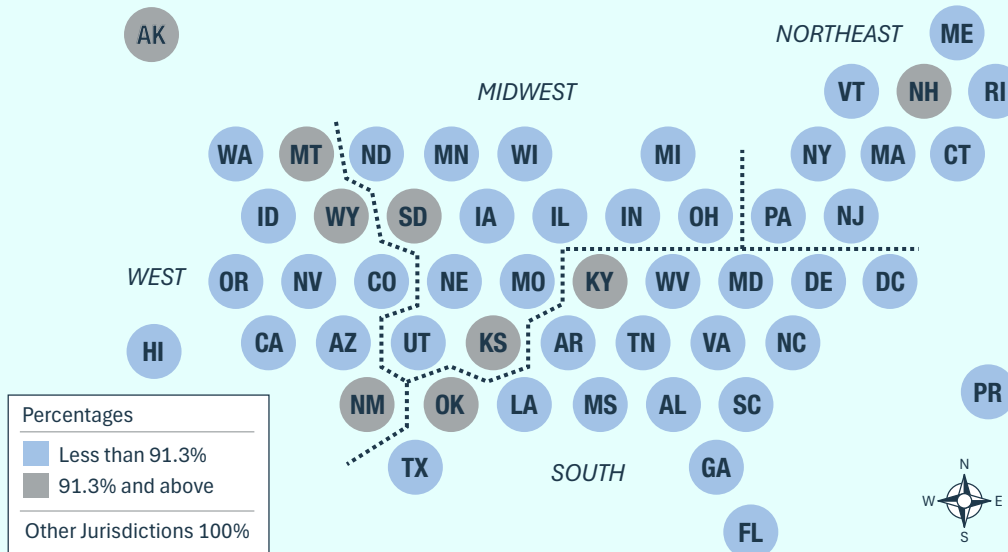
Among SU treatment facilities, 63.5% (n = 10,131) provided suicide prevention services. Of the top 10 geographic distributions, Other jurisdictions, and Puerto Rico, more than 76.5% of SU treatment facilities provided suicide prevention services: Wyoming (88.9%, n = 48), Other jurisdictions (87.5% n = 7), Arkansas (82.6%, n = 128), Puerto Rico (81.7%, n = 58), Alaska (80.0%, n = 72), Missouri (79.2%, n = 228), Montana (78.8%, n = 78), Kentucky (78.0%, n = 460), Oklahoma (76.8%, n = 136), and Nevada (76.5%, n = 101).^{2,4,8}

Figure 4.6.2: Percentages of MH Treatment Facilities Providing Suicide Prevention Services



Among MH treatment facilities, 68.4% (n = 9,645) provided suicide prevention services. Of the top 10 geographic distributions, more than 79.3% of SU treatment facilities provided suicide prevention services: Oklahoma (89.8%, n = 141), South Dakota (89.1%, n = 41), Wyoming (87.8%, n = 43), South Carolina (85.3%, n = 81), Delaware (83.0%, n = 39), New Mexico (82.0%, n = 100), Puerto Rico (82.0% n = 50), Indiana (80.7%, n = 331), Missouri (80.4%, n = 221), and Louisiana (79.3%, n = 138).^{2.4.8}

Figure 4.6.3: Percentages of Combined SU and MH Treatment Facilities Providing Suicide Prevention Services



Among combined SU and MH treatment facilities, 82.8% (n = 7,317) provided suicide prevention services. Of the top 10 geographic distributions, more than 91.3% of combined SU and MH treatment facilities provided suicide prevention services: Other jurisdictions (100.0%, n = 6), Oklahoma (97.6%, n = 123), Montana (96.9%, n = 62), South Dakota (96.2%, n = 25), Kansas (93.3%, n = 83), New Hampshire (92.9%, n = 39), Wyoming (92.9%, n = 39), Alaska (92.8%, n = 64), New Mexico (92.2%, n = 95), and Kentucky (91.3%, n = 283).^{2.4.8}

Endnotes

1. The 2024 N-SUMHSS was a voluntary facility survey that collected data from SU and MH treatment facilities across 50 states, 7 territories, and the District of Columbia. The territories included American Samoa (AS), the Commonwealth of the Northern Mariana Islands (MP), the Federated States of Micronesia (FM), Guam (GU), the Republic of Palau (PW), Puerto Rico (PR), and the U.S. Virgin Islands (VI). In 2024, AS, FM, and PW did not have any eligible facilities.
2. Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Since the overall facility counts vary (i.e., SU = 15,953, MH = 14,091, and SU/MH = 8,839), readers should use both counts and percentages when interpreting the findings on SU, MH, and SU/MH facility characteristics. Caution should be exercised when adding, comparing, and interpreting findings across facility types.
3. A total of 21,205 unique facilities were included in this report: facilities providing only SU treatment services = $15,953 - 8,839 = 7,114$; facilities providing only MH treatment services = $14,091 - 8,839 = 5,252$; and facilities providing both SU and MH treatment facilities = 8,839.
4. Definitions of SU, MH, and SU/MH include: SU – Facilities providing substance use services, MH – Facilities providing mental health services, SU/MH – Facilities providing substance use and mental health services.
5. The American Association for Public Opinion Research (AAPOR) Response Rate 4 (RR4) is the proportion of completed and partially completed surveys divided by the total number of eligible cases in the sample. The total number of eligible cases is comprised of completes and partials, as well as eligible, non-completed surveys and estimated eligible non-respondents. For more details on the 2024 N-SUMHSS response rate calculations, please refer to Appendix B.
6. Eligibility criteria can be found in the Inclusion and Exclusion Criteria and Guidelines for Substance Use and Mental Health Facilities section in Appendix B.
7. Due to unknown or missing data, the following data were excluded: 12 SU facilities, 60 MH facilities, one combined SU/MH facility in 2021; 16 SU facilities, 15 MH facilities, five combined SU/MH facilities in 2022; 15 SU facilities, 10 MH facilities, one combined SU/MH facility in 2023; 11 SU facilities, five MH facilities, and zero combined SU/MH facilities in 2024.
8. Percentages are based on the total facility count within each facility type. For example, SU facility percentages are calculated using the total SU facilities ($n = 15,953$).
9. In 2024, excluded from this data were 1 SU facility and 78 MH facilities with unknown or missing type of service data.
10. Percent of all SU treatment facilities included in the report ($N = 15,953$). Percentages may exceed 100% as some facilities offer multiple pharmacotherapies for SU treatment.
11. Includes facilities that report providing methadone, buprenorphine, or injectable naltrexone for OUD. Methadone is only available at opioid treatment programs (OTPs) certified by SAMHSA's Center for Substance Abuse Treatment (CSAT). Excludes facilities that report accepting clients using MOUD prescribed by another facility.
12. Includes facilities that report providing disulfiram, naltrexone, or acamprosate for AUD. Excludes facilities that report accepting clients using MAUD prescribed by another facility.
13. Includes facilities that report providing nicotine replacement pharmacotherapies or non-nicotine smoking/tobacco cessation medication.
14. Percent of all MH treatment facilities included in the report ($N = 14,091$). Percentages may exceed 100% as some facilities offer multiple pharmacotherapies for MH treatment.
15. Excluded from the report are non-respondents, facilities whose client counts were included or rolled into other facility counts (i.e., roll-ups), and facilities that did not meet the SU and MH guidelines (i.e., halfway houses).
16. Percentages may be slightly less or more than 100 due to rounding.
17. The N-SUMHSS does not operationally define adolescents, young adults, and older adults age categories for SU facilities. Rather, age categories are defined by individual facilities. For mental health facilities, these age groups are explicitly defined as adolescents (ages 13–17), young adults (ages 18–25), and older adults (ages 65+). Therefore, caution should be exercised when interpreting findings, as SU facilities may define these age groups differently. For example, some facilities define 18-year-olds as adolescents while some identify them as young adults.

Appendix A. Data Tables Corresponding to Figures Included in the Report

This appendix provides the data associated with the figures included in Sections 1–4 of this report. Data from the prior years are included in Tables A.8 through A.20. For each table included here, the corresponding figure number is included in the parentheses.

Table A.1: Response Rates by State, Territory, and the District of Columbia, 2024 N-SUMHSS (Figure 1.2)

State/Territory	Rate ¹	State/Territory	Rate ¹	State/Territory	Rate ¹
Alabama (AL)	88.6%	Kentucky (KY)	89.7%	Oklahoma (OK)	93.8%
Alaska (AK)	88.6%	Louisiana (LA)	91.2%	Oregon (OR)	93.5%
American Samoa (AS) ²	–	Maine (ME)	84.1%	Pennsylvania (PA)	92.5%
Arizona (AZ)	91.7%	Maryland (MD)	87.4%	Puerto Rico (PR)	93.0%
Arkansas (AR)	90.2%	Massachusetts (MA)	90.7%	Republic of Palau (PW) ²	–
California (CA)	87.8%	Michigan (MI)	91.4%	Rhode Island (RI)	91.4%
Colorado (CO)	88.7%	Minnesota (MN)	90.2%	South Carolina (SC)	92.4%
Connecticut (CT)	94.9%	Mississippi (MS)	90.3%	South Dakota (SD)	94.1%
Delaware (DE)	83.0%	Missouri (MO)	96.1%	Tennessee (TN)	92.0%
District of Columbia (DC)	91.1%	Montana (MT)	92.5%	Texas (TX)	88.2%
Fed. States of Micronesia (FM) ²	–	Nebraska (NE)	90.8%	Utah (UT)	93.8%
Florida (FL)	90.4%	Nevada (NV)	88.4%	Vermont (VE)	89.7%
Georgia (GA)	90.5%	New Hampshire (NH)	91.9%	U.S. Virgin Islands (VI)	100.0%
Guam (GU)	100.0%	New Jersey (NJ)	88.1%	Virginia (VA)	90.3%
Hawaii (HI)	96.1%	New Mexico (NM)	91.6%	Washington (WA)	87.8%
Idaho (ID)	84.6%	New York (NY)	93.0%	West Virginia (WV)	95.9%
Illinois (IL)	92.1%	North Carolina (NC)	88.2%	Wisconsin (WI)	90.5%
Indiana (IN)	93.4%	North Dakota (ND)	78.4%	Wyoming (WY)	90.6%
Iowa (IA)	89.0%	No. Mariana Islands (MP)	100.0%		
Kansas (KS)	94.3%	Ohio (OH)	91.0%		

¹ Calculated using the American Association for Public Opinion Research (AAPOR) Response Rate 4 (RR4) definition, which is the proportion of completed as well as partially completed surveys divided by the total number of eligible cases in the sample. The total number of eligible cases is comprised of completes and partials as well as eligible, non-completed surveys and estimated eligible non-respondents. For more details on the 2024 N-SUMHSS response rate calculations, please refer to Appendix B.

² American Samoa, the Federated States of Micronesia, and the Republic of Palau did not have any eligible facilities in 2024.

Table A.2: Counts and Percentages of Substance Use and Mental Health Treatment Facilities by Operation Type, 2024 N-SUMHSS (Figure 2.1.1 through Figure 2.1.3)

Operation Type	SU Facilities ^{1,2}		MH Facilities ^{1,2}		Combined SU and MH Facilities ^{1,2}	
	Counts	Percentages (n = 15,953)	Counts	Percentages (n = 14,091)	Counts	Percentages (n = 8,839)
A private for-profit organization	6,724	42.1	4,199	29.8	2,993	33.9
A private non-profit organization	7,697	48.2	7,915	56.2	4,740	53.6
State government	331	2.1	621	4.4	231	2.6
Local, county, or community government	655	4.1	817	5.8	450	5.1
Tribal government	176	1.1	110	0.8	107	1.2
Federal government	359	2.3	424	3.0	318	3.6
Department of Veterans Affairs	311	1.9	397	2.8	293	3.3
Department of Defense	22	0.1	9	0.1	9	0.1
Indian Health Service	16	0.1	11	0.1	10	0.1
Other ³	9	0.1	7	0.0	6	0.1

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² Excluded from the table were 11 SU facilities and five MH facilities with unknown or missing data.

³ Examples of other federal government agencies include the Substance Abuse and Mental Health Administration (SAMHSA), the Health Resources and Services Administration (HRSA), and the Department of Health and Human Services (HHS).

Table A.3: Counts and Percentages of Substance Use and Mental Health Treatment Facilities by Type of Care, 2024 N-SUMHSS (Figure 2.2)

Type of Care	SU Facilities ^{1,2}		MH Facilities ^{1,2}		Combined SU and MH Facilities ^{1,2}	
	Counts	Percentages (n = 15,953)	Counts	Percentages (n = 14,091)	Counts	Percentages (n = 8,839)
Hospital inpatient	1,201	7.5	1,288	9.1	786	8.9
Residential	3,596	22.5	2,460	17.5	1,639	18.5
Outpatient	13,369	83.8	12,015	85.3	8,088	91.5

¹ Facilities counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² Excluded from the table were one SU facility and 78 MH facilities with unknown or missing data.

Table A.4: Counts and Percentages of Substance Use and Mental Health Treatment Facilities by Tailored Programs for Specific Groups, 2024 N-SUMHSS (Figure 2.3)

Tailored Programs for Specific Groups	SU Facilities ¹		MH Facilities ¹		Combined SU and MH Facilities ¹	
	Counts	Percentages (N = 15,953)	Counts	Percentages (N = 14,091)	Counts	Percentages (N = 8,839)
Veterans	5,740	36.0	4,001	28.4	3,967	44.9
Active-duty military service members	3,811	23.9	2,370	16.8	2,744	31.0
Members of military families	4,287	26.9	2,923	20.7	3,128	35.4
Criminal justice clients (other than DUI/DWI)	7,506	47.1	3,990	28.3	5,002	56.6
Clients with co-occurring mental and substance use disorders	10,349	64.9	8,933	63.4	7,435	84.1
Clients with HIV or AIDS	5,017	31.4	2,830	20.1	3,351	37.9
Clients who have experienced intimate partner violence/domestic violence	6,507	40.8	1,989	14.1	4,131	46.7
Clients who have experienced trauma	8,981	56.3	7,544	53.5	6,278	71.0

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

Table A.5: Counts and Percentages of Substance Use Treatment Facilities Utilizing Pharmacotherapies, 2024 N-SUMHSS (Figure 2.4)

	Counts ¹	Percentages ^{1,2} (N = 15,953)
Facilities using any pharmacotherapy	12,207	76.5
Facilities providing medications for opioid use disorder (MOUD) ³	9,842	61.7
Facilities providing medications for alcohol use disorder (MAUD) ⁴	7,953	49.9
Facilities using pharmacotherapies for tobacco cessation ⁵	7,628	47.8

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² Percentage of all SU treatment facilities included in the report (n = 15,953). The sum of the percentages may exceed 100% as some facilities offer multiple pharmacotherapies for SU treatment.

³ Includes facilities that report providing methadone, buprenorphine, or injectable naltrexone for opioid use disorder (OUD). Methadone is only available at opioid treatment programs (OTPs) certified by SAMHSA's Center for Substance Abuse Treatment (CSAT). Excludes facilities that report accepting clients using MOUD prescribed by another facility.

⁴ Includes facilities that report providing disulfiram, naltrexone, or acamprosate for alcohol use disorder (AUD). Excludes facilities that report accepting clients using MAUD prescribed by another facility.

⁵ Includes facilities that report providing nicotine replacement pharmacotherapies or non-nicotine smoking/tobacco cessation medications.

Table A.6: Counts and Percentages of Mental Health Treatment Facilities Utilizing Pharmacotherapies, 2024 N-SUMHSS (Figure 2.5)

	Counts ¹	Percentages ^{1,2} (N = 14,091)
Facilities providing medications for smoking cessation	6,071	43.1
Facilities using antipsychotics for treatment of SMI	8,489	60.2

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² Percentages of all MH treatment facilities included in the report (n = 14,091). The sum of the percentages may exceed 100% as some facilities offer multiple pharmacotherapies for MH treatment.

Table A.7: Counts and Percentages of Substance Use Treatment Facilities Offering Overdose and Naloxone Education, 2024 N-SUMHSS (Figure 2.6)

	Counts ¹	Percentages ¹ (N = 15,953)
Facilities providing overdose and Naloxone education	12,814	80.3

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

Table A.8: Counts and Percentages of Substance Use Treatment Facilities' Licensing, Certification, or Accreditation, 2021–2024 N-SUMHSS (Figure 3.1.1)

	Counts ¹				Percentages ¹			
	2021	2022	2023	2024	2021 (n = 14,010)	2022 (n = 14,854)	2023 (n = 14,620)	2024 (n = 15,953)
Any listed agency/organization	13,263	14,063	13,987	15,288	94.7	94.7	95.7	95.8
State substance use treatment agency	10,632	10,881	10,591	11,009	75.9	73.3	72.4	69.0
State mental health department	5,427	5,555	5,571	6,201	38.7	37.4	38.1	38.9
State department of health	6,787	7,041	6,963	7,374	48.4	47.4	47.6	46.2
Hospital licensing authority	564	534	514	603	4.0	3.6	3.5	3.8
The Joint Commission	3,346	3,587	3,660	4,133	23.9	24.1	25.0	25.9
Commission on Accreditation of Rehabilitation Facilities (CARF)	4,354	4,875	4,902	5,414	31.1	32.8	33.5	33.9
National Committee for Quality Assurance (NCQA)	317	310	303	314	2.3	2.1	2.1	2.0
Council on Accreditation (COA)	596	581	568	683	4.3	3.9	3.9	4.3
Healthcare Facilities Accreditation Program (HFAP)	116	116	119	110	0.8	0.8	0.8	0.7
SAMHSA certification for opioid treatment program (OTP)	1,607	1,977	2,085	2,258	11.5	13.3	14.3	14.2
Drug Enforcement Administration (DEA)	1,650	2,072	2,247	2,510	11.8	13.9	15.4	15.7
Other ²	972	1,288	1,317	1,673	6.9	8.7	9.0	10.5
Not licensed, certified, or accredited	488	519	431	468	3.5	3.5	2.9	2.9

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² Other includes any local, state, federal agency that is not listed but provides licensing, certification, or accreditation to substance use treatment facilities.

Table A.9: Counts and Percentages of Mental Health Treatment Facilities' Licensing, Certification, or Accreditation, 2021–2024 N-SUMHSS (Figure 3.1.2)

	Counts ¹				Percentages ¹			
	2021	2022	2023	2024	2021 (n = 9,050)	2022 (n = 9,586)	2023 (n = 9,856)	2024 (n = 14,091)
Any listed agency/organization	8,854	9,409	9,662	13,701	97.8	98.2	98.0	97.2
State mental health authority	6,226	6,341	6,578	8,468	68.8	66.1	66.7	60.1
State substance use treatment agency	2,976	3,102	3,460	6,052	32.9	32.4	35.1	42.9
State department of health	4,533	4,619	4,893	6,917	50.1	48.2	49.6	49.1
State or local Department of Family and Children's Services	1,678	1,696	1,718	2,244	18.5	17.7	17.4	15.9
Hospital licensing authority	1,166	1,082	983	1,054	12.9	11.3	10.0	7.5
The Joint Commission	3,198	3,441	3,359	4,499	35.3	35.9	34.1	31.9
Commission on Accreditation of Rehabilitation Facilities (CARF)	2,325	2,438	2,499	3,938	25.7	25.4	25.4	27.9
Council on Accreditation (COA)	872	962	928	1,107	9.6	10.0	9.4	7.9
Centers for Medicare & Medicaid Services (CMS)	4,608	4,411	4,534	5,588	50.9	46.0	46.0	39.7
Other ²	683	864	984	1,514	7.5	9.0	10.0	10.7
Not licensed, certified, or accredited	93	101	119	286	1.0	1.1	1.2	2.0

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² Other includes any local, state, federal agency that is not listed but provides licensing, certification, or accreditation to substance use treatment facilities.

Table A.10: Counts and Percentages of Substance Use Treatment Facilities by Operation Type, 2021–2024 N-SUMHSS (Figure 3.2.1)

Operation Type	Counts ^{1,2}				Percentages ^{1,2}			
	2021	2022	2023	2024	2021 (n = 14,010)	2022 (n = 14,854)	2023 (n = 14,620)	2024 (n = 15,953)
Private	12,715	13,481	13,355	14,421	90.8	90.8	91.3	90.4
Private for-profit	5,783	6,311	6,393	6,724	41.3	42.5	43.7	42.1
Private non-profit	6,932	7,170	6,962	7,697	49.5	48.3	47.6	48.2
Public	1,283	1,357	1,250	1,521	9.2	9.1	8.5	9.5
State government	264	297	298	331	1.9	2.0	2.0	2.1
Local, county, or community government	555	598	566	655	4.0	4.0	3.9	4.1
Tribal government	203	210	171	176	1.4	1.4	1.2	1.1
Federal government	261	252	215	359	1.9	1.7	1.5	2.3

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² Excluded from this table were 12 SU facilities (2021), 16 SU facilities (2022), 15 SU facilities (2023), and 11 SU facilities (2024) due to unknown or missing data.

Table A.11: Counts and Percentages of Mental Health Treatment Facilities by Operation Type, 2021–2024 N-SUMHSS (Figure 3.2.2)

	Counts ^{1,2}				Percentages ^{1,2}			
	2021	2022	2023	2024	2021 (n = 9,050)	2022 (n = 9,586)	2023 (n = 9,856)	2024 (n = 14,091)
Private	7,363	7,800	8,171	12,114	81.4	81.4	82.9	86.0
Private for-profit	1,528	1,834	2,295	4,199	16.9	19.1	23.3	29.8
Private non-profit	5,835	5,966	5,876	7,915	64.5	62.2	59.6	56.2
Public	1,627	1,771	1,675	1,972	18.0	18.5	17.0	14.0
State government	531	605	591	621	5.9	6.3	6.0	4.4
Local, county, or community government	661	721	691	817	7.3	7.5	7.0	5.8
Tribal government	24	27	29	110	0.3	0.3	0.3	0.8
Federal government	411	418	364	424	4.5	4.4	3.7	3.0

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² Excluded from this table were 60 MH facilities (2021), 15 MH facilities (2022), 10 MH facilities (2023), and five MH facilities (2024) due to unknown or missing data.

Table A.12: Counts and Percentages of Combined Substance Use and Mental Health Treatment Facilities by Operation Type, 2021–2024 N-SUMHSS (Figure 3.2.3)

	Counts ^{1,2}				Percentages ^{1,2}			
	2021	2022	2023	2024	2021 (n = 2,438)	2022 (n = 3,280)	2023 (n = 3,795)	2024 (n = 8,839)
Private	2,084	2,748	3,259	7,733	85.5	83.8	85.9	87.5
Private for-profit	538	755	1,133	2,993	22.1	23.0	29.9	33.9
Private non-profit	1,546	1,993	2,126	4,740	63.4	60.8	56.0	53.6
Public	353	527	535	1,106	14.5	16.1	14.1	12.5
State government	84	126	136	231	3.4	3.8	3.6	2.6
Local, county, or community government	169	251	263	450	6.9	7.7	6.9	5.1
Tribal government	17	23	22	107	0.7	0.7	0.6	1.2
Federal government	83	127	114	318	3.4	3.9	3.0	3.6

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² Excluded from this table were one SU/MH facility (2021), five SU/MH facilities (2022), one SU/MH facility (2023), and zero SU/MH facilities (2024) due to unknown or missing data.

Table A.13: Counts and Percentages of Substance Use Treatment Facilities Offering Tailored Programs for Specific Age Groups, 2021–2024 N-SUMHSS (Figure 3.3.1)

	Counts ¹				Percentages ¹			
	2021	2022	2023	2024	2021 (n = 14,010)	2022 (n = 14,854)	2023 (n = 14,620)	2024 (n = 15,953)
Adolescents ²	3,400	3,521	3,618	4,026	24.3	23.7	24.7	25.2
Young adults ²	4,983	5,446	5,716	6,329	35.6	36.7	39.1	39.7
Adult women ²	7,610	8,036	8,366	9,013	54.3	54.1	57.2	56.5
Adult men ²	7,471	7,930	8,236	8,930	53.3	53.4	56.3	56.0
Seniors or older adults ²	4,222	4,755	5,308	5,892	30.1	32.0	36.3	36.9

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² Data on age category as selected by facilities. Please note that the N-SUMHSS does not operationally define adolescents, young adults, and older adults for SU facilities. For MH facilities, these age groups are explicitly defined as adolescents (ages 13–17), young adults (ages 18–25), and older adults (ages 65+). Therefore, caution should be exercised when interpreting findings, as SU facilities may define these age groups differently. For example, some facilities define 18-year-olds as adolescents while some identify them as young adults.

Table A.14: Counts and Percentages of Mental Health Treatment Facilities Offering Tailored Programs for Specific Age Groups, 2021–2024 N-SUMHSS (Figure 3.3.2)

	Counts ¹				Percentages ¹			
	2021	2022	2023	2024	2021 (n = 9,050)	2022 (n = 9,586)	2023 (n = 9,856)	2024 (n = 14,091)
Children/adolescents with serious emotional disturbance (SED) ²	3,461	3,624	3,760	4,484	38.2	37.8	38.1	31.8
Young adults ²	2,791	3,133	3,554	5,099	30.8	32.7	36.1	36.2
Clients 18 and older with serious mental illness (SMI) ²	4,773	4,934	5,208	6,748	52.7	51.5	52.8	47.9
Older adults ²	2,758	3,035	3,391	5,165	30.5	31.7	34.4	36.7

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² Data on age category as selected by facilities. Please note that the N-SUMHSS does not operationally define adolescents, young adults, and older adults for SU facilities. For MH facilities, these age groups are explicitly defined as adolescents (ages 13–17), young adults (ages 18–25), and older adults (ages 65+). Therefore, caution should be exercised when interpreting findings, as SU facilities may define these age groups differently. For example, some facilities define 18-year-olds as adolescents while some identify them as young adults.

Table A.15: Counts and Percentages of Substance Use and Mental Health Treatment Facilities Offering Treatment for Co-occurring Disorders Plus Either Serious Mental Illness (SMI) in Adults and/or Serious Emotional Disturbance (SED) in Children, 2021–2024 N-SUMHSS (Figure 3.4)

Facility Type	Counts ¹				Percentages ¹			
	2021	2022	2023	2024	2021	2022	2023	2024
SU ²	8,320	8,833	8,694	9,944	59.4	59.5	59.5	62.3
MH ³	6,869	7,637	7,998	11,334	75.9	79.7	81.1	80.4

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² In 2021, there were 14,101 SU facilities; in 2022, there were 14,854 SU facilities; in 2023, there were 14,620 SU facilities; in 2024, there were 15,953 SU facilities.

³ In 2021, there were 9,050 MH facilities; in 2022, there were 9,586 MH facilities; in 2023, there were 9,856 MH facilities; in 2024, there were 14,091 MH facilities.

Table A.16: Counts and Percentages of Substance Use Treatment Facilities Offering Recovery Support Services, 2021–2024 N-SUMHSS (Figure 3.5)

Service	Counts ¹				Percentages ¹			
	2021	2022	2023	2024	2021 (n = 14,010)	2022 (n = 14,854)	2023 (n = 14,620)	2024 (n = 15,953)
Mentoring/peer support	9,127	9,883	10,189	11,254	65.1	66.5	69.7	70.5
Self-help groups	6,697	7,024	6,920	7,481	47.8	47.3	47.3	46.9
Assistance in locating housing	9,359	10,013	10,111	11,335	66.8	67.4	69.2	71.1
Employment counseling or training	6,423	6,859	7,009	7,813	45.8	46.2	47.9	49.0
Assistance obtaining social services	10,089	10,834	10,884	12,151	72.0	72.9	74.4	76.2
Recovery coach	4,392	5,128	5,391	6,069	31.3	34.5	36.9	38.0
None of the recovery support services above are offered at this facility	1,138	1,143	1,120	1,058	8.1	7.7	7.7	6.6

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

Table A.17: Counts and Percentages of Substance Use and Mental Health Treatment Facilities Offering Treatment at No Charge or Minimal Payment to clients who Cannot Afford to Pay, 2021–2024 N-SUMHSS (Figure 3.6.1)

Facility Type	Counts ¹				Percentages ¹			
	2021	2022	2023	2024	2021	2022	2023	2024
SU ²	6,334	6,832	6,593	7,383	45.2	46.0	45.1	46.3
MH ³	4,958	5,155	5,243	7,167	54.8	53.8	53.2	50.9
Combined SU and MH ⁴	1,350	1,878	2,114	4,630	55.4	57.3	55.7	52.4

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² In 2021, there were 14,101 SU facilities; in 2022, there were 14,854 SU facilities; in 2023, there were 14,620 SU facilities; in 2024, there were 15,953 SU facilities.

³ In 2021, there were 9,050 MH facilities; in 2022, there were 9,586 MH facilities; in 2023, there were 9,856 MH facilities; in 2024, there were 14,091 MH facilities.

⁴ In 2021, there were 2,438 SU/MH facilities; in 2022, there were 3,280 SU/MH facilities; in 2023, there were 3,795 SU/MH facilities; in 2024, there were 8,839 SU/MH facilities.

Table A.18: Counts and Percentages of Substance Use Treatment Facilities by Types of Client Payments or Insurance Accepted, 2021–2024 N-SUMHSS (Figure 3.6.2)

	Counts ¹				Percentages ¹			
	2021	2022	2023	2024	2021 (n = 14,010)	2022 (n = 14,854)	2023 (n = 14,620)	2024 (n = 15,953)
No payment accepted	286	286	254	253	2.0	1.9	1.7	1.6
Cash or self-payment	12,680	13,338	13,242	14,337	90.5	89.8	90.6	89.9
Medicare	6,334	6,857	7,123	8,397	45.2	46.2	48.7	52.6
Medicaid	10,259	11,021	11,214	12,418	73.2	74.2	76.7	77.8
State-financed health insurance plan other than Medicaid	6,971	7,358	7,533	8,380	49.8	49.5	51.5	52.5
Federal military insurance	5,641	5,986	6,332	7,484	40.3	40.3	43.3	46.9
Private insurance	10,539	11,150	11,309	12,494	75.2	75.1	77.4	78.3

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

Table A.19: Counts and Percentages of Mental Health Treatment Facilities by Types of Client Payments or Insurance Accepted, 2021–2024 N-SUMHSS (Figure 3.6.3)

	Counts ¹				Percentages ¹			
	2021	2022	2023	2024	2021 (n = 9,050)	2022 (n = 9,586)	2023 (n = 9,856)	2024 (n = 14,091)
Cash or self-payment	7,529	7,950	8,385	12,114	83.2	82.9	85.1	86.0
Private insurance	7,247	7,710	8,162	11,710	80.1	80.4	82.8	83.1
Medicare	6,143	6,450	6,631	8,953	67.9	67.3	67.3	63.5
Medicaid	7,951	8,426	8,579	11,904	87.9	87.9	87.0	84.5
State-financed health insurance plan other than Medicaid	5,242	5,335	5,568	7,523	57.9	55.7	56.5	53.4
Federal military insurance	4,435	4,425	4,774	6,255	49.0	46.2	48.4	44.4

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

Table A.20: Counts and Percentages of Substance Use and Mental Health Treatment Facilities Offering Tailored Programs for Clients with HIV or AIDS, 2021–2024 N-SUMHSS (Figure 3.7.1)

Facility Type	Counts ¹				Percentages ¹			
	2021	2022	2023	2024	2021	2022	2023	2024
SU ²	3,604	4,117	4,494	5,017	25.7	27.7	30.7	31.4
MH ³	1,243	1,415	1,663	2,830	13.7	14.8	16.9	20.1
Combined SU and MH ⁴	578	899	1,299	3,351	23.7	27.4	34.2	37.9

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² In 2021, there were 14,101 SU facilities; in 2022, there were 14,854 SU facilities; in 2023, there were 14,620 SU facilities; in 2024, there were 15,953 SU facilities.

³ In 2021, there were 9,050 MH facilities; in 2022, there were 9,586 MH facilities; in 2023, there were 9,856 MH facilities; in 2024, there were 14,091 MH facilities.

⁴ In 2021, there were 2,438 SU/MH facilities; in 2022, there were 3,280 SU/MH facilities; in 2023, there were 3,795 SU/MH facilities; in 2024, there were 8,839 SU/MH facilities.

Table A.21: Counts and Percentages of Substance Use and Mental Health Treatment Facilities Offering Tailored Programs for Veterans, 2021–2024 N-SUMHSS (Figure 3.7.2)

Facility Type	Counts ¹				Percentages ¹			
	2021	2022	2023	2024	2021	2022	2023	2024
SU ²	4,023	4,569	4,965	5,740	28.7	30.8	34.0	36.0
MH ³	1,842	2,156	2,434	4,001	20.4	22.5	24.7	28.4
Combined SU and MH ⁴	754	1,144	1,542	3,967	30.9	34.9	40.6	44.9

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² In 2021, there were 14,101 SU facilities; in 2022, there were 14,854 SU facilities; in 2023, there were 14,620 SU facilities; in 2024, there were 15,953 SU facilities.

³ In 2021, there were 9,050 MH facilities; in 2022, there were 9,586 MH facilities; in 2023, there were 9,856 MH facilities; in 2024, there were 14,091 MH facilities.

⁴ In 2021, there were 2,438 SU/MH facilities; in 2022, there were 3,280 SU/MH facilities; in 2023, there were 3,795 SU/MH facilities; in 2024, there were 8,839 SU/MH facilities.

Table A.22: Counts and Percentages of Substance Use and Mental Health Treatment Facilities Offering Tailored Programs for Active-duty Military, 2021–2024 N-SUMHSS (Figure 3.7.3)

Facility Type	Counts ¹				Percentages ¹			
	2021	2022	2023	2024	2021	2022	2023	2024
SU ²	2,507	2,923	3,307	3,811	17.9	19.7	22.6	23.9
MH ³	873	1,112	1,408	2,370	9.6	11.6	14.3	16.8
Combined SU and MH ⁴	503	759	1,129	2,744	20.6	23.1	29.7	31.0

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² In 2021, there were 14,101 SU facilities; in 2022, there were 14,854 SU facilities; in 2023, there were 14,620 SU facilities; in 2024, there were 15,953 SU facilities.

³ In 2021, there were 9,050 MH facilities; in 2022, there were 9,586 MH facilities; in 2023, there were 9,856 MH facilities; in 2024, there were 14,091 MH facilities.

⁴ In 2021, there were 2,438 SU/MH facilities; in 2022, there were 3,280 SU/MH facilities; in 2023, there were 3,795 SU/MH facilities; in 2024, there were 8,839 SU/MH facilities.

Table A.23: Counts and Percentages of Substance Use and Mental Health Treatment Facilities Offering Suicide Prevention Services, 2021–2024 N-SUMHSS (Figure 3.8)

Facility Type	Counts ¹				Percentages ¹			
	2021	2022	2023	2024	2021 (n = 14,010)	2022 (n = 14,854)	2023 (n = 14,620)	2024 (n = 15,953)
SU ²	6,503	7,854	8,490	10,131	46.4	52.9	58.1	63.5
MH ³	6,332	6,646	6,924	9,645	70.0	69.3	70.3	68.4
Combined SU and MH ⁴	2,062	2,824	3,247	7,317	84.6	86.1	85.6	82.8

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² In 2021, there were 14,101 SU facilities; in 2022, there were 14,854 SU facilities; in 2023, there were 14,620 SU facilities; in 2024, there were 15,953 SU facilities.

³ In 2021, there were 9,050 MH facilities; in 2022, there were 9,586 MH facilities; in 2023, there were 9,856 MH facilities; in 2024, there were 14,091 MH facilities.

⁴ In 2021, there were 2,438 SU/MH facilities; in 2022, there were 3,280 SU/MH facilities; in 2023, there were 3,795 SU/MH facilities; in 2024, there were 8,839 SU/MH facilities.

Table A.24: Counts and Percentages of Substance Use Treatment Facilities Utilizing Pharmacotherapies, 2021–2024 N-SUMHSS (Figure 3.9)

	Counts ¹				Percentages ^{1,2}			
	2021	2022	2023	2024	2021 (n = 14,010)	2022 (n = 14,854)	2023 (n = 14,620)	2024 (n = 15,953)
Facilities using any pharmacotherapy	9,736	10,627	10,746	12,207	69.5	71.5	73.5	76.5
Facilities providing medication for opioid use disorder ³	7,753	8,468	8,690	9,842	55.3	57.0	59.4	61.7
Facilities providing medication for alcohol use disorder ⁴	5,312	6,221	6,749	7,953	37.9	41.9	46.2	49.9
Facilities using pharmacotherapies for tobacco cessation ⁵	5,705	6,225	6,482	7,628	40.7	41.9	44.3	47.8

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² The sum of the percentages may exceed 100% as some facilities offer multiple pharmacotherapies for SU treatment.

³ Includes facilities that report providing methadone, buprenorphine, or injectable naltrexone for opioid use disorder (OUD). Methadone is only available at opioid treatment programs (OTPs) certified by SAMHSA's Center for Substance Abuse Treatment (CSAT). Excludes facilities that report accepting clients using MOUD prescribed by another facility.

⁴ Includes facilities that report providing disulfiram, naltrexone, or acamprosate for alcohol use disorder (AUD). Excludes facilities that report accepting clients using MAUD prescribed by another facility.

⁵ Includes facilities that report providing nicotine replacement pharmacotherapies or non-nicotine smoking/tobacco cessation medications.

Table A.25: Counts and Percentages of Substance Use Treatment Facilities Offering Assessment and Pre-treatment Services, 2021–2024 N-SUMHSS (Figure 3.10)

Service	Counts ¹				Percentages ¹			
	2021	2022	2023	2024	2021 (n = 14,010)	2022 (n = 14,854)	2023 (n = 14,620)	2024 (n = 15,953)
Screening for substance use	13,615	14,402	14,296	15,623	97.2	97.0	97.8	97.9
Screening for mental health disorders	11,300	12,125	12,106	13,519	80.7	81.6	82.8	84.7
Comprehensive substance use assessment or diagnosis	13,302	14,067	13,974	15,239	94.9	94.7	95.6	95.5
Comprehensive mental health assessment or diagnosis	8,355	8,942	9,079	10,507	59.6	60.2	62.1	65.9
Complete medical history and physical exam by healthcare practitioner	4,858	5,913	6,363	7,435	34.7	39.8	43.5	46.6
Screening for tobacco use	11,149	11,858	11,978	13,384	79.6	79.8	81.9	83.9
Outreach to persons in the community who may need treatment	9,513	10,194	10,200	11,330	67.9	68.6	69.8	71.0
Interim services for clients when immediate admission is not possible	7,174	7,521	7,415	8,245	51.2	50.6	50.7	51.7
Professional interventionist/educational consultant	3,478	4,183	4,457	5,273	24.8	28.2	30.5	33.1
None of the services above	76	51	42	53	0.5	0.3	0.3	0.3

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

Table A.26: Counts and Percentages of Substance Use Treatment Facilities Offering Testing Services, 2021-2024 N-SUMHSS (Figure 3.11)

Service	Counts ¹				Percentages ¹			
	2021	2022	2023	2024	2021 (n = 14,010)	2022 (n = 14,854)	2023 (n = 14,620)	2024 (n = 15,953)
Drug and alcohol oral fluid testing	5,226	5,942	6,210	7,029	37.3	40.0	42.5	44.1
Breathalyzer or other blood alcohol testing	8,960	9,210	9,143	9,719	64.0	62.0	62.5	60.9
Drug or alcohol urine screening	12,384	13,024	12,974	14,134	88.4	87.7	88.7	88.6
Testing for hepatitis B	4,026	4,307	4,384	5,114	28.7	29.0	30.0	32.1
Testing for hepatitis C	4,607	4,911	5,062	5,830	32.9	33.1	34.6	36.5
HIV testing	4,685	5,016	5,071	5,874	33.4	33.8	34.7	36.8
STD testing	3,924	4,225	4,340	5,082	28.0	28.4	29.7	31.9
TB screening	6,175	6,676	6,791	7,607	44.1	44.9	46.5	47.7
Testing for metabolic syndrome	2,522	3,226	3,501	4,495	18.0	21.7	23.9	28.2
None of the services above	893	951	882	970	6.4	6.4	6.0	6.1

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

Table A.27: Counts and Percentages of Substance Use Treatment Facilities Offering Medical Services, 2021-2024 N-SUMHSS (Figure 3.12)

Service	Counts ¹				Percentages ¹			
	2021	2022	2023	2024	2021 (n = 14,010)	2022 (n = 14,854)	2023 (n = 14,620)	2024 (n = 15,953)
Hepatitis A (HAV) vaccination	1,621	1,724	1,689	2,118	11.6	11.6	11.6	13.3
Hepatitis B (HBV) vaccination	1,624	1,744	1,760	2,207	11.6	11.7	12.0	13.8
None of the services above	9,708	10,542	10,621	11,398	69.3	71.0	72.6	71.4

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

Table A.28: Counts and Percentages of Substance Use Treatment Facilities Offering Education and Counseling Services, 2021-2024 N-SUMHSS (Figure 3.13)

Service	Counts ¹				Percentages ¹			
	2021	2022	2023	2024	2021 (n = 14,010)	2022 (n = 14,854)	2023 (n = 14,620)	2024 (n = 15,953)
HIV or AIDS education, counseling, or support	8,720	9,196	9,085	9,950	62.2	61.9	62.1	62.4
Hepatitis education, counseling, or support	8,009	8,499	8,582	9,453	57.2	57.2	58.7	59.3
Health education other than HIV/AIDS or hepatitis	8,544	9,080	8,949	9,967	61.0	61.1	61.2	62.5
Substance use education	13,601	14,364	14,221	15,587	97.1	96.7	97.3	97.7

Service	Counts ¹				Percentages ¹			
	2021	2022	2023	2024	2021 (n = 14,010)	2022 (n = 14,854)	2023 (n = 14,620)	2024 (n = 15,953)
Smoking/ tobacco cessation counseling	9,093	10,007	10,192	11,582	64.9	67.4	69.7	72.6
Individual counseling	13,456	14,179	14,045	15,313	96.0	95.5	96.1	96.0
Group counseling	12,816	13,551	13,366	14,469	91.5	91.2	91.4	90.7
Family counseling	10,803	11,098	10,652	11,442	77.1	74.7	72.9	71.7
Marital/couples counseling	7,600	7,580	7,226	7,636	54.2	51.0	49.4	47.9
Vocational training or educational support	2,901	3,324	3,388	3,919	20.7	22.4	23.2	24.6
None of the services above	39	20	44	24	0.3	0.1	0.3	0.2

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

Table A.29: Counts and Percentages of Substance Use Treatment Facilities Offering Ancillary Services, 2021-2024 N-SUMHSS (Figure 3.14)

Service	Counts ¹				Percentages ¹			
	2021	2022	2023	2024	2021 (n = 14,010)	2022 (n = 14,854)	2023 (n = 14,620)	2024 (n = 15,953)
Case management services	11,727	12,343	12,322	13,515	83.7	83.1	84.3	84.7
Integrated primary care services	3,428	4,132	4,369	5,227	24.5	27.8	29.9	32.8
Social skills development	10,293	10,710	10,545	11,460	73.5	72.1	72.1	71.8
Child care for clients' children	725	714	664	663	5.2	4.8	4.5	4.2
Domestic violence services, including family or partner violence services, for physical, sexual, or emotional abuse	5,021	5,062	4,855	5,253	35.8	34.1	33.2	32.9
Early intervention for HIV	3,175	3,283	3,309	3,596	22.7	22.1	22.6	22.5
Transportation assistance to treatment	6,946	7,275	7,387	8,406	49.6	49.0	50.5	52.7
Mental health services	9,931	10,512	10,551	12,019	70.9	70.8	72.2	75.3
Suicide prevention services	6,503	7,854	8,490	10,131	46.4	52.9	58.1	63.5
Acupuncture	764	759	689	764	5.5	5.1	4.7	4.8
Residential beds for clients' children	358	397	382	373	2.6	2.7	2.6	2.3
None of the services above	273	324	308	254	1.9	2.2	2.1	1.6

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

Table A.30: Counts and Percentages of Mental Health Treatment Facilities Offering Services and Practices, 2021-2024 N-SUMHSS (Figure 3.15)

Service	Counts ¹				Percentages ¹			
	2021	2022	2023	2024	2021 (n = 9,050)	2022 (n = 9,586)	2023 (n = 9,856)	2024 (n = 14,091)
Assertive community treatment (ACT)	1,449	1,498	1,632	2,239	16.0	15.6	16.6	15.9
Intensive case management (ICM)	2,227	2,255	2,368	3,179	24.6	23.5	24.0	22.6
Case management (CM)	6,512	6,991	7,222	10,561	72.0	72.9	73.3	74.9
Court-ordered treatment	4,806	5,139	5,592	8,401	53.1	53.6	56.7	59.6
Assisted Outpatient Treatment (AOT)	417	460	532	682	4.6	4.8	5.4	4.8
Chronic disease/illness management (CDM)	1,717	1,726	1,715	2,288	19.0	18.0	17.4	16.2
Illness management and recovery (IMR)	2,352	2,249	2,290	2,829	26.0	23.5	23.2	20.1
Integrated primary care services	2,607	2,593	2,719	3,889	28.8	27.0	27.6	27.6
Diet and exercise counseling	3,213	3,231	3,257	4,491	35.5	33.7	33.0	31.9
Family psychoeducation	6,351	6,580	6,825	9,235	70.2	68.6	69.2	65.5
Education services	2,915	2,996	3,104	4,693	32.2	31.3	31.5	33.3
Housing services	2,196	2,377	2,419	3,487	24.3	24.8	24.5	24.7
Supported housing	1,623	1,698	1,738	2,499	17.9	17.7	17.6	17.7
Psychosocial rehabilitation services	3,778	3,889	3,956	5,068	41.7	40.6	40.1	36.0
Vocational rehabilitation services	1,672	1,718	1,689	2,412	18.5	17.9	17.1	17.1
Supported employment	1,910	1,944	1,944	2,537	21.1	20.3	19.7	18.0
Therapeutic foster care	386	436	428	532	4.3	4.5	4.3	3.8
Legal advocacy	588	598	659	927	6.5	6.2	6.7	6.6
Psychiatric emergency walk-in services	2,845	2,736	2,770	3,292	31.4	28.5	28.1	23.4
Suicide prevention services	6,332	6,646	6,924	9,645	70.0	69.3	70.3	68.4
Peer support services	3,716	4,155	4,564	7,196	41.1	43.3	46.3	51.1
Testing for Hepatitis B (HBV)	979	953	908	1,451	10.8	9.9	9.2	10.3
Testing for Hepatitis C (HCV)	1,007	990	972	1,544	11.1	10.3	9.9	11.0
Laboratory tests	2,406	2,886	3,103	4,487	26.6	30.1	31.5	31.8
Metabolic syndrome monitoring	2,449	2,912	3,032	4,288	27.1	30.4	30.8	30.4
HIV testing	1,279	1,125	1,069	1,872	14.1	11.7	10.8	13.3
STD testing	1,197	1,030	1,005	1,740	13.2	10.7	10.2	12.3
TB screening	1,588	1,405	1,346	2,091	17.5	14.7	13.7	14.8
Screening for tobacco use	6,144	6,533	6,796	9,937	67.9	68.2	69.0	70.5
Smoking/vaping/tobacco cessation counseling	4,681	4,916	5,212	7,969	51.7	51.3	52.9	56.6
Nicotine replacement therapy	3,136	3,359	3,439	5,301	34.7	35.0	34.9	37.6

Service	Counts ¹				Percentages ¹			
	2021	2022	2023	2024	2021 (n = 9,050)	2022 (n = 9,586)	2023 (n = 9,856)	2024 (n = 14,091)
Non-nicotine smoking/tobacco cessation medications (by prescription)	2,987	3,138	3,301	5,095	33.0	32.7	33.5	36.2
Other ²	280	361	466	703	3.1	3.8	4.7	5.0
None of these services and practices are offered	91	114	159	249	1.0	1.2	1.6	1.8

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² Other includes other services offered that are not specifically listed in this table. Examples may include substance use screening, anxiety/depression screening, nutrition services, etc.

Table A.31: Counts and Percentages of Substance Use Treatment Facilities Using Medications for Opioid Use Disorder (MOUD) and Medications for Alcohol Use Disorder (MAUD) by State, 2024 N-SUMHSS. (Figure 4.1.1 and Figure 4.1.2)

State/Territory	Counts ¹		Percentages ¹	
	Medications for Opioid Use Disorder ²	Medications for Alcohol Use Disorder ³	Medications for Opioid Use Disorder ²	Medications for Alcohol Use Disorder ³
Alabama	69	42	46.0	28.0
Alaska	44	30	48.9	33.3
Arizona	330	299	61.1	55.4
Arkansas	106	85	68.4	54.8
California	823	641	55.1	42.9
Colorado	175	147	54.5	45.8
Connecticut	149	144	77.6	75.0
Delaware	45	41	73.8	67.2
District of Columbia	11	6	37.9	20.7
Florida	460	358	66.4	51.7
Georgia	194	121	55.4	34.6
Hawaii	14	11	13.1	10.3
Idaho	53	46	49.1	42.6
Illinois	315	261	48.5	40.2
Indiana	311	265	62.7	53.4
Iowa	65	61	33.3	31.3
Kansas	92	81	53.5	47.1
Kentucky	424	357	71.9	60.5
Louisiana	130	102	65.3	51.3
Maine	103	66	68.7	44.0
Maryland	382	285	68.2	50.9
Massachusetts	309	267	72.0	62.2

State/Territory	Counts ¹		Percentages ¹	
	Medications for Opioid Use Disorder ²	Medications for Alcohol Use Disorder ³	Medications for Opioid Use Disorder ²	Medications for Alcohol Use Disorder ³
Michigan	247	215	54.8	47.7
Minnesota	169	121	44.5	31.8
Mississippi	59	51	50.9	44.0
Missouri	207	191	71.9	66.3
Montana	24	20	24.2	20.2
Nebraska	47	57	34.3	41.6
Nevada	75	64	56.8	48.5
New Hampshire	73	64	77.7	68.1
New Jersey	296	245	71.7	59.3
New Mexico	93	71	62.4	47.7
New York	701	640	87.8	80.2
North Carolina	313	214	59.1	40.4
North Dakota	27	27	39.1	39.1
Ohio	548	474	74.4	64.3
Oklahoma	97	70	54.8	39.5
Oregon	140	104	56.5	41.9
Pennsylvania	441	327	75.4	55.9
Puerto Rico	37	21	52.1	29.6
Rhode Island	39	27	63.9	44.3
South Carolina	78	44	75.0	42.3
South Dakota	19	16	36.5	30.8
Tennessee	242	166	63.4	43.5
Texas	297	198	52.9	35.3
Utah	171	149	61.3	53.4
Vermont	39	30	69.6	53.6
Virginia	289	236	77.1	62.9
Washington	175	145	46.4	38.5
West Virginia	123	95	78.8	60.9
Wisconsin	136	128	48.2	45.4
Wyoming	33	25	61.1	46.3
Other jurisdictions/territories ⁴	3	2	37.5	25.0
Total	9,842	7,953	61.7	49.9

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² Includes facilities that report providing methadone, buprenorphine, or injectable naltrexone for opioid use disorder (OUD). Methadone is only available at opioid treatment programs (OTPs) certified by SAMHSA's Center for Substance Abuse Treatment (CSAT). Excludes facilities that report accepting clients using MOUD prescribed by another facility.

³ Includes facilities that report providing disulfiram, naltrexone, or acamprosate for alcohol use disorder (AUD). Excludes facilities that report accepting clients using MAUD prescribed by another facility.

⁴ Other jurisdictions/territories include the Commonwealth of the Northern Mariana Islands, Guam, and the U.S. Virgin Islands.

Table A.32: Counts of Substance Use Treatment Facilities Treating Opioid Use Disorder (OUD) by State, 2024 N-SUMHSS
(Figure 4.2.1 through Figure 4.2.6)

State/Territory	Counts ¹					
	Does not treat OUD	Facilities treat OUD				Accepts clients using medications for OUD
		Does not use medications	Federally certified Opioid Treatment Program (OTP)	Prescribes naltrexone	Utilizes prescribers of buprenorphine	
Alabama	7	11	20	37	58	93
Alaska	1	4	10	30	36	72
Arizona	70	24	71	299	313	326
Arkansas	8	19	23	80	86	91
California	131	36	185	661	741	1,033
Colorado	34	9	34	145	145	188
Connecticut	13	4	48	132	140	130
Delaware	1	0	17	35	43	33
District of Columbia	6	0	5	6	8	17
Florida	72	33	127	370	376	330
Georgia	54	30	76	129	128	143
Hawaii	11	55	4	10	12	30
Idaho	1	5	7	44	49	83
Illinois	98	32	96	239	264	361
Indiana	25	16	36	260	286	293
Iowa	12	2	8	57	41	154
Kansas	13	5	11	75	80	101
Kentucky	30	7	46	367	364	369
Louisiana	8	19	13	107	107	110
Maine	10	2	19	59	93	82
Maryland	40	13	119	265	315	346
Massachusetts	14	2	92	246	264	285
Michigan	32	19	56	223	206	302
Minnesota	13	11	24	106	152	291
Mississippi	11	8	5	46	51	72
Missouri	23	3	32	177	171	164
Montana	1	4	3	16	24	83
Nebraska	17	22	7	36	39	81
Nevada	12	7	15	58	70	88
New Hampshire	4	0	16	57	64	58
New Jersey	16	14	69	243	252	270

State/Territory	Counts ¹					
	Does not treat OUD	Facilities treat OUD				Accepts clients using medications for OUD
		Does not use medications	Federally certified Opioid Treatment Program (OTP)	Prescribes naltrexone	Utilizes prescribers of buprenorphine	
New Mexico	10	9	22	66	81	60
New York	13	4	143	608	637	523
North Carolina	63	55	88	189	268	224
North Dakota	6	6	3	25	18	51
Ohio	47	10	131	501	469	428
Oklahoma	11	7	19	46	83	74
Oregon	15	7	33	99	112	174
Pennsylvania	8	6	115	340	379	397
Puerto Rico	8	12	11	10	30	21
Rhode Island	5	1	20	31	32	37
South Carolina	8	5	29	50	64	50
South Dakota	6	7	2	16	14	28
Tennessee	36	11	34	176	170	219
Texas	50	30	97	197	220	322
Utah	24	18	21	149	149	173
Vermont	2	1	10	30	36	26
Virginia	28	7	71	223	258	151
Washington	33	10	40	132	152	257
West Virginia	3	4	15	100	118	73
Wisconsin	25	9	23	131	115	183
Wyoming	1	4	2	24	33	42
Other jurisdictions/territories ²	2	1	1	2	1	3
Total	1,192	640	2,224	7,760	8,417	9,595

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² Other jurisdictions/territories include the Commonwealth of the Northern Mariana Islands, Guam, and the U.S. Virgin Islands.

Table A.33: Percentages of Substance Use Treatment Facilities Treating Opioid Use Disorder (OUD) by State, 2024 N-SUMHSS (Figure 4.2.1 through Figure 4.2.6)

State/Territory	Percentages ¹					
	Does not treat OUD	Facilities treat OUD				Accepts clients using medications for OUD
		Does not use medications	Federally certified Opioid Treatment Program (OTP)	Prescribes naltrexone	Utilizes prescribers of buprenorphine	
Alabama	4.7	7.3	13.3	24.7	38.7	62.0
Alaska	1.1	4.4	11.1	33.3	40.0	80.0
Arizona	13.0	4.4	13.1	55.4	58.0	60.4
Arkansas	5.2	12.3	14.8	51.6	55.5	58.7
California	8.8	2.4	12.4	44.2	49.6	69.1
Colorado	10.6	2.8	10.6	45.2	45.2	58.6
Connecticut	6.8	2.1	25.0	68.8	72.9	67.7
Delaware	1.6	0.0	27.9	57.4	70.5	54.1
District of Columbia	20.7	0.0	17.2	20.7	27.6	58.6
Florida	10.4	4.8	18.3	53.4	54.3	47.6
Georgia	15.4	8.6	21.7	36.9	36.6	40.9
Hawaii	10.3	51.4	3.7	9.3	11.2	28.0
Idaho	0.9	4.6	6.5	40.7	45.4	76.9
Illinois	15.1	4.9	14.8	36.8	40.6	55.5
Indiana	5.0	3.2	7.3	52.4	57.7	59.1
Iowa	6.2	1.0	4.1	29.2	21.0	79.0
Kansas	7.6	2.9	6.4	43.6	46.5	58.7
Kentucky	5.1	1.2	7.8	62.2	61.7	62.5
Louisiana	4.0	9.5	6.5	53.8	53.8	55.3
Maine	6.7	1.3	12.7	39.3	62.0	54.7
Maryland	7.1	2.3	21.3	47.3	56.3	61.8
Massachusetts	3.3	0.5	21.4	57.3	61.5	66.4
Michigan	7.1	4.2	12.4	49.4	45.7	67.0
Minnesota	3.4	2.9	6.3	27.9	40.0	76.6
Mississippi	9.5	6.9	4.3	39.7	44.0	62.1
Missouri	8.0	1.0	11.1	61.5	59.4	56.9
Montana	1.0	4.0	3.0	16.2	24.2	83.8
Nebraska	12.4	16.1	5.1	26.3	28.5	59.1
Nevada	9.1	5.3	11.4	43.9	53.0	66.7
New Hampshire	4.3	0.0	17.0	60.6	68.1	61.7
New Jersey	3.9	3.4	16.7	58.8	61.0	65.4
New Mexico	6.7	6.0	14.8	44.3	54.4	40.3

State/Territory	Percentages ¹					
	Does not treat OUD	Facilities treat OUD				Accepts clients using medications for OUD
		Does not use medications	Federally certified Opioid Treatment Program (OTP)	Prescribes naltrexone	Utilizes prescribers of buprenorphine	
New York	1.6	0.5	17.9	76.2	79.8	65.5
North Carolina	11.9	10.4	16.6	35.7	50.6	42.3
North Dakota	8.7	8.7	4.3	36.2	26.1	73.9
Ohio	6.4	1.4	17.8	68.0	63.6	58.1
Oklahoma	6.2	4.0	10.7	26.0	46.9	41.8
Oregon	6.0	2.8	13.3	39.9	45.2	70.2
Pennsylvania	1.4	1.0	19.7	58.1	64.8	67.9
Puerto Rico	11.3	16.9	15.5	14.1	42.3	29.6
Rhode Island	8.2	1.6	32.8	50.8	52.5	60.7
South Carolina	7.7	4.8	27.9	48.1	61.5	48.1
South Dakota	11.5	13.5	3.8	30.8	26.9	53.8
Tennessee	9.4	2.9	8.9	46.1	44.5	57.3
Texas	8.9	5.3	17.3	35.1	39.2	57.4
Utah	8.6	6.5	7.5	53.4	53.4	62.0
Vermont	3.6	1.8	17.9	53.6	64.3	46.4
Virginia	7.5	1.9	18.9	59.5	68.8	40.3
Washington	8.8	2.7	10.6	35.0	40.3	68.2
West Virginia	1.9	2.6	9.6	64.1	75.6	46.8
Wisconsin	8.9	3.2	8.2	46.5	40.8	64.9
Wyoming	1.9	7.4	3.7	44.4	61.1	77.8
Other jurisdictions/territories ¹	25.0	12.5	12.5	25.0	12.5	37.5
Total	7.5	4.0	13.9	48.6	52.8	60.1

¹ Other jurisdictions/territories include the Commonwealth of the Northern Mariana Islands, Guam, and the U.S. Virgin Islands.

Table A.34: Counts of Substance Use Treatment Facilities Providing Medication Services for Opioid Use Disorder (OUD) by State, 2024 N-SUMHSS (Figure 4.3.1 through Figure 4.3.7)

State/ Territory	Counts ¹						
	Maintenance with methadone or buprenorphine	Medically supervised withdrawal	Detoxification with methadone or buprenorphine	Detoxification with lofexidine or clonidine	Relapse prevention	Other	None
Alabama	59	34	19	11	29	0	4
Alaska	32	19	5	3	16	6	9
Arizona	261	165	60	34	212	50	24
Arkansas	83	58	10	9	78	7	4
California	576	464	392	254	523	138	57
Colorado	115	73	33	16	98	25	32
Connecticut	118	58	20	10	100	43	12
Delaware	43	30	2	2	32	10	2
District of Columbia	10	4	3	2	6	0	0
Florida	308	209	129	88	290	82	26
Georgia	135	100	64	39	93	37	16
Hawaii	10	7	7	3	7	1	1
Idaho	34	26	9	6	34	7	8
Illinois	234	128	67	29	184	65	28
Indiana	230	108	63	45	164	33	37
Iowa	47	28	13	7	51	7	3
Kansas	70	38	8	8	55	9	13
Kentucky	358	217	63	41	282	58	28
Louisiana	85	65	50	41	77	13	8
Maine	84	49	14	8	54	13	4
Maryland	312	193	96	65	224	66	27
Massachusetts	235	126	59	39	208	52	22
Michigan	179	153	68	40	172	52	16
Minnesota	104	50	14	10	78	24	42
Mississippi	47	29	24	14	32	6	4
Missouri	146	91	22	12	162	8	6
Montana	18	13	7	3	13	4	4
Nebraska	26	17	7	5	29	3	7
Nevada	68	44	25	18	43	17	2
New Hampshire	62	34	11	9	50	16	2
New Jersey	221	103	49	31	186	43	23
New Mexico	72	35	27	18	48	10	6

State/ Territory	Counts ¹						
	Maintenance with methadone or buprenorphine	Medically supervised withdrawal	Detoxification with methadone or buprenorphine	Detoxification with lofexidine or clonidine	Relapse prevention	Other	None
New York	567	261	105	60	474	147	42
North Carolina	253	138	55	41	131	78	23
North Dakota	14	13	8	8	21	9	3
Ohio	439	266	139	110	361	67	51
Oklahoma	74	37	17	4	48	5	14
Oregon	104	53	22	14	76	28	11
Pennsylvania	353	158	90	62	279	56	24
Puerto Rico	31	16	5	3	14	4	0
Rhode Island	31	27	11	6	26	11	3
South Carolina	64	41	15	8	33	23	4
South Dakota	13	10	5	4	14	1	1
Tennessee	153	102	46	27	127	34	18
Texas	190	144	106	62	149	47	26
Utah	93	69	34	23	101	14	35
Vermont	34	16	4	3	24	7	3
Virginia	238	154	74	57	174	67	13
Washington	128	71	36	26	107	44	18
West Virginia	103	67	27	23	83	24	4
Wisconsin	101	51	20	13	86	24	8
Wyoming	24	23	8	8	23	2	7
Other jurisdictions/ territories ²	1	0	1	0	2	0	0
Total	7,390	4,485	2,268	1,482	5,983	1,597	785

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² Other jurisdictions/territories include the Commonwealth of the Northern Mariana Islands, Guam, and the U.S. Virgin Islands.

Table A.35: Percentages of Substance Use Treatment Facilities Providing Medication Services for Opioid Use Disorder (OUD) by State, 2024 N-SUMHSS (Figure 4.3.1 through Figure 4.3.7)

State/ Territory	Percentages ¹						
	Maintenance with methadone or buprenorphine	Medically supervised withdrawal	Detoxification with methadone or buprenorphine	Detoxification with lofexidine or clonidine	Relapse prevention	Other	None
Alabama	39.3	22.7	12.7	7.3	19.3	0.0	2.7
Alaska	35.6	21.1	5.6	3.3	17.8	6.7	10.0
Arizona	48.3	30.6	11.1	6.3	39.3	9.3	4.4

State/ Territory	Percentages ¹						
	Maintenance with methadone or buprenorphine	Medically supervised withdrawal	Detoxification with methadone or buprenorphine	Detoxification with lofexidine or clonidine	Relapse prevention	Other	None
Arkansas	53.5	37.4	6.5	5.8	50.3	4.5	2.6
California	38.5	31.0	26.2	17.0	35.0	9.2	3.8
Colorado	35.8	22.7	10.3	5.0	30.5	7.8	10.0
Connecticut	61.5	30.2	10.4	5.2	52.1	22.4	6.3
Delaware	70.5	49.2	3.3	3.3	52.5	16.4	3.3
District of Columbia	34.5	13.8	10.3	6.9	20.7	0.0	0.0
Florida	44.4	30.2	18.6	12.7	41.8	11.8	3.8
Georgia	38.6	28.6	18.3	11.1	26.6	10.6	4.6
Hawaii	9.3	6.5	6.5	2.8	6.5	0.9	0.9
Idaho	31.5	24.1	8.3	5.6	31.5	6.5	7.4
Illinois	36.0	19.7	10.3	4.5	28.3	10.0	4.3
Indiana	46.4	21.8	12.7	9.1	33.1	6.7	7.5
Iowa	24.1	14.4	6.7	3.6	26.2	3.6	1.5
Kansas	40.7	22.1	4.7	4.7	32.0	5.2	7.6
Kentucky	60.7	36.8	10.7	6.9	47.8	9.8	4.7
Louisiana	42.7	32.7	25.1	20.6	38.7	6.5	4.0
Maine	56.0	32.7	9.3	5.3	36.0	8.7	2.7
Maryland	55.7	34.5	17.1	11.6	40.0	11.8	4.8
Massachusetts	54.8	29.4	13.8	9.1	48.5	12.1	5.1
Michigan	39.7	33.9	15.1	8.9	38.1	11.5	3.5
Minnesota	27.4	13.2	3.7	2.6	20.5	6.3	11.1
Mississippi	40.5	25.0	20.7	12.1	27.6	5.2	3.4
Missouri	50.7	31.6	7.6	4.2	56.3	2.8	2.1
Montana	18.2	13.1	7.1	3.0	13.1	4.0	4.0
Nebraska	19.0	12.4	5.1	3.6	21.2	2.2	5.1
Nevada	51.5	33.3	18.9	13.6	32.6	12.9	1.5
New Hampshire	66.0	36.2	11.7	9.6	53.2	17.0	2.1
New Jersey	53.5	24.9	11.9	7.5	45.0	10.4	5.6
New Mexico	48.3	23.5	18.1	12.1	32.2	6.7	4.0
New York	71.1	32.7	13.2	7.5	59.4	18.4	5.3
North Carolina	47.7	26.0	10.4	7.7	24.7	14.7	4.3
North Dakota	20.3	18.8	11.6	11.6	30.4	13.0	4.3
Ohio	59.6	36.1	18.9	14.9	49.0	9.1	6.9

State/ Territory	Percentages ¹						
	Maintenance with methadone or buprenorphine	Medically supervised withdrawal	Detoxification with methadone or buprenorphine	Detoxification with lofexidine or clonidine	Relapse prevention	Other	None
Oklahoma	41.8	20.9	9.6	2.3	27.1	2.8	7.9
Oregon	41.9	21.4	8.9	5.6	30.6	11.3	4.4
Pennsylvania	60.3	27.0	15.4	10.6	47.7	9.6	4.1
Puerto Rico	43.7	22.5	7.0	4.2	19.7	5.6	0.0
Rhode Island	50.8	44.3	18.0	9.8	42.6	18.0	4.9
South Carolina	61.5	39.4	14.4	7.7	31.7	22.1	3.8
South Dakota	25.0	19.2	9.6	7.7	26.9	1.9	1.9
Tennessee	40.1	26.7	12.0	7.1	33.2	8.9	4.7
Texas	33.9	25.7	18.9	11.1	26.6	8.4	4.6
Utah	33.3	24.7	12.2	8.2	36.2	5.0	12.5
Vermont	60.7	28.6	7.1	5.4	42.9	12.5	5.4
Virginia	63.5	41.1	19.7	15.2	46.4	17.9	3.5
Washington	34.0	18.8	9.5	6.9	28.4	11.7	4.8
West Virginia	66.0	42.9	17.3	14.7	53.2	15.4	2.6
Wisconsin	35.8	18.1	7.1	4.6	30.5	8.5	2.8
Wyoming	44.4	42.6	14.8	14.8	42.6	3.7	13.0
Other jurisdictions/ territories ¹	12.5	0.0	12.5	0.0	25.0	0.0	0.0
Total	46.3	28.1	14.2	9.3	37.5	10.0	4.9

¹ Other jurisdictions/territories include the Commonwealth of the Northern Mariana Islands, Guam, and the U.S. Virgin Islands.

Table A.36: Counts of Substance Use Treatment Facilities Treating Alcohol Use Disorder (AUD) by State, 2024 N-SUMHSS (Figure 4.4.1 through Figure 4.4.6)

State/Territory	Counts ¹					
	Does not treat AUD	Facilities treat AUD				Accepts clients using medications for AUD
		Does not use medications	Disulfiram	Naltrexone	Acamprosate	
Alabama	21	16	22	42	27	96
Alaska	7	2	14	29	13	73
Arizona	65	48	187	284	219	285
Arkansas	21	21	65	85	71	77
California	169	138	401	628	463	989
Colorado	33	15	123	143	126	201
Connecticut	7	11	122	140	122	104

State/Territory	Counts ¹					Accepts clients using medications for AUD
	Does not treat AUD	Facilities treat AUD				
		Does not use medications	Disulfiram	Naltrexone	Acamprosate	
Delaware	1	0	21	39	21	32
District of Columbia	4	5	4	6	4	18
Florida	101	84	151	352	211	322
Georgia	90	62	48	118	53	150
Hawaii	5	53	7	11	8	41
Idaho	2	7	32	46	29	77
Illinois	67	87	142	253	170	373
Indiana	62	21	203	260	224	280
Iowa	10	10	37	61	46	153
Kansas	15	15	52	80	64	113
Kentucky	55	19	211	346	218	349
Louisiana	21	28	49	99	67	102
Maine	36	9	51	65	54	81
Maryland	73	42	188	279	181	324
Massachusetts	31	10	191	260	210	263
Michigan	41	33	125	207	144	305
Minnesota	34	28	77	119	89	276
Mississippi	10	14	25	49	28	67
Missouri	27	10	136	185	144	150
Montana	6	5	11	19	12	81
Nebraska	11	24	36	57	45	87
Nevada	14	8	38	63	40	92
New Hampshire	13	1	51	63	53	49
New Jersey	46	19	114	240	131	279
New Mexico	23	15	43	70	50	63
New York	48	19	438	610	505	464
North Carolina	87	103	141	202	145	212
North Dakota	0	10	19	26	22	51
Ohio	61	42	281	467	309	418
Oklahoma	15	41	24	50	40	67
Oregon	19	20	66	98	75	160
Pennsylvania	79	17	149	320	172	373
Puerto Rico	20	13	13	9	8	23
Rhode Island	10	7	23	25	23	37

State/Territory	Counts ¹					Accepts clients using medications for AUD
	Does not treat AUD	Facilities treat AUD				
		Does not use medications	Disulfiram	Naltrexone	Acamprosate	
South Carolina	26	12	20	44	30	44
South Dakota	4	13	12	15	11	25
Tennessee	64	48	76	164	72	196
Texas	110	75	115	191	132	262
Utah	21	34	93	148	98	169
Vermont	4	6	26	30	26	25
Virginia	68	18	140	228	165	153
Washington	41	42	75	135	83	227
West Virginia	23	7	70	94	75	64
Wisconsin	18	27	97	128	109	171
Wyoming	1	5	21	25	10	37
Other jurisdictions/territories ²	0	3	2	2	1	5
Total	1,840	1,422	4,878	7,709	5,448	9,135

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² Other jurisdictions/territories include the Commonwealth of the Northern Mariana Islands, Guam, and the U.S. Virgin Islands.

Table A.37: Percentages of Substance Use Treatment Facilities Treating Alcohol Use Disorder (AUD) by State, 2024 N-SUMHSS (Figure 4.4.1 through Figure 4.4.6)

State/Territory	Percentages ¹					Accepts clients using medications for AUD
	Does not treat AUD	Facilities treat AUD				
		Does not use medications	Disulfiram	Naltrexone	Acamprosate	
Alabama	14.0	10.7	14.7	28.0	18.0	64.0
Alaska	7.8	2.2	15.6	32.2	14.4	81.1
Arizona	12.0	8.9	34.6	52.6	40.6	52.8
Arkansas	13.5	13.5	41.9	54.8	45.8	49.7
California	11.3	9.2	26.8	42.0	31.0	66.2
Colorado	10.3	4.7	38.3	44.5	39.3	62.6
Connecticut	3.6	5.7	63.5	72.9	63.5	54.2
Delaware	1.6	0.0	34.4	63.9	34.4	52.5
District of Columbia	13.8	17.2	13.8	20.7	13.8	62.1
Florida	14.6	12.1	21.8	50.8	30.4	46.5
Georgia	25.7	17.7	13.7	33.7	15.1	42.9
Hawaii	4.7	49.5	6.5	10.3	7.5	38.3

State/Territory	Percentages ¹					Accepts clients using medications for AUD
	Does not treat AUD	Facilities treat AUD				
		Does not use medications	Disulfiram	Naltrexone	Acamprosate	
Idaho	1.9	6.5	29.6	42.6	26.9	71.3
Illinois	10.3	13.4	21.8	38.9	26.2	57.4
Indiana	12.5	4.2	40.9	52.4	45.2	56.5
Iowa	5.1	5.1	19.0	31.3	23.6	78.5
Kansas	8.7	8.7	30.2	46.5	37.2	65.7
Kentucky	9.3	3.2	35.8	58.6	36.9	59.2
Louisiana	10.6	14.1	24.6	49.7	33.7	51.3
Maine	24.0	6.0	34.0	43.3	36.0	54.0
Maryland	13.0	7.5	33.6	49.8	32.3	57.9
Massachusetts	7.2	2.3	44.5	60.6	49.0	61.3
Michigan	9.1	7.3	27.7	45.9	31.9	67.6
Minnesota	8.9	7.4	20.3	31.3	23.4	72.6
Mississippi	8.6	12.1	21.6	42.2	24.1	57.8
Missouri	9.4	3.5	47.2	64.2	50.0	52.1
Montana	6.1	5.1	11.1	19.2	12.1	81.8
Nebraska	8.0	17.5	26.3	41.6	32.8	63.5
Nevada	10.6	6.1	28.8	47.7	30.3	69.7
New Hampshire	13.8	1.1	54.3	67.0	56.4	52.1
New Jersey	11.1	4.6	27.6	58.1	31.7	67.6
New Mexico	15.4	10.1	28.9	47.0	33.6	42.3
New York	6.0	2.4	54.9	76.4	63.3	58.1
North Carolina	16.4	19.4	26.6	38.1	27.4	40.0
North Dakota	0.0	14.5	27.5	37.7	31.9	73.9
Ohio	8.3	5.7	38.1	63.4	41.9	56.7
Oklahoma	8.5	23.2	13.6	28.2	22.6	37.9
Oregon	7.7	8.1	26.6	39.5	30.2	64.5
Pennsylvania	13.5	2.9	25.5	54.7	29.4	63.8
Puerto Rico	28.2	18.3	18.3	12.7	11.3	32.4
Rhode Island	16.4	11.5	37.7	41.0	37.7	60.7
South Carolina	25.0	11.5	19.2	42.3	28.8	42.3
South Dakota	7.7	25.0	23.1	28.8	21.2	48.1
Tennessee	16.8	12.6	19.9	42.9	18.8	51.3
Texas	19.6	13.4	20.5	34.0	23.5	46.7
Utah	7.5	12.2	33.3	53.0	35.1	60.6

State/Territory	Percentages ¹					
	Does not treat AUD	Facilities treat AUD				Accepts clients using medications for AUD
		Does not use medications	Disulfiram	Naltrexone	Acamprosate	
Vermont	7.1	10.7	46.4	53.6	46.4	44.6
Virginia	18.1	4.8	37.3	60.8	44.0	40.8
Washington	10.9	11.1	19.9	35.8	22.0	60.2
West Virginia	14.7	4.5	44.9	60.3	48.1	41.0
Wisconsin	6.4	9.6	34.4	45.4	38.7	60.6
Wyoming	1.9	9.3	38.9	46.3	18.5	68.5
Other jurisdictions/territories ¹	0.0	37.5	25.0	25.0	12.5	62.5
Total	11.5	8.9	30.6	48.3	34.2	57.3

¹ Other jurisdictions/territories include the Commonwealth of the Northern Mariana Islands, Guam, and the U.S. Virgin Islands.

Table A.38: Mental Health Treatment Facilities Using MOUD and MAUD to Treat Clients with Co-occurring Mental and Substance Use Disorders by State, 2024 N-SUMHSS (Figure 4.5.1 through Figure 4.5.2)

State/Territory	Counts ¹		Percentages ¹	
	Medications for Opioid Use Disorder	Medications for Alcohol Use Disorder	Medications for Opioid Use Disorder	Medications for Alcohol Use Disorder
Alabama	24	18	19.5	14.6
Alaska	31	26	30.7	25.7
Arizona	232	192	45.0	37.3
Arkansas	65	68	30.4	31.8
California	426	375	35.4	31.1
Colorado	108	95	49.5	43.6
Connecticut	120	121	49.4	49.8
Delaware	29	29	61.7	61.7
District of Columbia	7	6	36.8	31.6
Florida	271	239	42.3	37.3
Georgia	95	63	38.8	25.7
Hawaii	14	13	36.8	34.2
Idaho	31	28	25.8	23.3
Illinois	172	136	34.1	27.0
Indiana	169	162	41.2	39.5
Iowa	57	58	34.8	35.4
Kansas	69	61	51.9	45.9

State/Territory	Counts ¹		Percentages ¹	
	Medications for Opioid Use Disorder	Medications for Alcohol Use Disorder	Medications for Opioid Use Disorder	Medications for Alcohol Use Disorder
Kentucky	179	145	48.2	39.1
Louisiana	78	63	44.8	36.2
Maine	56	45	33.1	26.6
Maryland	173	154	40.2	35.8
Massachusetts	153	143	47.1	44.0
Michigan	155	148	36.7	35.1
Minnesota	155	125	38.7	31.2
Mississippi	43	34	29.3	23.1
Missouri	148	133	53.8	48.4
Montana	17	16	17.7	16.7
Nebraska	31	32	21.5	22.2
Nevada	50	40	40.0	32.0
New Hampshire	39	36	53.4	49.3
New Jersey	171	139	47.8	38.8
New Mexico	54	41	44.3	33.6
New York	405	340	51.1	42.9
North Carolina	172	129	42.9	32.2
North Dakota	15	16	34.9	37.2
Ohio	306	266	40.7	35.4
Oklahoma	86	58	54.8	36.9
Oregon	56	49	26.4	23.1
Pennsylvania	146	135	27.8	25.7
Puerto Rico	23	16	37.7	26.2
Rhode Island	18	17	32.7	30.9
South Carolina	13	12	13.7	12.6
South Dakota	19	8	41.3	17.4
Tennessee	138	113	44.4	36.3
Texas	145	116	32.4	26.0
Utah	126	108	35.5	30.4
Vermont	21	13	31.8	19.7
Virginia	168	120	54.0	38.6
Washington	119	86	31.1	22.5
West Virginia	83	72	60.6	52.6
Wisconsin	95	92	30.0	29.0

State/Territory	Counts ¹		Percentages ¹	
	Medications for Opioid Use Disorder	Medications for Alcohol Use Disorder	Medications for Opioid Use Disorder	Medications for Alcohol Use Disorder
Wyoming	24	23	49.0	46.9
Other jurisdictions/territories ²	3	2	50.0	33.3
Total	5,603	4,775	39.8	33.9

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² Other jurisdictions/territories include the Commonwealth of the Northern Mariana Islands, Guam, and the U.S. Virgin Islands.

Table A.39: Substance Use and Mental Health Treatment Facilities Providing Suicide Prevention Services by State, 2024
N-SUMHSS (Figure 4.6.1 through Figure 4.6.3)

State/Territory		Counts ¹		Percentages ¹		
	SU	MH	Combined SU and MH facilities	SU	MH	Combined SU and MH facilities
Alabama	78	85	45	52.0	69.1	88.2
Alaska	72	73	64	80.0	72.3	92.8
Arizona	399	360	360	73.9	69.9	88.0
Arkansas	128	159	106	82.6	74.3	89.8
California	913	806	521	61.1	66.9	82.8
Colorado	200	161	155	62.3	73.9	86.1
Connecticut	146	159	127	76.0	65.4	80.4
Delaware	34	39	30	55.7	83.0	88.2
District of Columbia	17	11	9	58.6	57.9	64.3
Florida	447	436	340	64.5	68.0	82.5
Georgia	247	192	177	70.6	78.4	88.5
Hawaii	33	23	20	30.8	60.5	66.7
Idaho	74	80	57	68.5	66.7	87.7
Illinois	373	327	240	57.4	64.9	75.0
Indiana	341	331	281	68.8	80.7	89.8
Iowa	138	116	79	70.8	70.7	87.8
Kansas	114	105	83	66.3	78.9	93.3
Kentucky	460	290	283	78.0	78.2	91.3
Louisiana	121	138	100	60.8	79.3	88.5
Maine	71	70	55	47.3	41.4	67.9
Maryland	272	214	185	48.6	49.8	70.9
Massachusetts	257	192	184	59.9	59.1	75.7
Michigan	285	286	224	63.2	67.8	76.7

State/Territory		Counts ¹		Percentages ¹		
	SU	MH	Combined SU and MH facilities	SU	MH	Combined SU and MH facilities
Minnesota	198	252	160	52.1	62.8	72.7
Mississippi	86	102	74	74.1	69.4	89.2
Missouri	228	221	145	79.2	80.4	87.9
Montana	78	62	62	78.8	64.6	96.9
Nebraska	103	105	90	75.2	72.9	87.4
Nevada	101	90	75	76.5	72.0	85.2
New Hampshire	61	55	39	64.9	75.3	92.9
New Jersey	238	198	160	57.6	55.3	70.5
New Mexico	107	100	95	71.8	82.0	92.2
New York	494	592	276	61.9	74.7	81.9
North Carolina	236	175	184	44.5	43.6	63.2
North Dakota	39	34	28	56.5	79.1	90.3
Ohio	447	474	388	60.7	63.1	79.0
Oklahoma	136	141	123	76.8	89.8	97.6
Oregon	183	152	112	73.8	71.7	88.9
Pennsylvania	309	331	158	52.8	63.0	80.2
Puerto Rico	58	50	30	81.7	82.0	90.9
Rhode Island	40	37	28	65.6	67.3	77.8
South Carolina	47	81	10	45.2	85.3	66.7
South Dakota	34	41	25	65.4	89.1	96.2
Tennessee	252	217	185	66.0	69.8	80.8
Texas	342	336	228	61.0	75.2	84.1
Utah	212	248	171	76.0	69.9	83.8
Vermont	42	50	31	75.0	75.8	91.2
Virginia	225	200	196	60.0	64.3	85.2
Washington	254	258	199	67.4	67.4	88.4
West Virginia	110	99	84	70.5	72.3	82.4
Wisconsin	196	244	191	69.5	77.0	90.5
Wyoming	48	43	39	88.9	87.8	92.9
Other jurisdictions/territories ²	7	4	6	87.5	66.7	100.0
Total	10,131	9,645	7,317	3494.6	3746.7	4469.8

¹ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

² Other jurisdictions/territories include the Commonwealth of the Northern Mariana Islands, Guam, and the U.S. Virgin Islands.

Appendix B. N-SUMHSS Background and Methodology

Introduction

For more than two decades, the Substance Abuse and Mental Health Services Administration (SAMHSA) has been collecting data on the substance use (SU) and mental health (MH) services offered by treatment facilities in the United States and its territories¹ using two surveys—the National Survey of Substance Abuse Treatment Services (N-SSATS) and the National Mental Health Services Survey (N-MHSS). In 2021, the N-SSATS and the N-MHSS were combined into one survey—the National Substance Use and Mental Health Services Survey (N-SUMHSS).

The Center for Behavioral Health Statistics and Quality (CBHSQ) at SAMHSA administers the N-SUMHSS. The N-SUMHSS provides a mechanism to quantify the diverse characteristics and composition of SU and MH treatment delivery systems nationwide. The N-SUMHSS collects data on the location, characteristics, and utilization of SU and MH treatment facilities in the United States.

The N-SUMHSS collects multi-purpose data that can be used to:

- Assist SAMHSA, states, and local governments in assessing the nature and scope of services provided in state-supported, state-operated, private non-profit and for-profit SU and MH treatment facilities, and in forecasting SU and MH treatment resource requirements.
- Conduct comparative analyses on treatment provision across the nation, regions, and states. SAMHSA updates and releases N-SUMHSS Annual Reports, Detailed Tables, and State Profiles on an annual basis.²
- Update SAMHSA's Inventory of Substance Use and Mental Health Treatment Facilities (I-TF), an inventory of all SU and MH treatment facilities in the United States and its territories known to SAMHSA.
- Generate the National Directory of Drug and Alcohol Use Treatment Facilities and the National Directory of Mental Health Treatment Facilities (hereinafter referred to as the “National Directories”). Both National Directories are updated and released to the public on an annual basis.
- Update facility services information published on SAMHSA's [FindTreatment.gov](https://www.samhsa.gov/findtreatment), a comprehensive resource and searchable database of public and private facilities for the provision of SU and MH treatment.³

Survey Universe and Coverage

The I-TF is an electronic national inventory of behavioral health facilities maintained by SAMHSA. It contains all SU and MH facilities in the United States and its territories known to SAMHSA. The I-TF is the list frame for the N-SUMHSS. All active and eligible facilities listed in I-TF are contacted to complete the annual N-SUMHSS.⁴

The I-TF contained 27,957 unique behavioral health facilities in the United States and its territories, of which 4,009 unique facilities were found to be either closed or ineligible for the N-SUMHSS. Out of 23,948 facilities eligible for inclusion in the 2024 N-SUMHSS, 17,829 facilities provided SU treatment (SU facilities), 15,421 facilities provided MH treatment (MH facilities), and 9,302 facilities provided both SU and MH treatment (SU/MH facilities).⁵

¹ In the 2024 N-SUMHSS, territories included American Samoa, the Federated Commonwealth of the Northern Mariana Islands, the Federated States of Micronesia, Guam, the Republic of Palau, Puerto Rico, and the U.S. Virgin Islands. In 2024, American Samoa, the Federated States of Micronesia, and the Republic of Palau did not have any eligible facilities.

² To view these reports, please go to <https://www.samhsa.gov/data/data-we-collect/n-sumhss-national-substance-use-and-mental-health-services-survey>

³ For facility eligibility to be listed on [FindTreatment.gov](https://www.samhsa.gov/findtreatment), please see the [FindTreatment.gov](https://www.samhsa.gov/findtreatment) Eligibility section below.

⁴ The inclusion and exclusion criteria for the subset of facilities contacted are listed in the Survey Inclusion and Exclusion Criteria (Appendix B Section 2.2) in this report.

⁵ Facility counts are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH treatment facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

Survey Frame List and Updates

The I-TF contains basic information about each facility, such as name, location address, mailing address, telephone number, director name, and general services offered. Keeping the I-TF facility list updated is of critical importance to improve survey coverage. SAMHSA utilizes comprehensive sources and undertakes major activities to ensure a complete and accurate inventory of behavioral health treatment facilities:

- *Facility List Updated Through States:* Information about new facilities, closed facilities, and changes in facility information in the I-TF is provided primarily by the Single State Agencies (SSAs) and the State Mental Health Agencies (SMHAs). State Representatives designated by the SSAs and SMHAs review, add, and/or update the facility list and information for their states in the I-TF system as they identify, license, certify/decertify facilities, and learn of facilities that no longer provide SU and/or MH treatment services or have physically closed or moved to a different location.
- *Facility List Updated Through the N-SUMHSS:* N-SUMHSS respondents are asked to report all the treatment facilities in their administrative networks when responding to the survey. Facilities identified by the N-SUMHSS respondents are compared against the existing I-TF facility list. Facilities that are determined not to currently exist in the I-TF are added to the I-TF.
- *Facility List Updated Through I-TF Augmentation:* SAMHSA conducts annual augmentation activities to continuously identify new SU and MH facilities that are not currently included in the I-TF. SAMHSA searches for and obtains source files of behavioral health facilities from an array of nationally recognized behavioral health organizations⁶ that maintain facility listings, such as the American Hospital Association. Facilities listed in these sources are compared against the existing I-TF facility list. Facilities that are determined not to currently exist in the I-TF are added to the I-TF and called by SAMHSA to undergo a screening process to determine their eligibility for inclusion in the N-SUMHSS, using an Augmentation Screener Questionnaire.
- *Facility List Updated Through Individual Facilities:* A facility can request to be registered and added to the I-TF through contacting their State Representatives for review and approval or contacting SAMHSA directly by submitting a [Facility Registration Application Form](#). SAMHSA reviews facility registration applications throughout the year and compares the submitted facility information against the existing I-TF facility list. Depending on the time of the year, self-registered facilities that are determined not to currently exist in the I-TF will be added to the I-TF after receiving approval from their State Representatives, completing the screening process, or being invited for and submitting N-SUMHSS responses as special cases, whichever is completed sooner.

Survey Inclusion and Exclusion Criteria

To be eligible for the N-SUMHSS, facilities must first be registered and included in the I-TF. This section details the N-SUMHSS inclusion and exclusion criteria.⁷

A. N-SUMHSS Inclusion Criteria and Guidelines:

The following types of SU and MH treatment facilities are included in the N-SUMHSS:

- *Psychiatric hospitals* are facilities licensed and operated as either state and/or public psychiatric hospitals or as state-licensed private psychiatric hospitals that primarily provide 24-hour inpatient care to persons with mental illness. They may also provide 24-hour residential care and/or less-than-24-hour care (i.e., outpatient, partial hospitalization/day treatment), but these additional service settings are not requirements.
- *General hospitals with a separate inpatient SU and/or psychiatric unit* are licensed general hospitals (public or private) that provide inpatient SU and MH services in separate units. These units must have specifically allocated staff and space for the treatment of persons with SU problems and/or mental illness. The units may be located in the hospital itself or in a separate building that is owned by the hospital.

⁶ New facilities identified through the opioid treatment programs (OTPs) list maintained by SAMHSA's Center for Substance Abuse Treatment (CSAT) are included in the I-TF without going through the screening process.

⁷ Please refer to Appendix B, Survey Inclusion and Exclusion Criteria for various N-SUMHSS data products, [FindTreatment.gov](#), and the National Directories. Please note multi-layered inclusion and exclusion criteria for the I-TF, N-SUMHSS, N-SUMHSS data products, [FindTreatment.gov](#), and the National Directories are interdependent and the criteria are built on another.

- *State hospitals* are hospitals funded and operated by the government of a state.
- *Veterans Affairs (VA) medical centers* are facilities operated by the U.S. Department of Veterans Affairs, including general hospitals with separate SU and/or psychiatric inpatient units, residential treatment programs, and/or outpatient clinics.
- *Certified community behavioral health clinics* are responsible for directly providing (or contracting with partner organizations to provide) nine types of services, with an emphasis on the provision of 24-hour crisis care, utilization of evidence-based practices, care coordination, and integration with physical health care.
- *Partial hospitalization/day treatment facilities* provide only partial day SU and MH services to ambulatory clients, typically in sessions of three or more hours on a regular schedule.
- *Outpatient facilities* provide only outpatient SU and/or MH services to ambulatory clients, typically for less than three hours at a single visit. The services may include detoxification, methadone, and/or buprenorphine treatment.
- *Residential treatment centers (RTCs) for children* are facilities not licensed as psychiatric hospitals that primarily provide individually planned programs of mental health treatment in a residential care setting for children under age 18 years. (Some RTCs for children may also treat young adults.) RTCs for children must have a clinical program that is directed by a psychiatrist, psychologist, social worker, or psychiatric nurse who has a master's or doctoral degree.
- *RTCs for adults* are facilities not licensed as psychiatric hospitals that primarily provide individually planned programs of mental health treatment in a residential care setting for adults.
- *Multi-setting mental health facilities*⁸ provide mental health services in two or more service settings (non-hospital residential, plus either outpatient and/or partial hospitalization or day treatment), and are not classified as a psychiatric hospital, general hospital, medical center, or residential treatment center.
- *Community mental health centers (CMHCs)* provide either (1) outpatient services, including specialized outpatient services for children, the elderly, individuals who are chronically mentally ill, and residents of its mental health service area who have been discharged from inpatient treatment at a mental health facility; (2) 24-hour emergency care services; (3) day treatment or other partial hospitalization services, or psychosocial rehabilitation services; or (4) screening for patients being considered for admission to state mental health facilities to determine the appropriateness of the admission. To be classified as a CMHC, a facility must meet applicable licensing or certification requirements for CMHCs in the state in which it is located.
- *Other types of residential treatment facilities* refer to facilities not licensed as psychiatric hospitals. The primary purpose of other types of residential treatment facilities is to provide individually planned programs of mental health treatment services in a residential care setting; such facilities are not specifically for children or adults only.

B. N-SUMHSS Exclusion Criteria and Guidelines:

The following types of SU and MH treatment facilities are excluded in the N-SUMHSS based on their responses to the prior N-SUMHSS and/or the most recent information in the I-TF:

- SU and/or MH treatment facilities that were identified as closed and/or with a “closed” status in the I-TF;
- SU and/or MH treatment facilities self-reported that they were a jail, prison, or detention center that provided treatment exclusively for incarcerated persons or juvenile detainees;
- SU treatment facilities that self-reported that they only provide non-treatment services (with the exception of state approved non-treatment substance use halfway houses);
- MH treatment facilities self-reported that they were either a solo or small group practice and they were not licensed or accredited as a mental health clinic or a mental health center;
- MH treatment facilities self-reported that they were operated by the United States Department of Defense (DoD); and

⁸ The classification of psychiatric hospital, general hospital, medical center, or residential treatment center—any of which can offer mental health services in two or more service settings—takes precedence over a multi-setting classification.

- MH treatment facilities that self-reported that they only provided non-treatment services, that is, facilities only provided one or more of the following services: crisis intervention services, psychosocial rehabilitation, cognitive rehabilitation, intake, referral, MH evaluation, health promotion, psychoeducational services, transportation services, respite services, consumer-run and peer support services, housing services, or legal advocacy, and/or are residential facilities whose primary function was not to provide specialty MH treatment services. These facilities included nursing homes, foster care/therapeutic foster care, assisted living/supported housing, and group homes.

Survey Content

The 2024 N-SUMHSS questionnaire contained 68 numbered questions. The survey consisted of two screening questions and five separate modules:

- Module A focused on SU treatment services;
- Module B focused on MH treatment services;
- Module C focused on facility characteristics; and
- Module D focused on the number of clients in treatment for a given day, month, or 12-month period.

The screening questions requested information on the primary treatment focus of the facility and whether the facility was a jail. These two questions indicated which module(s) of the survey each facility would be asked to complete. Facilities providing both SU and MH treatment services were asked to complete all four modules, whereas those providing only SU treatment services were asked to complete Modules A, C, and a portion of Module D. Facilities providing only MH treatment services were asked to complete Modules B, C, and a portion of Module D. In addition, facilities operated by the U.S. Department of Veterans Affairs (VA) were asked to complete additional questions regarding services offered. These addenda included seven questions for SU facilities and 12 questions for MH facilities.

Screening topics included:

- primary treatment focus; and
- jail, prison, or detention center status.

Module A - Topics included in the SU portion of the survey included:

- SU treatment services available;
- detoxification from alcohol, benzodiazepines, opioids, cocaine, methamphetamines, or other drugs and routine use of medication during detoxification;
- type of care provided, i.e.,
 - outpatient treatment services (regular outpatient treatment, intensive outpatient treatment, day treatment or partial hospitalization, detoxification, methadone maintenance, buprenorphine maintenance, or naltrexone treatment);
 - residential (non-hospital) treatment services (long-term—more than 30 days, short-term—30 days or fewer, detoxification); and
 - hospital inpatient treatment services (inpatient treatment, inpatient detoxification);
- treatment services offered (assessment and pre-treatment services, medical services, recovery support services, education and counseling services, pharmacotherapies, testing, transitional services, ancillary services, and other services);
- treatment(s) available for opioid use disorder;
- treatment(s) available for alcohol use disorder;

- clinical therapeutic approaches;
- provision of treatment services in sign language and/or in languages other than English;
- special programs or groups provided for specific client types;
- licensure, certification, or accreditation agencies or organizations; and
- payment type or insurance accepted.

Module B - Topics included in the MH portion of the survey included:

- services offered;
- type of care provided (i.e., 24-hour hospital inpatient, 24-hour residential, partial hospitalization/day treatment, and outpatient treatment services);
- type of MH facility (e.g., psychiatric hospital, separate inpatient unit of a general hospital, state hospital, residential treatment center for children, etc.);
- treatment modalities for MH treatment;
- use of antipsychotics and type of medications used;
- services offered (intensive case management, court-ordered treatment, integrated primary care services, education services, suicide prevention services, etc.);
- age groups accepted for treatment;
- special programs or groups provided for specific client types;
- availability of emergency psychiatric services;
- provision of treatment services in sign language and/or in languages other than English;
- quality improvement practices;
- use of seclusion or restraint;
- payment types or insurance accepted; and
- licensure, certification, or accreditation agencies or organizations.

Module C - Topics in the treatment facility information portion of the survey included:

- Federally Qualified Health Center (FQHC) status;
- facility operating entity;
- religious or faith-based organization affiliation;
- facility smoking and vaping policies;
- payment and fee information;
- listing preferences for SAMHSA's [FindTreatment.gov](https://www.samhsa.gov/findtreatment) and National Directories; and
- parent organization information, if applicable.

Module D - Topics in the client counts portion of the survey included:

- number of facilities included in the counts;
- number of clients receiving inpatient, residential, and outpatient SU disorder treatment services on March 29, 2024;
- number of inpatient and residential beds designated for SU disorder treatment on March 29, 2024;

- number of clients in treatment for use of 1) both alcohol and substances other than alcohol, 2) only alcohol, 3) only substances other than alcohol;
- percentage of SU clients with diagnosed co-occurring mental disorder and SU disorder;
- number of SU disorder treatment admissions for the most recent 12-month period;
- number of clients receiving inpatient, residential, and outpatient MH treatment on March 29, 2024;
- number of inpatient and residential beds designated for mental disorders treatment on March 29, 2024;
- number of mental disorder treatment admissions, readmissions, and incoming transfers between March 30, 2023, and March 29, 2024; and
- percentage of total admissions that were military veterans.

Survey Method and Data Collection

Survey Fielding Periods and Reference Date

The 2024 N-SUMHSS field period was from March 29, 2024, through December 9, 2024. The survey reference date was March 29, 2024.

In addition to the annual N-SUMHSS, a “Between Cycle” N-SUMHSS (hereinafter referred to as the “Mini N-SUMHSS”) was conducted as a mechanism of collecting services data from new facilities identified and added to the I-TF between June 2024 and December 2024 (Appendix B, Section 2.1). The Mini N-SUMHSS is a subset of questions derived from the N-SUMHSS. New facilities that have completed the Augmentation Screener Questionnaire and Mini N-SUMHSS, meeting all survey eligibility criteria, may choose to be added to [FindTreatment.gov](https://www.findtreatment.gov) and the relevant National Directories without having to wait for the following year’s survey cycle. Although Mini N-SUMHSS data were not included in the 2024 N-SUMHSS analyses and public use data files, they were used to update information [FindTreatment.gov](https://www.findtreatment.gov) to ensure that new treatment facilities and their service information is available to the public in a timely manner.

Survey Mode

The 2024 N-SUMHSS was a multimode survey. Data collection included three modes: 1) a secure web-based questionnaire, 2) a paper questionnaire, and 3) a computer-assisted telephone interview (CATI). All data collection modes of the N-SUMHSS were available to facilities throughout the data collection period.

Outreach Strategies

Continuous Collaboration with States

States serve as critical influencers, stakeholders, and partners for the N-SUMHSS. The N-SUMHSS Support Team established early contact with State Representatives to inform them of the upcoming survey and request states’ support in encouraging facility participation. Throughout the survey administration, State Representatives were regularly informed of their state’s progress and encouraged to further reinforce survey participation.

Advance Outreach with Facilities

Advance letters and emails announcing the upcoming N-SUMHSS administration were sent to treatment facilities in late February 2024, approximately six weeks before the launch of the survey. The advance contact served as an introduction to the N-SUMHSS to encourage treatment facilities’ participation and reinforce that the effort is nationwide.

Invitation Packet(s)

The first invitation packet was sent via mail and email in late March 2024. The invitation packet included a personalized cover letter, an N-SUMHSS invitation letter, a personalized web survey access flyer with instructions and help desk contact information, FAQs, and a Client Counts Worksheet. Facilities that have not responded by around late May 2024 and by middle July 2024 were sent a second and third packet including a cover letter, a Letter of Support, a personalized web survey access flyer, a Client Counts Worksheet, and a brochure. A hard copy of the paper survey and postage paid business reply envelope were also included in the third packet.

Thank-you Letters and Reminder Emails

The thank-you letter distribution started in May 2024 and continued weekly throughout the survey administration period. Up to 11 reminder emails were sent to facilities that did not respond to the N-SUMHSS approximately one week, three weeks, nine weeks, 26 weeks, 29 weeks, 31 weeks, and 33 weeks after the first invitation packet was sent.

CATI Follow-ups

SAMHSA used CATI calls to follow up with facilities that did not complete the N-SUMHSS starting by July 2024 and continued through the end of October 2024. The calls occurred in two phases:

- Phase 1: Calls to facilities that had not completed the N-SUMHSS to verify the best point of contact for the facility; confirm receipt of the survey materials; and remind them to complete the survey. If the facility requested it, the N-SUMHSS was administered on the phone during this reminder call.
- Phase 2: Calls made in an attempt to complete the N-SUMHSS over the phone.

Technical Assistance

Throughout the survey fielding period, resources were available to help facilities complete the survey, such as a toll-free N-SUMHSS hotline, a designated N-SUMHSS email address, and a dedicated N-SUMHSS information website (<https://info.nsumhss.samhsa.gov/>). In addition, the N-SUMHSS Support Team communicated directly with facilities to answer questions, relayed facility updates to the data team, and helped troubleshoot technical issues. The N-SUMHSS Support Team's ability to pivot from technical support to survey administration, as appropriate, resulted in a better customer service experience and delivered more immediate resolution for participating facilities.

Data Validation and Quality Assurance

Data validation and quality assurance measures are taken throughout all stages of the survey cycle:

- *Pre-survey*: a thorough technical review of the 2024 survey, both web and paper versions, was conducted. Both the web and paper versions of the approved survey were developed and tested in English and Spanish to ensure that the web instrument was programmed accurately with logic and edit checks, and the paper instrument was formatted with clear instructions and navigation guidance.
- *During Survey*: the web instrument included automated validation checks that flagged potential inconsistencies in reported responses and prompted respondents to review, confirm, and/or reconcile answers before advancing to the next question. All completed web surveys underwent an automated review for inconsistencies and missing data. Emails and calls to facilities were placed to clarify questionable responses and obtain missing data. All completed mail questionnaires underwent a manual review for inconsistencies and missing data. Emails and calls to facilities were placed to clarify questionable responses and obtain missing data. After data entry of mail questionnaires, automated quality assurance reviews were conducted on all survey response data. The reviews incorporated the rules used in manual editing plus consistency checks not readily identified by manual review.

- *Post-survey:* once the survey was closed and data validations were completed, a final data cleaning step was performed using an automated machine cleaning program. Extensive, rigorous, and multi-layered data quality control and review process were implemented across all N-SUMHSS data products.

Survey Products and Dissemination

The N-SUMHSS data is disseminated in the format of N-SUMHSS Annual Report,⁹ N-SUMHSS State Profiles,¹⁰ Public Use Files (PUFs),¹¹ FindTreatment.gov, and the National Directories,¹² in addition to various short reports, infographics, and special studies.

N-SUMHSS Annual Report

A. Data Presented in 2024 N-SUMHSS Annual Report

There were 20,813 SU, 17,754 MH, and 10,610 combined SU/MH treatment facilities in the survey, of which 2,984 (14.3%) SU, 2,333 (13.1%) MH, and 1,308 (12.3%) SU-MH treatment facilities were found to be closed or ineligible for the N-SUMHSS. Of the treatment facilities eligible for the survey, 16,071 SU facilities, 14,121 MH facilities, and 8,879 combined SU/MH facilities completed the survey. With the exclusion of 118 SU facilities, 30 MH facilities, and 40 SU/MH facilities (see footnotes and “N-SUMHSS Annual Report Inclusion and Exclusion Criteria” below for details), 15,953 SU facilities, 14,091 MH facilities, and 8,839 combined SU/MH facilities were included in the 2024 N-SUMHSS Annual Report. Of those treatment facilities eligible for the report, 98.7% of SU facilities, 99.1% of MH facilities, and 99.3% of combined SU/MH facilities completed the survey on the web, respectively.

B. N-SUMHSS Annual Report Inclusion and Exclusion Criteria

Among the population of facilities included in the 2024 N-SUMHSS survey universe, the following facilities were excluded from the 2024 N-SUMHSS Annual Report based on their responses to the 2024 N-SUMHSS:

- SU and/or MH treatment facilities that were identified as closed;
- SU and/or MH treatment facilities self-reported that they were a jail, prison, or detention center that provided treatment exclusively for incarcerated persons or juvenile detainees;
- SU and/or MH facilities whose client counts were included in or “rolled into” other facilities’ counts and whose facility characteristics were not reported separately;
- SU treatment facilities that self-reported being a solo practice, that is, an office with only one independent practitioner or counselor, and have not been approved by their State Representative;
- SU treatment facilities self-reported that they only provide nontreatment services, that is, facilities that only provide intake assessment, referral, or any other substance use nontreatment services;
- Halfway houses that did not provide SU treatment were excluded from the N-SUMHSS Annual Report, analyses, and public use data files. However, state approved halfway houses were included in the I-TF and the survey universe so that they could be listed in the National Directory of Drug and Alcohol Use Treatment Facilities and on [FindTreatment.gov](https://www.samhsa.gov/data/data-we-collect/n-sumhss-national-substance-use-and-mental-health-services-survey);
- MH treatment facilities self-reported that they were either a solo or small group practice and they were not licensed or accredited as a mental health clinic or a mental health center;
- MH treatment facilities self-reported that they were operated by the U.S. Department of Defense (DoD); and

⁹ N-SUMHSS annual report are available on the SAMHSA website

<https://www.samhsa.gov/data/data-we-collect/n-sumhss-national-substance-use-and-mental-health-services-survey>.

¹⁰ N-SUMHSS state profiles are available on the SAMHSA website <https://www.samhsa.gov/data/quick-statistics>.

¹¹ N-SUMHSS PUFs and Codebooks are available on the SAMHSA website

<https://www.samhsa.gov/data/data-we-collect/n-sumhss-national-substance-use-and-mental-health-services-survey>

¹² The National Directories are available (in both PDF and Excel format) on the SAMHSA website

<https://www.samhsa.gov/data/data-we-collect/n-sumhss-national-substance-use-and-mental-health-services-survey>.

- MH treatment facilities self-reported that they only provided non-treatment services, that is, facilities only provided one or more of the following services: crisis intervention services, psychosocial rehabilitation, cognitive rehabilitation, intake, referral, MH evaluation, health promotion, psychoeducational services, transportation services, respite services, consumer-run and peer support services, housing services, or legal advocacy, and/or are residential facilities whose primary function was not to provide specialty MH treatment services. These facilities included nursing homes, foster care/therapeutic foster care, assisted living/supported housing, and group homes.

Table B.1 presents a summary of eligibility and response information.

Table B.1: Treatment Facilities by Status, Mode of Response, and Type, 2024 N-SUMHSS

	SU Facilities		MH Facilities		Combined SU and MH Facilities ¹³	
	N	%	N	%	N	%
Total facilities in survey	20,813	100.0	17,754	100.0	10,610	100.0
Closed or ineligible	2,984	14.3	2,333	13.1	1,308	12.3
Eligible	17,829	85.7	15,421	86.9	9,302	87.7
Total eligible	17,829	100.0	15,421	100.0	9,302	100.0
Non-respondents	1,758	9.9	1,300	8.4	423	4.5
Respondents	16,071	90.1	14,121	91.6	8,879	95.5
Excluded from report ¹⁴	38	0.2	0	0	25	0.3
Roll-ups ¹⁵	80	0.4	30	0.2	15	0.2
Eligible for report	15,953	89.5	14,091	91.4	8,839	95.0
Mode of response	15,953	100.0	14,091	100.0	8,839	100.0
Internet	15,744	98.7	13,970	99.1	8,781	99.3
Mail	72	0.5	44	0.3	9	0.1
Telephone	137	0.9	77	0.5	49	0.6

¹³ Facility counts and percentages are not mutually exclusive because facilities offering both SU and MH treatment services are included in SU and MH facility counts. Therefore, caution should be exercised when adding, comparing, or interpreting counts and percentages across facility types.

¹⁴ Facilities excluded from the SU counts included 10 halfway houses, 26 solo practitioners, and two facilities that provided administrative services only. Of those, one halfway house facility, 22 solo practitioners, and two facilities that provided administrative services only are excluded from the combined SU and MH counts.

¹⁵ Facilities whose client counts were included in or “rolled into” other facilities’ counts and whose facility characteristics were not reported separately.

C. Response Rate Calculations

The 2024 N-SUMHSS unit response rate among facilities eligible for the survey was calculated using the American Association for Public Opinion Research (AAPOR) Response Rate 4 (RR4) standard definition.¹⁶ AAPOR is the industry standard for survey research and the nationally recognized body with the mission to advance the science of survey research. The RR4 was specifically selected since it meets the following key criteria:

1. RR4 is a well-documented methodology commonly used to calculate survey response rates in surveys of establishments and individuals.
2. RR4 recognizes that a proportion of cases in the frame whose eligibility is unknown may, in fact, be eligible for participation. Applying information about observed eligibility to the subgroup of cases with unknown eligibility, RR4 calculates an eligibility rate to estimate the number of facilities that are likely to be eligible for the survey.
 - a. This happens when the number of cases coded as out of sample (e.g., sample duplicates, establishments that are out of business or have merged with another establishment, or whose services have changed in a manner that renders them ineligible for the N-SUMHSS) is determined to inform the estimated proportion of the non-finalized sample that is ineligible.
3. RR4 includes only completed and partially completed surveys in the numerator of the response rate calculation so that the estimated eligibility can be calculated correctly. The N-SUMHSS methodology designates closed facilities and jails as ineligible for participation and, therefore, removes them from the denominator.
4. RR4 allows the N-SUMHSS multi-mode outcomes from telephone and mail contact to be taken into consideration at the appropriate times.

Table B.2 below provides the 2024 N-SUMHSS completion status and corresponding RR4 code.

Table B.2: Survey Completion Status and RR4 Code, 2024 N-SUMHSS

Final Status	Definition	Count	RR4 Code
001	Complete – Telephone	163	1.0
002	Complete – Mail	105	1.0
004	Closed – No longer exists	1,043	4.1
006	No participation ¹⁷	2,326	3.0
007	Refused ¹⁸	309	2.1
008	Closed – Ineligible or other ¹⁹	2,092	4.1
009	Complete – Web	19,308	1.0
011	Client counts only	95	1.2
015	Client counts unavailable	1,644	1.2
024	Closed – Duplicate facility	495	4.1
034	Closed – Merged	24	4.1
044	Closed – No SU or no MH treatment ²⁰	390	4.1
054	Closed – Satellite facility	19	4.1
064	Primary focus is SU treatment	6	1.0
071	Jail only	5	4.1

¹⁶ <https://aapor.org/wp-content/uploads/2024/03/Standards-Definitions-10th-edition.pdf>

¹⁷ No participation is defined by a facility that did not respond to the survey.

¹⁸ Refused is defined as a facility that actively declined to participate in the survey.

¹⁹ Facility determined to be included in the sample in error because it is not a behavioral health (SU/MH) facility (e.g., bus company).

²⁰ Closed – No SU or no MH treatment is a result of a survey response to that question.

Final Status	Definition	Count	RR4 Code
072	Halfway house only	8	1.0
074	Individual or small group	64	1.0

The overall response rate for the 2024 N-SUMHSS was 90.4%, which was calculated by applying the RR4 codes outlined in the table B.1 as follows:

Response Rate:

$$\frac{(1.0 + 1.2)}{((1.0 + 1.2) + (2.1) + e(3.0))}$$

Where:

- 1.0** is a completed interview,
- 1.2** is a partial interview,
- 2.1** is a refusal,
- 3.0** represents non-contacts (e.g., answering machines, fax machines, callbacks, etc.),
- 4.1** represents ineligible contact (e.g., facility closed, merged with another facility, duplicate of another facility, jail),
- e** is the estimated proportion of cases of unknown eligibility that are eligible.

$$\frac{(1.0 + 1.2 + 2.1)}{(1.0 + 1.2 + 2.1 + 4.1)}$$

Unrolling Client Counts to ‘Child’ Facilities

In the 2024 N-SUMHSS, facilities had the option to (1) report SU and MH client counts separately for each of three service settings (inpatient, residential, and outpatient) for their facility only; (2) report client counts for their facility plus additional facilities; or (3) rely on another facility to report client counts for them. A self-reported facility reported client counts for only itself. A parent facility reported client counts for other facilities, in addition to itself. A child facility relied on a parent facility to report its client counts. The parent facility together with its child facilities is a family of facilities. In tabulations for this report, client counts reported by the parent facility were used. However, for accuracy in certain calculations, such as median counts, clients per facility, and mean beds per facility, client counts reported by a parent facility needed to be distributed (unrolled) among the family of facilities.

To unroll client counts to several facilities within a family of facilities, an empirical distribution of client counts by patients who received each service type was used. This distribution was completed separately for SU and MH client counts and was conditional on the service setting or settings in which the facility provides SU or MH treatment.

The SU inpatient client counts reported by the parent facility were equally distributed among the 24-hour hospital inpatient SU facilities within the family of facilities. The SU residential client counts reported by the parent facility were equally distributed amongst the 24-hour residential SU facilities within the family of facilities. The SU outpatient client counts reported by the parent facility were equally distributed amongst the outpatient SU facilities within the family of facilities. The same methodology was used in the distribution of MH client counts across the MH facilities within a family of facilities.

Table B.3 is an example that best illustrates how to distribute or unroll parent facility SU or MH client counts to a family of facilities: If a parent facility reported 60 inpatient clients, 300 residential clients, and 550 outpatient clients and it indicated that they were reporting for three facilities, then the parent facility’s client counts needed to be distributed among three facilities—A, B, and C. Facility A offers all three service settings, facility B offers hospital inpatient and outpatient service settings, and facility C offers only the hospital inpatient service setting. Therefore, this parent facility is reporting for three inpatient facilities, one residential facility, and two outpatient facilities. Drawing on this information, the following proportions are used to unroll the parent client counts to the three facilities:

Table B.3: Example of Unrolled Facility Client Counts

		Service Setting Type		
		Inpatient	Residential	Outpatient
Parent count		60	300	550
Facility A	Offers all three services	20	300	275
Facility B	Offers hospital inpatient and outpatient	20	0	275
Facility C	Offers only hospital inpatient	20	0	0

National Directories and FindTreatment.gov

Selected information collected from the N-SUMHSS is published and disseminated through the National Directories. The National Directories are listings of public and private SU and MH treatment facilities in the states and jurisdictions respectively. Information about each facility includes facility name, address, telephone number, types of services offered, type of payment, and other features of the facility (such as services for hearing impaired and non-English-speaking clients). The Directories are ordered by state, city, and facility name within each city.

Selected information collected from the N-SUMHSS is also published on SAMHSA's [FindTreatment.gov](https://www.samhsa.gov/findtreatment) which is authorized by the 21st Century Cures Act (Public Law 114-255, Section 9006; 42 U.S.C. 290bb-36d). It is a publicly available and searchable online resource for persons seeking SU and MH treatment services in the United States and its territories. [FindTreatment.gov](https://www.samhsa.gov/findtreatment) had more than 2.8 million views in 2024, reiterating the critical importance of providing comprehensive, timely, and accurate facility and treatment service information to the public through the N-SUMHSS data collection. To be listed on [FindTreatment.gov](https://www.samhsa.gov/findtreatment) is one of the major incentives for facilities responding to the N-SUMHSS.

In addition to meeting the Inclusion and Exclusion Criteria for the I-TF and N-SUMHSS, facilities must also meet the following criteria to be included in the National Directories and [FindTreatment.gov](https://www.samhsa.gov/findtreatment):²¹

- Eligible facilities must have responded to the most recent N-SUMHSS;
- Eligible facilities must provide consent to be listed on the respective National Directory and [FindTreatment.gov](https://www.samhsa.gov/findtreatment) when responding to the N-SUMHSS; and
- Eligible SU facilities must be licensed, certified, or otherwise approved for inclusion in the National Directory and [FindTreatment.gov](https://www.samhsa.gov/findtreatment) by their SSAs.

Data Considerations and Limitations

As with any data collection effort, certain procedural considerations and data limitations must be considered when interpreting data from the 2024 N-SUMHSS, as discussed below:

- The N-SUMHSS is a voluntary facility survey. While substantial effort is made to obtain responses from all known SU and MH treatment facilities within the scope of the survey, some facilities did not respond. There was no adjustment for the 9.6% facility non-response.
- The N-SUMHSS is a point-prevalence survey. It provides information on the SU and MH treatment system and its clients as of a pre-selected reference date (March 29, 2024). Client counts reported here do not represent annual totals. Rather, the N-SUMHSS provides a snapshot of SU and MH treatment facilities and clients for a specific day, month, or 12-month period.
- Multiple responses were allowed for certain questionnaire items (e.g., services provided in non-English languages and type of payment or insurance accepted for treatment services). Tabulations of data for these items include the total number of facilities reporting each response category.

²¹ A step-by-step infographic of "How Are Facilities Listed on Treatment.gov" could be accessed at: https://www.samhsa.gov/data/sites/default/files/How_are_facilities_listed_on_FindTreatment.pdf

- Reported client count totals that fell within a variance of +/- 10 and client percentages that fell within a variance of +/- 5 were not adjusted. Therefore, the public-use file contains percentage values of more than 100%.
- The N-SUMHSS is an integration of the legacy N-SSATS and N-MHSS surveys. The creation of a single survey instrument introduced a data validation issue with the way facilities responded to Questions 1, 1a, A1, A1a, and B1, where respondents were asked about the type of services offered. Some facilities would only accept patients for either MH or SU treatment services. However, if that patient had a co-occurring SU or MH need outside of their intake diagnostic requirement, they would be offered treatment at that facility, as well. For these facilities that will only accept either SU or MH clients, but will offer treatment for a co-occurring disorder, a data change was made to accurately reflect their primary mode of treatment.
- The integration of the N-SSATS and the N-MHSS surveys also introduced the possibility of duplicate records for facilities that had previously completed each survey and beginning in 2021 were now only asked to complete the N-SUMHSS once. Facility information was carefully reviewed to ensure unduplicated information, but it is possible that some duplicate facilities with updated contact information were not identified.
- The N-SUMHSS methodology (including, but not limited to, target population and sampling frame, inclusion and exclusion criteria, survey mode, question wording, data collection period and reference period) is unique and specific to treatment services reported voluntarily by both public and private facilities. Readers of this report should exercise caution when using the N-SUMHSS data for comparisons with any other survey or administrative data, as various data sources may not be directly comparable.

Photos are for illustrative purposes only.
Any person depicted in a photo is a model.

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SAMHSA

Substance Abuse and Mental Health
Services Administration

SAMHSA's mission statement "SAMHSA's mission is to lead the public health service delivery efforts that promote mental health, prevent substance misuse, and provide treatments and supports to foster recovery while ensuring access and better outcomes for all.

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